CHAPTER

1 Basic Concepts of Crisis Intervention

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PREVIEW

Crises occur in a variety of settings for a variety of reasons. Responses to crises can come in various forms and can include multiple levels of complexities. In this chapter, we discuss how crisis intervention may differ from traditional counseling, describe the roles of various professionals and paraprofessionals who work in crisis teams, describe actions crisis workers can take depending on the crisis affecting the client, describe basic historical and contemporary crisis theories, and describe models of crisis assessment and intervention. We start our exploration of crisis intervention by discussing key terms and concepts related to crisis.

LEARNING OBJECTIVES

Upon completion of this chapter, the reader will be able to:

1.1 Describe the key terms and concepts in the field of crisis intervention.
1.2 Compare crisis intervention and traditional counseling.
1.3 Describe the roles of various professionals and paraprofessionals who work in crisis teams.
1.4 Describe approaches crisis workers can take depending on the crisis affecting the client.
1.5 Describe historical and contemporary crisis theories.
1.6 Apply the task model of crisis assessment and intervention to crisis situations.
1.7 Apply other models of crisis assessment and intervention to crisis situations.

KEY TERMS AND CONCEPTS RELATED TO CRISIS

Learning Objective 1.1 Describe the key terms and concepts in the field of crisis intervention.

To help plan effective crisis response strategies, it is important to keep a few key concepts in mind and to be aware that many of the terms used in stress and crisis literature are used inconsistently, interchangeably, or without specificity. While the definitions of many terms seem obvious, some have unique connotations within the context of crisis intervention. In this section, we define key phrases and concepts used to describe crisis intervention theories and models.

What Is a Crisis?

If asked to think about a crisis, what comes to mind? Natural disasters? School shootings? Suicide? Domestic violence? How do some people survive crisis events adaptively and with resilience, while others endure mental health issues for months, years, or a lifetime?

To begin, situations such as tornadoes, earthquakes, acts of terror, and suicide do not in and of themselves constitute crises. A crisis is an event that may or may not be perceived as a disruption in life. A crisis does not necessarily lead to trauma. Typically, a crisis is described using a trilogy definition; that is, there are three essential elements that must be present for an event to be considered a crisis: (1) a precipitating event, (2) a perception of the event that leads to subjective distress, and (3) diminished functioning when the distress is not alleviated by customary coping mechanisms or other resources. Using the trilogy definition, some experience diminished functioning following a crisis event experience, while others, however affected or horrified, usually continue to function as normal. For these individuals, the crisis event was not perceived as traumatic, and it did not disrupt their everyday lives.

James and Gilliland (2017) reviewed numerous definitions of crisis that exist in the literature and summarized crisis as "a perception or experiencing of an event or situation as an intolerable difficulty that exceeds the person's current resources and coping mechanisms" (p. 3). When a crisis is perceived as disruptive to one's life, the crisis may be experienced as a traumatic event. Fortunately, most individuals can work through crisis events. For many who experience a crisis, the experience can assist in preventing or working through future crises, therefore decreasing trauma in the future.

No formula exists that explains why some individuals can work through a crisis, while others who experience the same crisis find it traumatic and disruptive to daily functioning. Therefore, the idea that an individual's perception of an event determines whether the event will become traumatic is paramount to understanding an individual and making a plan of action. However, an individual's perception of an event can be influenced by various determinants, such as the individual's level of resilience, resources, coping mechanisms, and support system.

Often a crisis can lead to additional crises that can be debilitating and affect not only the person but also the entire family or community system. When a person encounters multiple crises, or when past crises the person was exposed to were not resolved effectively, the person or system could experience trauma. When a person experiences trauma, they may need assistance from a crisis counselor to assess the situation, evaluate options for adjusting to the crisis, explore resources currently available to the client, work through the current crisis, and connect with new resources and referrals. As a first exposure to the potential characteristics of a crisis, see Case Study 1.1 and answer the discussion questions that follow. Also read Voices from the Field 1.1.

CASE STUDY 1.1

The Nguyens: A Natural Disaster Affects a Family System

Vin and Li Nguyen are recent immigrants to the United States. They reside in a small town along the Gulf Coast of Mississippi, where many other Vietnamese immigrants have settled. Like many new members of the community, the Nguyens are learning

to speak, read, and write English and are hoping to become naturalized citizens of the United States someday. After arriving in the United States, the Nguyens invested all their money in an old shrimp boat to support themselves by selling their daily catch to local seafood processing facilities.

Recently, the shrimp boat was heavily damaged, and the seafood processing facilities were destroyed by a hurricane. Subsequently, the Nguyens had no income for quite a while. With that and no health insurance, they relied on the county department of public health for prenatal care when Li became pregnant. Li's pregnancy progressed normally; however, her daughter was born with spina bifida. As you read this chapter, try to conceptualize the Nguyens' situation according to the crisis models presented.

Discussion Questions

1. What incidents have occurred in the Nguyens' lives that could be considered provoking stressor events?
2. Beyond these events, are there additional stressors that the Nguyens must address?
3. What resources are the Nguyens using?
4. What additional information do you need to determine whether the Nguyens are in crisis?
5. What factors will predict the outcome for this family?

VOICES FROM THE FIELD 1.1

My First Day

Beth Graney

I spent the summer planning all the classroom lessons and groups I would offer students in my first position as the only school counselor in a K–12 school in rural Iowa. After the principal shared with me that the previous counselor never really connected with many of the kids, I knew I needed to be especially creative to win their trust. The principal told me the town had a saying, "If you aren't born here, you're not from here!" How would a big-city girl from Chicago ever fit in?

All these thoughts raced through my head, drowning out the din of my radio as I drove the 20 miles to school. The newscaster's report that a couple died in a motorcycle accident the previous night barely registered. When I arrived early that morning, the principal greeted me at the door and pulled me into her office. "The parents of two of our students died last night, and other students have arrived at school crying. You have to do something," she blurted as she hurried off to take care of notifying the rest of the staff. My mouth went dry and my thoughts started to race. What should I say? What should I do? What strategies would be most effective? More importantly, I thought, I don't know a single student in this building!

As I entered the large classroom that was now my office, I saw 20 kids ranging in age from early elementary to high school. As I put down my bag on the desk, I looked at the crying kids, pulled up a chair, and said, "Who wants to start?" Someone began with, "They are my friends, my neighbors, and our classmates!" I listened. Soon another child said, "My grandma is sick," and then others said, "My dad lost his job" and "My parents are getting divorced." I listened. As the morning progressed, some kids went on to class, others went home, and more came from class or home to share their grief and fears with the group. I listened some more. When the long day finally ended, I did not know everyone's name that I had written on the sign-in sheet by memory, but I had a growing sense of community.

Two days later, after listening to many students and teachers explain how this tragedy had affected them, the principal told me the funeral service would be held in the gym because the gathering would be so large. She thought it would be important for me to

(*continued*)

be there to support the kids in case anyone needed immediate assistance. I listened as the minister and other family members eulogized the parents. After the service, I met many of the parents and community members, and again I listened to their grief and pain. When a person dies, the family and friends grieve. But in a small town, when someone dies, the whole town grieves.

As I drove home that day, I felt drained and wondered whether I had been helpful because I had no great insights or strategies to offer the students or parents as to why something so difficult and tragic had happened. All I really did was listen. It was then that it struck me that it was the first skill ever taught in my graduate counseling program: Listen! And so began one of my most memorable years in counseling. My phone did not stop ringing and my sign-up sheet was never empty. I made the transition from city girl to rural school counselor simply by listening.

Stress

In Western culture, the word *stress* is widely used to describe emotional phenomena ranging from feeling mild irritation and frustration to being frozen with fear. As the word relates to crises, however, it applies more to the ability to function than it does to affect. The term *stress* was introduced into medical literature by Hans Selye (1978), to describe nonspecific responses of the human body to demands that are placed on it. Stress activates what Selye called the "general adaptation syndrome," which is associated with biological responses. Changes in hormonal patterns, such as increased production of adrenaline and cortisol, over time, may deplete the body's energy resources, impair the immune system, and lead to illness. Selye distinguished between two types of stress: (1) distress (i.e., changes that are perceived negatively) and (2) eustress (i.e., changes that are perceived positively). He noted that distress tends to cause more biological damage than eustress, the latter seeming to contribute to well-being. Thus, how stress is perceived influences adaptation.

The terms *stress* and *crisis* often have been used interchangeably in the literature, thus creating a bit of confusion. Boss (2006) has attempted to distinguish between the two concepts, stating that *stress* is a continuous variable (i.e., stress may be measured by degree), whereas *crisis* is a dichotomous variable (i.e., there either is or is not a crisis). In other words, it is helpful to think of a crisis as simply an event. There either is a crisis or there is not a crisis. Stress may be thought of as a process that exists over time, such as the stress of having a loved one serving in the military in a hostile environment. In contrast, crisis may be thought of as a temporary period during which typical coping ceases and there is intense disorganization and disequilibrium. For example, a family accustomed to coping with the stress of having a loved one serving in the military and stationed in a combat zone may experience crisis when that individual returns home. The family's boundaries, structure, and coping mechanisms may have changed during the loved one's absence, leaving the family inadequately equipped to function with that loved one's homecoming.

Stress is the pressure or tension on an individual or family system. It is a response to demands brought about by a stressor event and represents a change in the equilibrium or steady state of an individual or family system (Boss, 2006; Bush & Price, 2020). The degree of stress experienced hinges on perceptions of, and meanings attributed to, the stressor event. While anything with the potential to change some aspect of the individual or family (e.g., boundaries, roles, beliefs) might produce stress, increased stress levels do not necessarily always lead to crises. Often, stress can be managed, and the family or individual can arrive at a new steady state.

Since stress can be measured by degree, a common and effective technique used in assessing stress levels and changes to stress is *scaling* (Erford, 2020). This simple technique can be used to monitor increases and decreases in stress (as well as just about any other symptom or variable such as depression, sadness, anger, etc.) and then to compare present distress to a baseline prior to the crisis event. This may occur, for example, when asking a client in crisis, "On a scale of 0 to 10 (with 0 being none and 10 being the highest you ever experienced), what would you rate the level of anxiety you are feeling right now?" If you have worked with the client previously, you can use the number as a comparison in the next contact, "Last time we met, you said your anxiety was an 8. Where would you rate it today?" Then the same scaling can be used to understand possible helpful or hindering coping resources, changes in symptoms, or new events. For example, in this dialogue, if the client says they are now at a 2, you could inquire; "The last time we met, you rated your stress at an 8 and now you report a 2. What has made the number lower today?" For a crisis worker, this simple assessment can help better understand whether coping tools or resources are helpful and allow for adjustments. We suggest using a scale from 0 to 10 and identifying the end points so there is no confusion or misinterpretation of rating levels.

Trauma

Stressor events that involve trauma are powerful and overwhelming, and they threaten perceptions of safety and security. Some may be single incidents of relatively short-term duration, whereas others may occur over longer periods of time, resulting in prolonged exposure to the threatening stressor.

Just as the terms *stress* and *crisis* have been incorrectly used interchangeably, so too have the terms *crisis* and *trauma*. A crisis is the event that, by itself, does not result in trauma. The crisis event cannot be controlled. If a person perceives the crisis event in a way that affects daily functioning, the crisis event may become a traumatic experience. Likewise, stress may or may not be experienced because of an event. Rather, it depends on the individual's perception of the crisis.

According to the fifth edition, text revision of the *Diagnostic and Statistical Manual of Mental Disorders* (DSM-5-TR; American Psychiatric Association, 2022), a traumatic event involves threatened or actual death or serious injury, or a threat to the well-being of oneself or another person. Traumatic events may be human-caused accidents or catastrophes. Other traumatic events include acts of deliberate cruelty, such as acts of terrorism, school and workplace shootings, and homicides and sexual assaults. Note that these crisis events created trauma in some people but not in all people. The crisis was an event; trauma was a reaction to that event.

Responses to Trauma

In general, people experiencing traumatic events respond with intense feelings of fear and helplessness (American Psychiatric Association, 2022). Most people respond to trauma within a normal range of reactions to abnormal events, whereby the individual's baseline is not disrupted to the point that it causes impairment or dysfunction. Others become significantly distressed and impaired, and a few develop illnesses such as acute stress disorder (ASD) and posttraumatic stress disorder (PTSD). The risk for psychological disturbance tends to increase with the magnitude or intensity of the traumatic stressor and with the degree to which the event was human caused and intended to harm.

Reactions to traumatic events typically include physical, behavioral, cognitive, emotional, and spiritual responses, which tend to occur in stages but ultimately are temporary. These transient reactions are often referred to as reactions to posttraumatic stress. Physical responses involve the autonomic nervous system as the person prepares to "fight, flee, or freeze" and may be experienced through symptoms such as palpitations, shortness of breath, nausea, muscle tension, headaches, and fatigue. Behaviorally, individuals may experience sleep and dietary changes, social withdrawal, purposeful avoidance of or attention to reminders of the trauma, changes in relationships, and increased use of alcohol or other mood-altering substances. Cognitive responses include rumination, preoccupation, forgetfulness, and difficulty concentrating. Emotional responses include distress, anxiety, impatience, irritability, anger, and symptoms of depression. Finally, spiritual responses are centered on existential questions and attempts to find meaning in the traumatic event. These reactions may transpire over a period of up to 2 years but are not considered pathological.

While most people return to a level of equilibrium and healthy functioning following a reaction to traumatic stress, some may experience consequences that impair their ability to function. Many of these people experience one or two of the trauma- and stressor-related disorders described in the DSM-5-TR: ASD or PTSD. These two disorders are similar in their symptomology and differ mainly in their temporal association with exposure to the traumatic event. According to the DSM-5-TR, the diagnostic criteria for ASD and PTSD include hyperarousal (e.g., hypervigilance, difficulty concentrating, exaggerated startle responses, sleep disturbance), reexperiencing (e.g., flashbacks, nightmares, intrusive thoughts), negative cognitions and mood (e.g., memory lapse, exaggerated negative beliefs, distorted cognitions, lessened interest in activities), and avoidance (e.g., attempts to avoid reminders of the traumatic event, inability to recall components of the event, detachment, dissociation, restricted affect)—symptoms that lead to distress and impairment in key areas of functioning such as work and interpersonal relationships. If these symptoms appear within 1 month of exposure to the trauma, ASD is diagnosed. If exposure to the traumatic event occurred more than a month prior to the development of these symptoms, PTSD is diagnosed. If symptoms persist for more than 3 months, PTSD is considered chronic. For the most part, the role of the crisis counselor would be to gather this information, indicate what the client reported about these events, and list with which symptoms the client presented.

Coping

A crisis event may be perceived as traumatic due to a lack of resources or failed coping strategies. *Coping* is defined as any behavioral or cognitive action that is taken to manage stress. In general, coping is considered a process (Erford, 2023). The process begins with an appraisal of a stressor and its potential for harm. If the situation is perceived as challenging or threatening (as opposed to irrelevant), further assessment takes place to determine what responses are possible and what the potential outcomes might be. Responses to stress fall into three broad categories: problem-focused coping, emotion-focused coping, and avoidance-focused coping (Linley & Joseph, 2004).

Problem-focused coping involves taking direct behavioral actions to change or modify aspects of the environment that are thought to be the causes of stress. When this type of coping strategy is employed, efforts are made to define the problem, generate

possible solutions, weigh alternatives in terms of their costs and benefits, choose an alternative plan, and act. *Emotion-focused coping*, on the other hand, is more likely to be used in situations deemed unlikely to change. The aim of such coping is to reduce affective arousal so that the stressful situation may be tolerated, and it generally involves cognitive processes that change the meaning of a stressor or create emotional distance from it. *Avoidance-focused coping*, the third type of response to stress, may be viewed as a subset of emotion-focused coping, in which responses such as distraction or diversion are employed to avoid the stressor and the emotions that would be associated with it.

Coping may lead to successful or unsuccessful and adaptive or maladaptive outcomes. Following the experience of a traumatic stress event, for example, some individuals may choose to increase their alcohol consumption. While this behavior does little to address the needs brought about by the stressor, it may be an effective (albeit unhealthy) way to keep unwanted emotions at bay (Bush & Price, 2020). Whether coping averts crisis and leads to adaptation depends on the person experiencing stress, the specific type of stress encountered, and the features of the stressful encounter that the person attempts to manage.

Although problem-focused, emotion-focused, and avoidance-focused coping have all been linked to adaptation or posttraumatic growth (Linley & Joseph, 2004), none of these approaches to coping guarantees a positive outcome. Problem-focused coping seems to be most valued in Western culture (Hays & Erford, 2023), yet it can be counterproductive if the situation causing stress is unalterable. If the stressor is unalterable, problem-focused efforts fail and distress is likely to persist. In such times, emotion-focused coping might be the better option. Avoidance-focused coping is least likely to result in adaptation or growth and most likely to lead to impaired well-being (Linley & Joseph, 2004). Complete Think About It 1.1 to assess how you cope with stress.

THINK ABOUT IT 1.1

Think about how you cope with stress (and possibly trauma). What material factors and cognitions help you cope? What people help you cope? How could these material factors, cognitions, and people be considered protective factors in your life?

Adaptation

Adaptation—an outcome of stress or crisis—is the degree to which functioning has changed over an extended period and may be measured by the fit between the individual or family system and the environment. Some individuals and families benefit from the challenges of adversity. Successfully dealing with adversity often results in an outcome that is better than one that might have been reached without the adversity. Many individuals and families change to the point where they have the resources to meet the demands of stressors while continuing to grow. Quite often, changes have occurred in functional behaviors such as rules, roles, boundaries, and interpersonal communication patterns, resulting in families being better equipped to meet the challenges of future stressors. Conversely, for some individuals and families, an imbalance continues between stress demands and the capability to meet those demands. Many families may adopt unhealthy and unproductive responses to stress. Unhealthy coping behaviors, such as

addictions or domestic violence, result in additional stress. Furthermore, it is often the case that coping behaviors that appear to be healthy contribute to stress. A parent, for example, might take a second job to increase the family's financial resources. Working extra hours, however, removes that parent from the home and may contribute to strained family relationships and a decrease in other nontangible resources.

Resilience

Individuals or families that bounce back from adversity are considered to have *resilience*. Numerous studies have attempted to identify factors that determine resiliency in individuals. Although there has been some confusion in the literature about whether to conceptualize resilience as a personal trait or as a dynamic process (Luthar, 2006), the prevailing viewpoint favors process. Thus, resilience may be defined as the dynamics that contribute to positive adaptation following exposure to experiences that have the potential to disrupt functioning (Masten & Obradovic, 2008).

Early studies of resilience focused on individuals with schizophrenia and factors that would predict the course of the illness for them, but most research on resilience has focused on children experiencing adversity. Among the multiple adverse conditions studied have been parental mental illness, chronic illness, poverty, community violence, abuse, and catastrophic events (Luthar, 2006). Through these studies, a variety of concepts were identified as protective factors that operate to ensure resilience. For the most part, these factors involve relationships.

Masten and Obradovic (2008) summarized several fundamental adaptive systems that seem to make a difference in human resilience. Beginning in early childhood, adaptation and the likelihood of resilience seem to depend on the quality of relationships with attachment figures. Attachments that are described as "secure" seem to moderate anxiety by allowing children to feel confident, connected, and reassured. As secure, confident children mature, they develop competence, self-efficacy, and mastery. Mastery develops as individuals learn they can interact successfully with their environments. In times of adversity, those with mastery motivation are likely to persist in their efforts to manage the environment.

Related to mastery is the ability to apply cognitive skills to solve problems. People who are resilient seem to be better at applying cognitive skills during times of adversity than those who are less well adapted. During times of high stress or threat of harm, it is important to be able to continue to think and plan effective responses. Thinking and problem solving during stressful situations are enhanced when emotions, arousal, and attention are self-regulated (Masten & Obradovic, 2008).

Being a part of larger social systems also seems to play a role in resilience. Social groups and networks provide resources such as information and support that are important in dealing with adversity. For children, attending school provides opportunities to acquire knowledge, develop social skills, and practice self-regulation, all resources that might be called for in times of stress. For older children, adolescents, and adults, being part of friendship networks, clubs, work groups, or civic organizations also has the potential to contribute to resilience.

Among the larger systems most studied are religion and spirituality. Religious and spiritual connections provide attachment-like relationships and social support, as well

as opportunities to practice self-regulation through prayer and meditation (Masten & Obradovic, 2008). In addition, religious and spiritual beliefs influence the way people attribute meaning to stressful situations and events and contribute to posttraumatic growth (Gerber et al., 2011).

Whether individuals experiencing adverse conditions demonstrate resilience depends on the operation of protective systems that have evolved through biology and culture (Masten & Obradovic, 2008). If the systems are operating normally, the capacity for resilience is optimal; if they are damaged, the potential for resilience is compromised. Read Voices from the Field 1.2, which discusses resilience within the context of a hurricane.

VOICES FROM THE FIELD 1.2

Human Resilience in the Aftermath of Katrina

Beth Graney

We landed in the New Orleans airport 6 weeks after Hurricane Katrina devastated the coastal areas and encountered the first of many eerie and unsettling sights we would experience over the next 2 weeks. Six school counselors from our county were given permission to volunteer with the Red Cross disaster relief effort to provide counseling to victims of the hurricane. Our mission, or so we thought, was to aid school-age children. However, upon arriving, we quickly found that the Red Cross had different ideas. Due to the devastation caused by the hurricane, most children were still with parents in shelters throughout Louisiana and other states.

As a licensed professional counselor, I was able to function as a counselor/therapist; however, "my office" was outside, at a drive-through Red Cross site handing out food and cleaning supplies to folks who had lost their homes or were beginning the clean-up process. As cars came through, we offered people advice on safety issues for cleaning up mold, pamphlets on medical concerns typical after a devastating event, and warning signs of depression and other mental health concerns. The days were long and hot with a steady stream of cars from 7:00 a.m. to 6:00 p.m. or later. From the beginning, I was talking and listening to people and their experiences, sharing their pain.

At first, I was not clear about how I was offering counseling or mental health assistance. Yet as the days passed, I noticed that many people came again and again, and each time I heard a little more of their story. Some days a person seemed upbeat, only to come back later in the day with tears, anger, or frustration at the bureaucracy involved in getting anything meaningful accomplished. I heard stories of amazing escapes from the hurricane or the flooding when the levees burst. People told me how they witnessed great acts of selflessness and heroism—stories that never made the papers. Others talked of all they lost: "I don't even have one picture of my three kids," "I not only lost my house, my possessions, my car, but my job too," "All of my important medical papers are gone as well as all of my insurance information." Most heartbreaking were the loss of friends and relatives: "I still can't reach my daughter," "My mother died because she wouldn't leave her house." One woman was trying to find a place to continue her chemotherapy treatment from breast cancer, while another talked about leaving with her son who was a quadriplegic and trying to replace important medical equipment she could not bring when she left.

Yet, each day, there was laughter and happiness despite the hurricane, because people had survived and so they could begin again. At times, people pulled their car out of the line so we could talk in-depth about their situation—not for answers necessarily, just for a few brief moments to allow themselves to grieve before they got back in line to pick up yet another bottle of bleach and a new mop to continue their clean-up.

I learned many things about people and how they come to terms with hardship, loss, and devastation. Firsthand, I learned that Maslow was right: People's primary needs such as food, clothing, shelter, and safety must first be met before they can deal with higher-level emotional needs. I learned that people can and do survive against overwhelming odds and the most difficult of circumstances. Often, they do so with great resolve, courage, and even optimism. Quickly I noted that most people were able to deal with the loss of their possessions because they realized they could have

(*continued*)

lost the important people in their lives. Humor, along with revisiting past success, was the most-used strategy to deal with the hardship people were experiencing. I learned that normalizing sadness, fear, grief, and many other emotions comforted people more than I would have expected and that even in the most difficult times, most people see humor and joy in life.

On the trip home, my colleagues and I, after lots of tears and discussion, individually and collectively concluded that people really are resilient and go on to become stronger through difficult times. As for me, I learned that I could sleep on a cot with 80 other people snoring and coughing in the same room. I can go a day without a shower; I can eat a meal from the ERVE (Red Cross Emergency Response Vehicle); and I can stand on my feet for 12 hours a day in hot weather. I learned that, just like the people who survived the storm, you bond with those who have responded to the same disaster because no one else fully understands what you have experienced. Most of all, I learned that I am more resilient than I knew and sometimes the best, most helpful counseling does not take place in a nice, quiet office!

CRISIS INTERVENTION AND TRADITIONAL COUNSELING

Learning Objective 1.2 Compare crisis intervention and traditional counseling.

Now that we have described some of the key terms and concepts related to crisis, let's discuss the similarities and differences between crisis intervention and traditional counseling.

Crisis intervention is a unique strategy within the profession of counseling, distinguished from other forms of counseling by its purpose, setting, time, and intervention plan. The American Counseling Association (Kaplan et al., 2014) defined *counseling* as a "professional relationship that empowers diverse individuals, families, and groups to accomplish mental health, wellness, education, and career goals." While crisis intervention does not stray from this definition, the ways in which relationships are formed, clients are empowered, or goals are accomplished often seem divergent from other forms of counseling. It is sometimes the case that crisis intervention occurs within counseling relationships that are ongoing. For example, a client who already has an established relationship with a counselor in a school or an agency may reveal suicidal ideation, making it necessary for the counselor to adjust to a crisis intervention strategy. At other times, crisis intervention occurs between counselors and clients who do not already know each other. Disaster mental health counselors respond by going to sites where catastrophes have occurred and meeting new clients in these environments.

Whether working in an office or in the field, within established counseling relationships or new ones, the crisis counselor's goal is the safety and stability of clients—that is, working toward restoring equilibrium. Once the client is safe and stable, other forms of counseling may be initiated or resumed. Consider, as an analogy, a car accident in which an individual may have experienced a severe medical trauma. It is essential for the emergency medical technician to stabilize the patient to prevent further injuries prior to transferring (referring) the patient to a surgeon at the hospital. Therefore, the goal of long-term counseling is usually to increase functioning, whereas crisis work prioritizes stabilization and immediate assistance with the presenting distress.

In addition, crisis counselors do not view termination through the same lens as traditional counseling because termination in crisis work may be a referral to other professionals or resources. In severe risk situations, involuntary hospitalization may mark the termination of the crisis counselor's involvement, as discharge from a hospital usually would be to long-term mental health outpatient care and likely not back to the crisis worker. For many crisis workers, termination comes with a referral for emergency

or stabilization care, and the crisis counselor may never learn the status or outcome of the individual they referred. Therefore, it is important for crisis workers to reframe their work with clients in crisis to consider, "How have I made the client's situation better right now?" For a crisis worker this means conducting comprehensive risk assessment to keep the person alive, get them to the next level of long-term care, or connect them with the resources they need. It will not mean working on long-standing issues that led the person to want to take their own life.

As noted, counselors in private practice may have clients who experience a crisis during their time in therapy. In these situations, addressing the crisis would need to be the priority over other long-term goals in a client's treatment plan. Also in these cases, the client may return to the counselor in private practice for continuing care after the crisis subsides.

Another difference between traditional counseling and crisis intervention is the time span and setting of the work. Traditional counseling is typically a scheduled event that lasts for a specific period of time (e.g., a weekly 45- to 60-minute session), and it is expected to consist of more than one such session over a period of time. Traditional counseling sessions take place in mental health agencies, private practices, hospitals, correctional facilities, schools, residential facilities, and other locations related to mental health services (Gladding, 2017). In pure crisis work, there may typically be only one meeting, which may be as brief as a few minutes or as long as half a day. That meeting sometimes occurs in a chaotic setting with minimal privacy. Most crisis workers will see a client in crisis for one and perhaps up to three to four contact sessions. Often at the end of the assessment of the problem, if the distress is not alleviated, the crisis worker will refer the client and provide other resources. Because a crisis is not a scheduled event, crisis intervention usually happens in the spur of the moment. While crisis intervention can take place in nearly any counseling setting, it has much broader borders and could take place in the client's home (e.g., after a child is reported missing), a makeshift shelter (e.g., after a hurricane), an emergency department (e.g., after a sexual assault), or any one of numerous other contexts.

Another difference between crisis and traditional counseling involves diagnosis of individuals in crisis. It is rare to diagnose a person in crisis since most persons are reacting normally to an abnormal event. A premature diagnosis may lead to a treatment plan that is ineffective or harmful. For example, in many situations, referral for medication may be too premature and cause a person to delay healing. Most individuals do not want to experience unpleasant feelings and thoughts even when the processing of them are beneficial to the healing process. Medication may numb the feelings and lead to delayed healing. Too often, clinicians rush too soon into a diagnosis rather than determining whether symptoms are improving or worsening and how the client's condition is impacting functional levels. Crisis workers need to consider risk assessments and client level of functioning prior to referrals for medication during crises. The DSM-5-TR suggests diagnosis only if "the disturbance causes clinically significant distress or impairment in social, academic, occupational, or other important areas of functioning." Crisis workers often document the onset of symptoms related to the presenting problem and continuing reports as symptoms change. In many situations, a client may report already having a medical or mental health diagnosis. In these situations, the information should be documented "as reported by the client" with available documentation of who the professional was who issued the diagnosis.

Crisis intervention may also be conceptualized differently by different types of counselors. For example, clinical mental health counselors and couples and family counselors typically schedule clients who meet on a regular basis and develop a plan to

help guide treatment. For these types of counselors, crisis intervention is seen as a part of counseling and could (in some ways) be anticipated if the counselor knows the client well enough. By contrast, school counselors often see students on an unscheduled basis and because of crises. Thus, for school counselors, crisis intervention is a model used more frequently and without much foresight of the crisis.

Given the distinctions between traditional counseling and crisis intervention, it is important for counselors-in-training to understand they need to reframe the work they will be doing with clients who experience a crisis as (1) conducting a thorough assessment of the person in crisis, (2) providing stabilization, and (3) connecting the person to the next level of care for longer-term counseling, other noncounseling connections, or additional resources to address the crisis. If a counselor tries to address long-term goals with a person in crisis, the assessment and treatment plan will not address the presenting problem.

Trauma-Informed Care

It is also imperative to understand the difference between trauma-informed care (TIC) and crisis assessment/intervention. Often these terms are inappropriately used interchangeably. Components of TIC may be integrated into the crisis assessment process, but TIC is not the same as crisis assessment and intervention.

Crisis assessment is assessing an individual to determine the impact of a crisis (the event). Specific questions and assessments are used depending on the crisis situation. These questions often come from a standard of practice in the field. *Trauma-informed care* recognizes that crisis events may overwhelm an individual and manifest in certain symptomatology. TIC assists an individual in restoring a sense of safety, power, and self-worth. The six guiding principles of TIC include (1) safety; (2) trustworthiness and transparency; (3) peer support and mutual help; (4) collaboration and mutuality; (5) empowerment, voice, and choice; and (6) cultural, historical, and gender issues (Substance Abuse and Mental Health Services Administration, 2014). Of course, the components of TIC may be integrated into the crisis assessment. However, the crisis assessment must be conducted at the foundational level in order to determine the perception of the event as traumatic or causing dysfunction to address the trauma and assist in long-term care.

ROLES OF AND COLLABORATION AMONG MENTAL HEALTH WORKERS DURING A CRISIS

Learning Objective 1.3 Describe the roles of various professionals and paraprofessionals who work in crisis teams.

Most individuals who work in mental health professions are familiar with basic counseling skills that can help clients adapt during crises or stressful situations. The brief descriptions that follow illustrate how different mental health professionals—including professional counselors, psychiatrists, psychologists, social workers, paraprofessionals, and hotline workers—can be helpful in crisis situations.

Professional Counselors

In practice, professional counselors differ from psychiatrists and psychologists in that they work from prevention, developmental, and wellness perspectives. Counselors often work through advocacy and collaboration and play a vital role in a comprehensive crisis

leadership team (Kerr & King, 2018). Professional counselors can provide short-term or long-term counseling and therapy and may work with individuals or groups. Like psychologists, most licensed professional counselors may diagnose and treat mental and emotional disorders. Overall, these mental health professionals need a minimum of a master's degree to become licensed to practice (Newsome & Gladding, 2017). Professional counselors can assist individuals in crisis situations by doing the following:

- Helping them to gain insight into the ways crises affect their lives in a cognitive, behavioral, and emotional manner over a lengthy period
- Providing specific treatment goals and objectives related to crisis
- Monitoring and assessing the magnitude of severity of a crisis
- Providing insight into co-occurring mental and emotional disorders and crisis (e.g., showing a client diagnosed with bipolar disorder how to cope with and monitor a crisis)
- Providing specific crisis intervention strategies during a crisis and over time
- Providing clients in crisis with resources and preventive measures
- Assisting in alleviating symptoms associated with the crisis
- Preparing clients to handle future crises

Some professional counselors bring various types of expertise in crisis intervention to their work, as these examples illustrate:

- Marriage and family counselors work from a systemic perspective. They provide support for couples and families and may involve the family in the resolution of the crisis on a short- and long-term basis.
- Pastoral counselors provide religious or spiritual integration in times of a crisis. Often, a crisis involves a spiritual or religious disconnect or dimension that must be explored. It is also important to understand any religious or spiritual coping mechanisms that were effective in the past that could be applied to the present crisis. Certainly, the notion of incorporating spirituality into crisis intervention has an abundance of benefits and is not the sole property of pastoral counselors. All counselors would want to explore how one's religious and spiritual beliefs can provide solace in times of crises.
- Chemical dependency counselors specifically address the use of drugs and alcohol as a coping mechanism during crisis.
- Professional counselors who specialize in treating children, adolescents, adults, or geriatric populations bring a developmental perspective to the conceptualization and treatment of crisis.
- Rehabilitation counselors assist people diagnosed with acute or chronic disabilities in the personal, social, or psychological effects of those disabilities on employment or independent living.
- School counselors work to maximize student achievement and to "promote equity and access to opportunities and rigorous educational experiences for all students" as well as helping to facilitate "a safe learning environment and work[ing] to safeguard the human rights of all members of the school community" (American School Counselor Association, 2022, p. 2). School counselors can be helpful in school crisis situations by using individual counseling, group counseling, and classroom guidance activities and by collaborating with key stakeholders. Individual counseling can be helpful to those who are directly affected by crisis

(e.g., by working with a student on expressing feelings after a home was destroyed). Providing group counseling to those who have been exposed to crisis (e.g., by establishing a support group for students who have divorced parents) could ease the pain of the initial impact of the crisis and create a support network among the group members. School counselors may provide primary or secondary preventive programs via classroom guidance activities on crisis, suicide, handling stress, self-regulation, and communication skills. Lastly, school counselors may collaborate with school system personnel as well as with parents, families, and members of the community on preventing and responding to crisis (e.g., by providing school staff with materials and training on recognizing suicidal behavior).

Psychiatrists

As noted earlier in this chapter, much of the early work in the field of crisis intervention was done by two psychiatrists working in the Boston area in the 1940s. According to the American Psychiatric Association (2023), psychiatrists are physicians who have obtained specific training and experience in treating mental and emotional disorders. The key difference between the scope of practice of psychiatrists and that of other mental health practitioners is that psychiatrists are medical doctors and can prescribe medication to clients.

Psychologists

Clinical and counseling psychologists are mental health practitioners who engage in the assessment, diagnosis, and treatment of mental and emotional disorders. To be licensed to practice, state licensure boards require completion of a doctoral degree. In a few areas of the United States, including Idaho, Iowa, Illinois, Louisiana, and New Mexico, and in the Indian Health Service and the military, where access to psychiatrists is often limited, psychologists with specific training may prescribe medication. Regarding psychologists' participation in crisis intervention, much of their work is identical to that of professional counselors.

Social Workers

Social workers engaged in crisis intervention also function like professional counselors. Depending on the scope and focus of practice, some social workers may be employed after earning a bachelor's degree, while other, more specialized clinical practice settings may require a master's degree. Although performing many of the same duties as counselors, social workers are well equipped to concentrate on the resource needs of their clients.

Paraprofessionals

Some individuals within the mental health community have limited formal education and training in the professional mental health field, but they perform essential tasks (e.g., case management duties, residential care) for individuals in crisis. Paraprofessionals can do the following:

- Manage resources that help facilitate stabilization (e.g., make sure clients keep all medical, financial, emotional, environmental, and social service appointments).
- Ensure clients are aware of appropriate resources that could be seen as preventive actions to crisis (e.g., make sure that a client has appropriate resources to pay the natural gas bill to have heat in the winter).

- Provide an outlet for clients to decrease isolation and talk to others (e.g., talk with individuals in crisis and provide assurance that someone cares for them and their situation).
- Participate in executing the modality and frequency aspects of the treatment plan—in other words, make the connections set out in the treatment plan. For example, the professional counselor may develop a treatment plan for someone in a residential program that involves attending Alcoholics Anonymous groups daily, attending a group on a specified psychosocial issue three times per week, and addressing a medical issue. The paraprofessional case worker may be responsible for following through with the treatment plan by assisting the client in making the appointments and setting up transportation to the appointments or groups.

To be effective, a crisis worker, whether a professional or a paraprofessional, must be able to (1) rely on life experiences and emotional maturity to remain stable and consistent, (2) remain calm and poised in order to deescalate the situation, (3) use creativity and flexibility to adapt to rapidly changing situations, (4) maintain energy and resilience to keep up with the rigor of working in a crisis situation, and (5) use effective clinical skills in a timely fashion in order to create a trusting and safe environment and suspend one's values just for the crisis time in order to stabilize and refer the client to another professional (James & Gilliland, 2017). Keep these characteristics in mind as you complete Activity 1.1.

ACTIVITY 1.1

In the crisis situations that follow, what might be some activities, interventions, responsibilities, and considerations for each of the mental health professionals discussed earlier? Practice using the task model of crisis assessment and intervention as you process each case.

Crisis Situation 1

Casey and Blake have been married for 3 years. Casey has been cheating on Blake for the past 6 months, and Blake has been cheating on Casey for the past year. Each is unaware of the other's infidelity, but the stress in their house is severe. They yell constantly, and Blake throws objects during arguments. In the past month, Casey has begun to slap Blake during these arguments. Last night Casey hit Blake so hard that Blake was knocked unconscious. Blake is now in the hospital talking to a social worker: "I'm afraid of my partner," Blake says, "but I don't have anywhere else to go. I feel so alone."

Crisis Situation 2

John F. Kennedy High School, located in a primarily low- to middle-class rural community, comprises approximately 1,000 students, most of whom know one another. Within the past 4 months, the school has been evacuated five times because of a call indicating that there is a bomb in the building and that "everyone should get out." As a result of these threats and evacuations, students are scared, parents do not want to send their children to school, and teachers and other school personnel are frightened to go to work. Students also have begun to accuse each other of calling in the bomb threats, and several students have been ostracized and bullied. The Parent-Teacher Association has called a meeting tonight to address this issue.

Crisis Situation 3

Walter has been married for 52 years and has four children and six grandchildren. He is a 74-year-old Black male who has worked all his life as a plumber. Due to intense flash floods within the past week from melting snow, their household is 7 feet under water. When the water came, Walter, his 40-year-old daughter June, and his 13-year-old granddaughter Beverly had to be rescued from their house by boat. Their house is destroyed, Beverly is having nightmares and does not want to leave her family, and June is recovering from a head trauma that occurred while being evacuated.

Hotline Workers

Hotline workers are often the first point of contact for many individuals experiencing a crisis and may handle any number of crises resulting from suicidal and homicidal ideation, domestic violence, substance use, and sexual assault. There are hotlines that specialize in these and other specific types of crisis situations. There are even crisis hotlines for specific age groups (e.g., a hotline dedicated to teen callers). Typically, hotline workers are not mental health professionals but volunteers who have undergone specific training in responding to crises. No matter what their focus, crisis hotlines play a vital role in assessing, intervening in, and preventing the occurrence of crises. Crisis hotline workers perform essential tasks during a crisis situation:

- They assess the severity of the crisis situation and the lethality of the caller.
- They provide immediate crisis intervention to the caller in an attempt to deescalate the crisis. This is critical in a crisis situation because the caller does not have to make an appointment with a professional or wait to get help. Most hotlines are 24-hour services open 365 days a year.
- They provide resources to the caller that may help resolve the crisis (see Table 1.2 at the end of this chapter).

Collaboration Among Crisis Workers

Professional counselors, psychiatrists, psychologists, social workers, various paraprofessionals, and hotline workers can all play vital roles in the mental health of people in crisis. Collaboration works best when the client's treatment goals are clearly communicated, the responsibilities of different crisis workers are known and respected, and an open dialogue in which all differing views of treatment are heard and considered. When crisis workers collaborate in a professional and respectful fashion, a broader scope of treatment that addresses multiple facets of a client's recovery can take place.

Collaboration with Other Professionals

More than any other mental health professionals, crisis workers tend to collaborate with other professionals in the community for the well-being of clients. For example, crisis workers may need to collaborate with religious and spiritual leaders, medical professionals, and law enforcement officers in crisis situations. Each of these professional groups have a different scope of practice that governs their actions with clients. For example, religious and spiritual leaders assist alignment with their religious and spiritual values, beliefs, and traditions. Traditional medical professionals tend to practice from a pure medical model and intervene with medical practices that may counter mental health practices. Law enforcement officers protect and safeguard the public by enforcing laws; their intervention may be quite different from crisis workers because it is based on training and protocols to protect the community and themselves.

Impact of COVID-19

The COVID-19 pandemic has forced changes in the field of crisis counseling and these changes will be integrated throughout the remaining chapters of this book. The transition

was quick to provide continued client care by medical and mental health professionals. While the transition was helpful for continuity of care, it also may have challenged certain ethical, legal, and best-practice safeguards in crisis situations. Historically, distance counseling was looked down upon; the pandemic forced mental health practitioners into providing some form of tele-mental health counseling when the world curtailed most human contacts. Society has now adapted telehealth as an alternative to face-to-face counseling. Although still not preferred by mental health professionals for many situations, including crisis assessments, the ability to receive some form of counseling via telehealth procedures is much better than the absence of counseling. However, in crisis situations, telehealth considerations should still be evaluated for effectiveness in high-risk situations, work with young children, stabilization of clients with serious mental illnesses, and in situations where individuals have limited privacy, such as clients at home who experience intimate partner violence.

APPROACHES TO WORKING WITH CLIENTS IN CRISIS

Learning Objective 1.4 Describe approaches crisis workers can take depending on the crisis affecting the client.

The procedures for working with clients in crisis begin with assessment of how best to approach such individuals to deescalate and stabilize the crisis. In other words, there is a need to assess whether the situation calls for counselors to be directive, nondirective, or collaborative.

Directive approaches call for counselors to "direct" or lead the person in crisis in a specific direction. Clients in crisis are typically distracted and less able to plan beyond their current situation. Therefore, providing some form of direction may help. For example, if someone is highly uncertain, spontaneous, or ambiguous and, at the same time, unable to get out of a crisis state, direction could provide immediate, though temporary, relief to feelings surrounding the crisis. As another example, a person who presents with highly suicidal behaviors requires the crisis counselor to initiate (directive approach) an emergency involuntary hospitalization for the safety of the client.

Nondirective approaches allow the person in crisis to come up with the directives while the crisis counselor facilitates that process. If the client can make rational decisions, even though in crisis, a nondirective approach may empower the client to make progress toward deescalation. For example, asking clients who are recent flood victims what might be of most help to them right now allows them to respond with specifics, rather than the counselor determining what is needed. The thought process and response of such clients may also empower them and allow them to feel they are regaining some control over their own lives.

A *collaborative approach* focuses on showing the person in crisis that the counselor is a participant in the journey toward stabilization and normalcy and that others can provide help, decrease isolation, and allocate resources to them. A collaborative approach is a blend of directive and nondirective approaches—but with a flavor of togetherness. In other words, a collaborative approach provides support and a sense of working together toward a common goal. Collaboration can be particularly beneficial to clients who feel

that they have too many choices, are aware of only options that have been ineffective in the past, or need some assistance in generating new options.

Complete Activity 1.2 to gain practice identifying these various approaches.

ACTIVITY 1.2

Examples of directive, nondirective, and collaborative approaches in crisis situations follow. Discuss how each could be an appropriate statement in specific circumstances.

EXAMPLE 1

- "Please put the gun down." (*directive*)
- "What would help you feel more comfortable about putting the gun down?" (*nondirective*)
- "I want to help, but knowing you have that gun in your hand scares me. Can you put the gun down for me so I can help you more?" (*collaborative*)

EXAMPLE 2

- "It sounds like you are thinking about killing yourself during most of the day and every day and have decided nothing else can take away your pain. You tell me that you will not follow through with whatever we plan and that you will kill yourself. I am concerned for your safety and want you to be here, alive. I really think the best option is for us to spend some time focusing on just you. I want to help you stay alive, and the only option I see to do that is for you to be admitted to the hospital." (*directive*)
- "You have generated some ideas on how you can stay safe, and they seem like good options." (*nondirective*)
- "I am happy to hear that you do not want to follow through on killing yourself. I also hear you saying that the pain at times is unbearable. You have been able to generate many options for staying alive. Can we continue to work together to see what resources are the best options and make a list so that when you start to have thoughts of wanting to kill yourself you can act on the best options first?" (*collaborative*)

EXAMPLE 3

- "I'm going to call the police." (*directive*)
- "Do you think calling the police will help?" (*nondirective*)
- "If I bring you the phone, would you call the police?" (*collaborative*)

CRISIS INTERVENTION THEORY

Learning Objective 1.5 Describe historical and contemporary crisis theories.

Let's now turn our attention to some historical and contemporary crisis theories. We start our discussion by examining the genesis of crisis intervention in the United States.

The Genesis of Crisis Intervention in the United States

The study of crisis intervention began in the 1940s in response to several stressor events. During World War II, numerous families experienced disorganization and changes in functioning after family members left home to participate in the war effort. In most cases, disorganization was temporary and families found ways to adjust. Families that had the most difficulty reorganizing and adapting to the absence of their loved ones seemed to experience the greatest degree of distress. Studies of families in crisis following war separation led Reuben Hill (1949) to propose a model through which family stress and crisis could be conceptualized by considering the family's resources, perception, and previous experience with crises.

Additional research on families and crisis events was launched following a more acute stressor event, the Cocoanut Grove nightclub fire that claimed nearly 500 lives in Boston, Massachusetts, in 1942. Studies of the responses of the survivors of the fire, family members of those who died, and the community illuminated some common reactions to such a traumatic event and led Gerald Caplan and Erich Lindemann to propose recommendations for responding to community crises. In the decades following the 1940s, the original models proposed by Hill, Caplan, and Lindemann were expanded, with more attention to contextual variables and outcomes.

Caplan and Lindemann are often credited as pioneers in the field of crisis intervention. Lindemann, a professor of psychiatry at Harvard Medical School and Massachusetts General Hospital, worked with patients dealing with grief following traumatic loss. Although many people died because of the Cocoanut Grove fire, hundreds who were at the nightclub on that fateful night survived. The survivors and the grieving relatives of those who perished provided Lindemann with an opportunity to study psychological and emotional reactions to disaster. Based on interviews with those who survived the fire as well as relatives of the deceased, Lindemann (1944) outlined several common clinical features, including somatic distress, feelings of guilt, hostility, disorganization, behavioral changes, and preoccupation with images of the deceased. Lindemann referred to these symptoms as "acute grief," which was not a psychiatric diagnosis but was a call for intervention, nonetheless. Today, many of the symptoms of acute stress disorder identified in the DSM-5-TR seem to parallel Lindemann's description of acute grief.

In addition to describing clinical features of acute grief, Lindemann outlined intervention strategies for dealing with the symptoms. Because acute grief was not considered a psychiatric diagnosis per se, Lindemann suggested that helpers other than psychiatrists could be of assistance. This idea was further fueled by the large number of people in need of intervention following the Cocoanut Grove fire and became a cornerstone in the conceptualization of community mental health.

In response to the needs of the number of people experiencing acute grief following the nightclub fire, Lindemann worked with his colleague Gerald Caplan to establish a community-wide mental health program in Cambridge, Massachusetts, known as the

Wellesley Project. By studying and working with individuals who had experienced loss through the fire or similar traumatic events, Caplan (1964) developed the concept of "preventive psychiatry," which proposed that early intervention following a disaster or traumatic event can promote growth and well-being. Lindemann's "basic crisis theory" focused on crisis intervention as a distinct area in the helping field requiring specific interventions. His theory allowed others to build upon and develop alternative perspectives. Crisis intervention is focused and brief but is not the same as brief therapy. Brief therapy attempts to decrease symptoms associated with ongoing mental health problems. Crisis intervention attempts to assist individuals who are experiencing temporary affective, behavioral, and cognitive symptoms associated with a crisis event.

Building on Caplan's model, Beverley Raphael (2000) coined the term "psychological first aid." Following a train accident in Australia in 1977 in which many people died, Raphael worked with bereaved families and injured survivors of the train disaster. She advocated for attention that included comfort and consolation, immediate physical assistance, reunification with loved ones, an opportunity to express feelings, and support during the initial period following a traumatic event. In particular, Raphael described the need to consider Maslow's hierarchy of needs and the importance of attending first to basic survival needs before attempting more traditional forms of counseling. In describing crisis reactions, Raphael noted that the full impact of trauma may be experienced a considerable time following the initial crisis event, thereby leading an individual to undergo a period of "disillusionment." Much of her work after the 1977 train accident focused on the prevention of PTSD. In a classic work, Caplan (1964) offered this explanation:

> People are in a state of crisis when they face an obstacle to important life goals—an obstacle that is, for a time, insurmountable by the use of customary methods of problem-solving. A period of disorganization ensues, a period of upset, during which many abortive attempts at solution are made. (p. 18)

What is important to note in Caplan's description is that the concept of crisis refers to an outcome of a precipitating event, not to the precipitating event itself. Like more recent definitions of *crisis*, Caplan described the outcome, or the crisis, as the state of disequilibrium that is experienced.

There are several schools of thought pertaining to crisis theory, and the concept of disequilibrium following a stressful event seems to be common to all of them. Some theories focus on the disequilibrium experiences of individuals, whereas others take a more contextual, systemic stance. Many researchers and theorists take the point of view that the disequilibrium that constitutes a crisis can be understood by examining an individual's past experiences, cognitive structures, behaviors, and competencies. In their studies on hysteria, Freud and Breuer (2004) concluded that many of the neurotic symptoms that contributed to clients' states of equilibrium were repressed memories of past traumatic events. Their disequilibrium was sustained by destructive defense mechanisms.

Other crisis theorists contend that the disequilibrium is the result of ineffective psychological tools, such as negative or faulty thinking, poor self-esteem, and maladaptive behavior. According to "adaptational" theory (James & Gilliland, 2017),

individuals who become incapacitated or dysfunctional following stressful events are those who perceive and interpret stressful events negatively. Negative thoughts, irrational beliefs, and defeating self-talk led to paralyzing rather than helpful behaviors. From a cognitive-behavioral perspective, crises may be ameliorated when individuals replace their faulty thinking and ineffective behaviors with more positive thoughts and adaptive behaviors. From an interpersonal perspective, self-confidence and self-esteem also counter the disequilibrium of crises as individuals become more focused and reliant on their own abilities.

Applied Crisis Theory

Applied crisis theory comprises four crisis domains: developmental, situational, existential, and ecosystemic (James & Gilliland, 2017):

- *Developmental crises* are events that are expected to be experienced by most individuals during normal development. However, for some, the developmental crisis event could cause considerable trauma. Examples of developmental crises include pregnancy, graduation, retirement, career transition, and aging.
- *Situational crises* are events that are often unexpected and involve some degree of a catastrophic, shocking, or random act. Examples of situational crises include a terrorist attack, a sexual assault, job loss, an accident, and a sudden illness.
- *Existential crises* may or may not involve religious faith or spirituality but are events that affect one's sense of meaning, purpose, freedom, independence, forgiveness, shame, or other core beliefs.
- *Ecosystemic crises*, which include natural or human-caused events, affect not only the individual but also any of the systems (e.g., families, schools, communities) connected with the crisis event.

It is important for the crisis counselor to understand the individual's perception of a crisis before categorizing the crisis into one or more of the four crisis domains. The reason is that the crisis domain acts as a framework for understanding the individual's perception of the event and its life effects. For example, consider the case of a woman who enters a crisis center pregnant. Initial observation may lead a crisis counselor to assume that the woman is experiencing a developmental crisis. However, upon learning that the pregnancy is the result of a rape, the crisis counselor may categorize the pregnancy as a situational crisis. It is also possible that the woman is thinking of terminating the pregnancy, which may constitute an existential crisis because following through would conflict with religious beliefs. As this example indicates, a crisis must be understood by how it is perceived by the individual affected by it.

Regardless of the theory adopted, crisis intervention requires that a crisis counselor undertake a series of tasks to understand the client's circumstances, ameliorate the most harmful aspects of the crisis, and help return the client to precrisis or baseline functioning as safely, quickly, and efficiently as possible. Theories are assumptions about *why* an individual may experience a crisis and perceive it as traumatic. Theories provide a rationale or explanation that informs a crisis counselor's guiding practice, or *how* a crisis counselor works through the crisis with a client. This guiding practice is one's model of practicing in crisis intervention.

THE TASK MODEL OF CRISIS ASSESSMENT AND INTERVENTION

Learning Objective 1.6 Apply the task model of crisis assessment and intervention to crisis situations.

The *task model of crisis assessment and intervention* developed by Lisa Jackson-Cherry (2018; see Table 1.1) focuses on four important tasks of crisis assessment that lead to an effective intervention plan for individuals in virtually every type of crisis, regardless of setting. Addressing safety, stabilization, and risk; following a holistic biopsychosocial-spiritual approach; clarifying the problem(s); and exploring coping skills, resources, and supports are essential crisis assessment tasks. Only when a thorough assessment is completed can an effective crisis plan be developed to address the actual problem(s). In traditional counseling, the definition of the problem may be very clear, and the plan may follow a prescribed sequence. However, due to the complex nature of individuals in crisis, a consistent and useful definition of the problem can only be clarified after appropriate and comprehensive assessment tasks are completed and followed by an appropriate crisis plan.

Four Essential Crisis Assessment Tasks

Let's examine the four essential crisis assessment tasks in more detail: address safety, stabilization, and risk; follow a holistic biopsychosocial-spiritual approach; clarify the problem(s); and explore coping skills, resources, and supports.

ASSESSMENT TASK 1: ADDRESS SAFETY, STABILIZATION, AND RISK A crisis counselor should always maintain a sense of safety for self and others (see Chapter 2). Safety should be the main priority for the client, community, and counselor. Therefore, a thorough risk assessment should be the first step in the assessment and the main goal for the counselor. Due to the nature of a crisis, risk should be assessed at varying times throughout crisis intervention. A client may be referred (or self-referred) for a perceived presenting problem that is not the primary problem causing the difficulties, and the client could present with varying degrees of lethality (i.e., none, low, moderate, or high) and types of risk (i.e., homicidal, suicidal). Lethality must always be assessed at the beginning of the

TABLE 1.1 The Task Model of Crisis Assessment and Intervention (Jackson-Cherry, 2018)

Level I: Four Essential Crisis Assessment Tasks
Assessment Task 1: Address Safety, Stabilization, and Risk
Assessment Task 2: Follow a Holistic Biopsychosocial-Spiritual Approach
Assessment Task 3: Clarify the Problem(s)
Assessment Task 4: Explore Coping Skills, Resources, and Supports
Level II: Four Essential Crisis Intervention Tasks
Intervention Task 1: Normalize and Educate
Intervention Task 2: Explore Options
Intervention Task 3: Develop a Plan and Obtain a Commitment
Intervention Task 4: Prepare Documentation, Follow Up, and Provide Referrals

relationship. If any degree of lethality is present, a comprehensive plan must be discussed with the client and documented. Not assessing for risk in a crisis situation, or waiting until the end of a session to conduct such an assessment that is not thorough, is negligent. If a person were to leave a session and follow through with suicidal or homicidal ideation or behavior that was not thoroughly assessed, the standard of care could be questioned. Adhering to the standard of care is a primary focus of Assessment Task 1 and, if lethality is present, it should continue to be a focus in subsequent steps.

Often a person who is at risk may not initially report ideation or intentions of suicidal or homicidal behavior. Clients need to know that the crisis counselor is willing to explore the delicate issues that are leading to thoughts or behaviors of suicide or homicide, can provide an emotionally safe environment, and is competent to assist with an appropriate plan. Allowing for an open and thorough assessment helps clients understand that the counselor is fully present, cares, and is competent to assist with lethal thoughts. An assessment of safety and lethality actually enhances the rapport between the client and counselor. Importantly, if a client is experiencing intense crisis circumstances that affect the client's baseline functioning, then it is unlikely that the counselor and client can collaboratively progress through the essential intervention tasks that follow.

A client also must be stabilized enough to progress through the intervention tasks of this model and collaborate in the intervention plan. Thus, client stabilization is a focus before moving forward with additional tasks. If the client is unable to work collaboratively toward the additional tasks and unable to implement lifesaving plans, the main goal for the counselor should be for the immediate safety of the client or others and options for intensive care, including hospitalization, should be pursued.

ASSESSMENT TASK 2: FOLLOW A HOLISTIC BIOPSYCHOSOCIAL-SPIRITUAL APPROACH

Next, crisis counselors should thoroughly assess a client for physical, medical, substance, psychological, social, spiritual, and other concerns. Often, individuals experience a crisis because of medical issues such as pain associated with a physical condition or noncompliance with medical practices (including medication noncompliance), religious/spiritual and/or other existential conflicts, or other reasons. These medical issues often present with symptoms consistent with mental health issues during a crisis. If medical and other issues the client faces are not assessed, the diagnosis of the client may be incorrect, leading to an ineffective and inappropriate treatment plan or referral. Without this kind of assessment, the root problem will never be addressed and symptoms may increase over time, making the situation worse for the client. A comprehensive assessment of all these considerations must be conducted to determine the cause of or connection to the perceived or actual crisis event. A medical assessment may include questions such as the following:

- When was your last medical examination?
- Have you presented to the emergency department or another physician with an acute or chronic medical complaint? How often? When was the last visit?
- Have you been diagnosed with any medical condition(s)? Have you been prescribed any medications for the medical condition(s)? Have you been compliant with the recommended medication regimen? If not, what has prevented you from continuing to take the medications as prescribed? Has the medication affected your thoughts, mood, or behavior? What was your life like prior to the medical condition(s)?

- Have you had medical complaints that are not being addressed by a medical doctor?
- Have you had any recent injuries? When was the injury? Did you receive medical attention? What was the treatment plan set by the physician? Have you been compliant with the treatment plan? Has there been a follow-up? Have you felt your mood change since the injury? If so, in what ways? What was your life like prior to the injury?

It is essential to assess the baseline functioning of the client during the present crisis situation and prior to the crisis. Later, the goal may be to formulate a plan that moves the person back to precrisis functioning or to develop a new baseline.

Most crisis models follow an ABC (*a*ffect, *b*ehaviors, *c*ognitions) assessment approach. However, this task model follows an ABCDE assessment approach (*a*ffect, *b*ehaviors, *c*ognitions, *d*evelopment, and *e*nvironment). Keep in mind that while some assessment measures may fall into more than one category, all impact the overall assessment.

Affect consists of how a person presents during the assessment and may include nonverbal communication. Is the client withdrawn, distant, restrictive in speech or space, agitated, incongruent in presentation, and so on?

Behaviors are assessed to determine a person's response and potential to move through the crisis. For example, since the precipitating crisis, what has the client done (or not done) in response to the crisis event? What resources has the person accessed or not accessed? How have the person's eating habits, sleep, work, and normal activities and behaviors changed from before the crisis? What is preventing the person from moving toward the behaviors that were evident in precrisis daily living?

Cognitions, or thoughts, include thinking patterns, distortions, and deficiencies. In most situations, cognitions affect behaviors and how a person presents emotionally (i.e., affect). Are cognitive distortions present that are preventing the person from moving personal behaviors toward the precrisis or new baseline functioning? Are cognitions restrictive? For example, does the client believe no actions will improve the situation? For risk assessment, does a person believe that there is only one way to "fix" the problem (e.g., suicide, homicide), and is the person not willing to evaluate other options in the development of a plan?

In addition to the ABC assessment commonly used as the main criteria in the field, crisis counselors should understand the appropriate development and environment factors that may affect a person's response to a crisis. A person's response and functioning should be assessed based on normal levels of *development*, and plans should be made based on the developmental functioning level of a client. How does a client's current functioning abilities affect how the client perceives the crisis? Are there limitations in the cognitive processing domain (e.g., how clients frame and perceive problems)? How do physical or emotional limitations affect the client's ability to use resources or follow through with a plan? For example, a young adult with autism spectrum disorder may need a plan established with goals appropriate for functional level and ability to process the situation, which may be far different from a person not diagnosed with autism spectrum disorder. A person with a lower intellectual ability attempting to process the impact of the death of a parent will need to be given

information in a very different way, with more concrete and simplified information than a person of higher intellectual ability.

The *environment*, which includes common multicultural components (e.g., socioeconomic status [SES], religious or spiritual influences, racial-ethnic identification with groups, gender, resilience, physical abilities, sexual or affectional orientation, educational level), must also be assessed and considered. In addition, personality characteristics formed by these components may promote or hinder moving past the crisis. For example, if a client is experiencing a spiritual crisis and, in the past, relied on a religious or spiritual leader to cope, a referral to the person's religious or spiritual leader may be crucial for intervention. However, this client, feeling guilty and intimidated by the perceptions and judgment of others, may not act (behavior) on this past coping skill in contacting a spiritual leader if the person believes (irrational cognitions) that a higher power will never forgive the action that is causing such emotional and spiritual pain.

A thorough ABCDE assessment allows the counselor to obtain information that is useful in developing a plan to get the person back to precrisis baseline functioning or to develop a plan to create a more stable new baseline.

ASSESSMENT TASK 3: CLARIFY THE PROBLEM(S) Although a client may present initially with a particular problem (which should be taken into consideration), the actual problem may not be known until a complete history is taken. For example, a person may have lost a life partner 5 years ago, but the death of the partner's pet may be the actual crisis due to the meaning placed both on the pet and the significant-other relationship. Counselors should be aware that a client may be facing multiple crises since an event may affect many parts of the client's life and cause multiple traumas or retraumatization. Counselors should also realize, when working with several individuals in the same family, that each person may have a different presenting problem.

During this task, active listening, offering unconditional positive regard, and being nonjudgmental are essential. Likewise, empathically phrased yet challenging questions may be required to understand the actual crisis so that an effective action plan can be developed. These steps are important for the client to gain control over the problem and begin the healing process. Providing a client with the space to share personal stories in their own words will allow the client to gain some control over the crisis. Identifying any attached meaning the client has to the perceived crisis can lead to an in-depth understanding of the actual crisis and a more effective intervention plan.

ASSESSMENT TASK 4: EXPLORE COPING SKILLS, RESOURCES, AND SUPPORTS After clarification of the problem(s), crisis counselors explore the coping skills, resources, and supports that were available to the client when dealing with past crises and adjustments. Crisis counselors also explore which coping skills, resources, and supports were effective and ineffective, as well as which are available and unavailable for the current crisis. The client and counselor also discuss potential new resources, coping skills, and supports that the client may not be aware of or may not have used in the past. Counselors assess these resources using a variety of research-based approaches, including resilience-based, positive psychology, hope-based, and wellness models. The motivation of the client is also assessed.

Four Essential Crisis Intervention Tasks

Let's now examine the four essential crisis intervention tasks in more detail: normalize and educate; explore options; develop a plan and obtain a commitment; and prepare documentation, follow up, and provide referrals.

INTERVENTION TASK 1: NORMALIZE AND EDUCATE In most situations, a vital aspect of crisis intervention is to normalize a person's response to the crisis. Unless the person has acted out of malicious intent to harm another person, a person in crisis may behave in a variety of ways. It is always easy to look back and think that one should have acted differently, but in a crisis, most people try to make the best decision during a situation that is often out of their control at the time. Often, clients experience normal reactions (e.g., fear, abandonment, hopelessness, loss of control) to an abnormal event. Because clients in crisis are feeling and thinking that things are out of control, they may be unable to control their feelings and thoughts as they typically do in other situations or do not like the feelings and thoughts they are experiencing. Clients can become overwhelmed, and the crisis can be even more debilitating due to these new feelings and thoughts. Counselors should understand the normal responses to crisis events and teach clients that many of their feelings and thoughts are normal. This normalizing can remove much of the power the client perceives the crisis to have.

INTERVENTION TASK 2: EXPLORE OPTIONS Using the information collected during assessment tasks 1–4, the crisis counselor and client should collaboratively explore and evaluate all the options known to the client, as well as those developed by the counselor. Even when there is evidence of lethality to self or others, the intent is to work collaboratively in developing a plan for the safety of the person or others. In some situations, when the client is not able to commit to any plan involving safety, the counselor may have to move into a directive approach. The safety of the crisis counselor is as important as the safety of the client in crisis; therefore, if there is any sense of potential harm by the client, the crisis counselor should make personal safety a priority. For example, if the crisis counselor believes the client should be hospitalized due to high suicide lethality, but through the discussion of this option the client becomes agitated and there is a fear of personal safety, then collaborating is no longer the mode of interaction and the counselor arranges to have the client hospitalized.

INTERVENTION TASK 3: DEVELOP A PLAN AND OBTAIN A COMMITMENT The resulting intervention plan is individualized and based on the comprehensive assessment of the client. The counselor must consider the risk factors, biopsychosocial-spiritual elements; medical information; and coping skills, supports, and resources. The plan should be documented as part of the discharge summary when referring the client to another mental health professional and used as a plan for follow-up. Client commitment is essential for ownership and responsibility. Plans should have attainable and measurable goals to demonstrate progress.

INTERVENTION TASK 4: PREPARE DOCUMENTATION, FOLLOW UP, AND PROVIDE REFERRALS In a typical crisis, a client may be referred to another mental health professional in the community for continued or long-term care to meet client needs as specified in the intervention plan. In this case, proper documentation should be completed to terminate the case with detailed information on the referral and the intervention plan. If the crisis occurred during a long-term case, documentation and follow-up must be made in a manner outlined in the plan.

See Case Studies 1.2 and 1.3, which incorporate the task model of crisis assessment and intervention. Also read Voices from the Field 1.3.

CASE STUDY 1.2

Integration, Demonstration, and Application of the Task Model of Crisis Assessment and Intervention: Ellery

Level I: Four Essential Crisis Assessment Tasks

Assessment Task 1: Address Safety, Stabilization, and Risk

Ellery presented with suicidal ideation over the breakup of an intimate, long-term relationship. After initial screening questions for suicide, homicide, and nonsuicidal self-injury (NSSI), Ellery is not experiencing homicidal ideation or NSSI. Ellery does admit to thoughts of self-harm, saying "I just want this horrible pain to end." The standard of care mandate here is to assess for risk factors and planning as well as to discern the risk of lethality, so a thorough assessment is needed; client stabilization is the primary focus before moving forward with additional tasks. The initial severity appears moderate, so the immediate safety of the client or others is the priority and options for more intensive care, including hospitalization, should be considered.

Assessment Task 2: Follow a Holistic Biopsychosocial-Spiritual Approach

The counselor undertook a holistic biopsychosocial-spiritual assessment with the following items of note. Regarding physical health, Ellery made no complaints and presented as a youthful and healthy person. Ellery's last general medical examination was 5 years earlier. Ellery has never presented to the emergency department or physician with an acute or chronic medical condition. Ellery reports only mild seasonal allergies, takes no medications, and reported no recent injuries or other medical treatments. Ellery overuses alcohol and marijuana, especially since the breakup. Ellery reports a family history of depression and periodic feelings of sadness. Socially, Ellery reports having few friends.

Ellery presented with some other affective, behavioral, cognitive, development, and environment concerns. Ellery's *affect* was depressed and introspective ("How could I have screwed up so bad?"). Ellery complained of feeling sad, depressed, hopeless, fatigued, and helpless to change the situation. Since the precipitating crisis and in response to the crisis event, Ellery reports increased *behaviors* of moping, drinking, smoking marijuana, and isolation. Ellery claims to have few resources and few close friends. Ellery sometimes will drink with coworkers after work but does not discuss personal issues with them. Ellery reports insomnia, no appetite, poor performance at work, and absenteeism due to excessive drinking since the breakup. Regarding *cognitions*, Ellery does not understand why the partner left and has stopped taking Ellery's calls or responding to texts. When asked about

ideas to "fix" the problem, Ellery just wants help for the pain to end and has been thinking about suicide. *Developmentally*, some physical or emotional limitations may affect Ellery's ability to use resources or follow through with a plan. For example, Ellery feels their current job is a dead-end, money and promotion wise, and has little motivation to change course socially or interpersonally. *Environment*, which includes common multicultural components (e.g., SES, religious or spiritual influences, racial-ethnic identification with groups, gender, resilience, physical abilities, sexual or affectional orientation, educational level), was also assessed and considered. Ellery is not currently spiritual and has no religious leader but feels comfortable contacting one. Ellery repeated that suicide is a sin and people who die from suicide will "burn in hell."

Other data were collected. Ellery reported the following responses to the Columbia Suicide Severity Rating Scale – Screen Version (C-SSR; *https://www.cms.gov/files/document/cssrs-screen-version-instrument.pdf*): Q1 (wished dead?) – Yes; Q2 (thoughts of killing self?) – Yes; Q3 (thinking how? No developed plan) – Yes; Q4 (intention? Minor.) – Yes; Q5 (plan?) – No; and Q6 (initiated plan?) – No. Therefore, Ellery was determined using this screener to be Moderate to High Risk.

Throughout the interview, Ellery divulged the following specific risk factors:

- **Demographics:** unwillingly separated from partner.
- **Psychosocial:** perceived lack of social support.
- **Psychiatric diagnosis:** probable mood disorder and substance use disorders.
- **Psychological dimensions:** feels five key suicidal emotions: hopelessness, helplessness, worthlessness, loneliness, and depression.
- **History:** family history of depression (client has made no previous suicide attempts and has no history of suicidal ideation).
- **Individual factors:** Current alcohol and drug use (no past suicidal ideation, plans, or intent; current ideation, with minor plan and minor intent but no suicidal behaviors); reports little reason for living (no homicidal ideation).

Ellery's responses to a Q-tree for suicide follow:

- *Do you have a plan for how you would attempt suicide?*
  Ellery: Not really. I haven't really planned for anything.
  - *(If yes:) What is the plan? Have you acted on the plan (rehearsed it, or made plans to follow through)? Have you used this plan in the past?*
- *Do you have a way (means) to carry out the plan?*
  Ellery: Not really. I don't own a gun or anything like that.
  - *(If yes:) Have you attempted or started to act on the plan? If that plan did not result in death, is there another plan and means? Have you used this plan and means in the past?*
- *Have you had any past suicidal attempts?*
  Ellery: No. None.
  - *(If yes:) How many? How has the plan and means increased in lethality and frequency? When? What was an underlying issue for the attempt?*
  - *What was the result of the attempt? Hospitalization, counseling, no intervention? Did others know of the attempt and what was their support? Who was part of the rescue?*
  - *If the result was hospitalization, what happened at discharge? Was there a diagnosis? What was the diagnosis? Was medication prescribed? If medication was prescribed, are you compliant? Was mental health counseling provided after discharge? Did you follow through with the outpatient mental health referral?*

- *Do you have a mental health diagnosis (whether you agree or disagree)?*

  Ellery: No. There is a history of depression in my family, but I have not been diagnosed with depression, although I feel really depressed right now. More than I have in the past.
  - *Are you under the care of a mental health provider? Are you still seeing a counselor? Were you prescribed medication? What was it? Are you compliant? If not, when was the medication stopped and why?*

  Ellery: No counselor. No medication.
- *Do you have a family member with a history of a substance use disorder or mood disorder (e.g., major depressive disorder, bipolar disorder, or other mood disorder)?*

  Ellery: Yes. My mother and my sister are both depressed. We all drink too much but I don't know if anyone has a substance use disorder diagnosis. They don't really talk about it.
- *Do you have a family history of suicide? (Family means any perception of family and does not have to be related by blood.) For children and teens, has a friend attempted suicide or died by suicide?*

  Ellery: No. No one of whom I am aware.
- *Has an issue occurred that has not been resolved?*

  Ellery: My breakup!
  - *Has the event caused distress that has impacted your daily functioning? Was the event humiliating? Has there been a recent change in life because of the issue (moving, separation from children/family)? Is there a sense of shame connected with the unresolved issue?*

  Ellery: Absolutely! I am gutted. In real agony. I don't know what to do. How to get them back. I planned to live with them the rest of my life!
  - *In past situations, what have you done to get past the issue?*

  Ellery: This is the first time it has happened. I don't know what to do.
- *Do you have a social support system?*

  Ellery: My mom and sister; my father abandoned us when I was a teenager, and we haven't spoken since. Work friends. Friends I shared with my partner, but I don't think they want anything to do with me anymore.
  - *Has the support changed? Is the support system reliable? Are there other new supports or supports you can think of that would be helpful?*

  Ellery: My friends either don't know or have not reached out to me. I am so embarrassed and don't want to talk with them about it. They probably would take my ex's side anyway.
- *Do you have hope that things will get better?*

  Ellery: No. Everything is so messed up. I am in such emotional pain. I can't see things ever being good again.
  - *How will things get better?*
  - *What has prevented you from following through on your suicidal thoughts?*

  Ellery: My dwindling hope of reconciliation and my family's religious beliefs.
- *Do you want to die?*

  Ellery: Not really, but the pain of living without my partner is overwhelming.
- *Can you commit to working on a plan to keep you alive?*

  Ellery: Yes. I definitely don't want to go to hell!

Given all of this information and evidence, determine Ellery's lethality rating in the context of Low to High, where:

Low: The client is not suicidal at the time of the assessment.

Low-Moderate: The client is somewhat suicidal but does not have any risk factors associated with suicide.

Moderate: The client is suicidal and has several risk factors.

Moderate-High: The client is determined to die and may commit suicide within the next 72 hours unless intervention occurs.

High: The client is currently in the process of committing suicide (e.g., already swallowed pills).

Given all of the assessment data collected above, Ellery's lethality rating appears to be moderate. At this time, Ellery is periodically considering suicide, has several risk factors, but has not formulated a plan or displayed intent of imminent action.

Assessment Task 3: Clarify the Problem(s)

Although Ellery presented initially with a particular problem (the breakup), the comprehensive history has helped enrich our understanding of the actual problem and context. This was accomplished through active listening, unconditional positive regard, and being nonjudgmental. The interview unearthed a previous issue of abandonment (father abandoning the family) that was explored in more detail, and Ellery admitted to thinking about that abandonment a lot more recently and wondering what personal failings have led to these broken relationships.

The counselor used empathically phrased yet mildly challenging questions to help the client better understand the actual crisis and to help Ellery gain more control over the problem to begin the healing process. This included identifying attached meanings Ellery had to the perceived crisis, including not really knowing what friends knew about the breakup and whether continuing supportive friendships were possible. The counselor explored this and other relationships (family, friends, work colleagues, potential spiritual leaders) to develop a better understanding of the actual crisis and allow for more effective intervention planning by tapping into potential emotional supports.

Assessment Task 4: Explore Coping Skills, Resources, and Supports

After clarification of the problem, it is essential for counselors to explore the coping skills, resources, and supports available to Ellery, especially when dealing with past crises and adjustments. Counselors should also explore and evaluate which coping skills, resources, and supports were effective and ineffective, as well as which are available and unavailable for the current crisis. The client and counselor also should discuss potential new resources, coping skills, and supports of which Ellery may currently be unaware of or may not have used in the past. These resources are explored using a variety of research-based approaches, including resilience-based, positive psychology, hope-based, and wellness models. Ellery's motivation to change was also assessed. At this time, Ellery claims some motivation to change but is in such emotional pain and low mood that a path forward has not been detected.

The counselor administered the Brief Resilience Scale (BRS; *https://ogg.osu.edu/media/documents/MB%20Stream/Brief%20Resilience%20Scale.pdf*) to assess Ellery's

resilience under normal circumstances (Ellery scored in the normal range of resilience) and the 5F-Wel (*https://www.mindgarden.com/99-five-factor-wellness-inventory*) to better understand Ellery's areas of personal strengths and challenges so as to better prepare a strengths-based treatment plan. From this assessment, family and friendship resources were discussed and plans were made to begin more healthful exercise (e.g., cardiovascular, strength training, yoga) and nutrition regimens. Plans were also discussed to reconnect Ellery with a previous spiritual guide.

Level II: Four Essential Crisis Intervention Tasks

Intervention Task 1: Normalize and Educate

The purpose of intervention is to normalize a person's response to the crisis. People in crisis may behave in a variety of ways. In hindsight, most clients reflect that they could have acted differently, but while in a crisis, people make the best decisions they can at the time in a situation that seems beyond their control. Ellery experienced normal reactions (i.e., abandonment, hopelessness) to an unexpected event (i.e., the breakup) that left them feeling out of control. The counselor helped Ellery explore emotions and the feelings of being out of control. This was supplemented by psychoeducation on processes of grief and loss to help Ellery understand that stages of grief are predictable and survivable. The goal was to help Ellery gain more control over feelings and thoughts, even though the feelings and thoughts experienced are uncomfortable and, at times, overwhelming. Ellery was helped to realize that these new feelings and thoughts can make the situation even more debilitating. The normalization of feelings and thoughts helps clients remove much of the power the client perceives the crisis to have.

Intervention Task 2: Explore Options

Ellery and the counselor used information gleaned from assessment tasks 1–4 to collaborate, develop, and explore options. The moderate lethality estimation (i.e., no plan or means, low to moderate intent) means that hospitalization is probably not necessary at this time but should be discussed as a continuum of care option. It is often helpful to accelerate outpatient care to make sure the client is safe, so they considered twice-a-week sessions, daily call-ins, and other options. Use of medication was discussed. They also considered who could be contacted or visited when Ellery felt depressed and needed to talk. Ellery suggested phone calls with their mother or sister, and even suggested talking to a spiritual leader and perhaps returning to more regular attendance. Suggestions from the 5F-Wel and BRS were also considered. Ellery and the counselor then discussed motivation and willingness to change and implement the strategies agreed to. By this time, while still depressed and heartbroken, Ellery was beginning to feel some hope that things might eventually get better and understand the importance of personal safety and self-care. Of course, if there is any sense of potential harm by the client, the crisis counselor should make personal safety a priority.

Intervention Task 3: Develop a Plan and Obtain a Commitment

Ellery and the counselor decided to schedule two 1-hour sessions per week for the next 2 weeks. Ellery will also call the counselor for a daily check-in at 5:00 p.m. for a brief confirmation that no or minimal suicidal ideation occurred. Ellery will contact family to arrange two 10-minute phone calls per week to share life events and seek their support. Ellery also decided to contact a spiritual leader to meet and discuss personal spiritual

issues and strengthen the expressed commitment to attend religious events again. Notice that the resulting intervention plan is individualized and based on the comprehensive assessment of the client. Client commitment is essential for ownership and responsibility. Plans should have attainable and measurable goals to demonstrate progress.

Intervention Task 4: Prepare Documentation, Follow Up, and Provide Referrals

The counselor kept excellent records for continuity of care in this outpatient context and in case future hospitalization or referral was necessary.

Discussion Questions

1. What other environment variables could have been explored to develop a holistic plan of action for Ellery?
2. What educational information could also be provided to Ellery and Ellery's support network?
3. What other referrals could be made to assist Ellery?
4. What assumptions did you make regarding Ellery's self-identifications?

CASE STUDY 1.3

Integration of the Task Model of Crisis Assessment and Intervention: London

Marty, a clinical mental health counselor in private practice, frequently receives referrals for at-risk students from schools and the community. The parents of 4-year-old London contacted the practice because London is at risk of being expelled from a third preschool for severe behavioral issues, including biting and hitting other children.

Before meeting London in the office, Marty conducted an observation at the preschool. There, Marty witnessed London punch another child in the face over a fire truck. In response, the teacher took the fire truck from London and comforted the child who London punched. Throughout the preschool observation, London does not appear to engage in any developmentally appropriate interaction with other children. At lunch, London sits with the other children and does not interact except to try to take food from a few of them using threatening gestures. In response, the other children give London their food. At the end of this observation, it seems clear that London demonstrates several defiant behaviors and appears to show no remorse for these behaviors.

A week later, London and London's parents arrive for their first in-office visit with Marty. After meeting with the parents and London together and then spending some time with London alone, Marty decides to have the parents join them again. When leaving to get the parents, Marty stops behind London as London colors and tells London that the parents will join them again. London does not respond, nor does London display any body movements in reaction to Marty's voice. Curious, Marty gives several other commands that are ignored by London, who simply continues to color. Marty keeps these observations in mind while proceeding with the assessment.

Assessment Task 1: Address Safety, Stabilization, and Risk

Because Marty often works with at-risk clients, Marty knows the need to assess and be mindful that any person, no matter the age, could exert harm to self or others. Children,

depending on their ability to express appropriate emotions and control their actions, could become violent (and Marty already witnessed London engage in violent behavior during the observation). With safety for self and others in mind, Marty starts an in-depth assessment of London. London's parents provide most of the information because London appears to be content coloring and does not respond to basic initial questions. Marty asks London's parents: Has London demonstrated threatening behaviors or actions to family members, friends, animals, or toys? In what environment does London show more anger or violence? Does London behave violently toward self (e.g., scratching, biting, cutting, hitting their head or other body parts against a wall or object)? and Has London made any verbal threats to others? The task of addressing safety, stabilization, and risk would continue during other assessment tasks.

Assessment Task 2: Follow a Holistic Biopsychosocial-Spiritual Assessment Approach

Marty continues the assessment to obtain a more holistic understanding of London, the family, and their social life. Marty asks London's parents: Has London witnessed any violence on television or with family members? Does London play with friends or other family members about the same age? How would you describe London's play? How would you describe your parenting style (e.g., how do you provide encouragement, reinforcement, rewards, consequences, and punishment)? What seems to calm London when frustrated? What activities does London enjoy? Did London reach developmental milestones on time? When did London learn to walk and talk? Does London have any chronic or acute medical conditions (e.g., ear infections, urinary infections)? Is London on any medications? Has there been a history of domestic violence or sexual violence in the family or with London?

Marty asks about London's most recent well visit with a pediatrician. London's parents indicated that, aside from vaccinations, London has not been seen by a doctor regularly, with the exception of the local urgent care center, where London's chronic ear infections are treated. Marty asks the parents whether they followed up after the ear infections. They say they assumed that the medicine healed the ear infections.

Noticing that London still does not respond, Marty initiates a conversation with London and begins to color with London. London grabs the crayons and throws them across the room and then goes to stand in the corner. London's parents indicate that this behavior is normal for London, in that when London becomes violent, standing in the corner is the punishment. Marty asks if there was ever any discussion between the parents and London when this happened. They say, "No, London learned this in preschool." Marty quietly walks behind London and snaps. No response. Marty asks London to turn around. No response.

At this point, Marty is highly suspicious that something more is happening with London than solely behavioral issues (the original presenting problem). Marty moves in front of London and motions for London to come and sit down. London proceeds to sit back on the floor. Marty (while covering their own mouth) starts talking to London. London reaches up and tries to lower Marty's hand to stare at Marty's lips. Marty then says, "Do you want to color or use the blocks?" London nods and says, "Color."

During this assessment, Marty noted the ABCDE observations. London's *affect* was observed when receiving directives (e.g., behaviors ranged from being withdrawn to being interested in activities to anger). London displayed a range of *behaviors* when frustrated (e.g., throwing the crayons and putting self in a time-out). London's *cognitive*

functioning appeared to be intact when London could focus on Marty's face. Marty inquired about *development* milestones such as talking, walking, and play activities to compare London's development to typical developmental functioning, although it seemed speech development was limited compared with other 4-year-old children. Finally, Marty assessed for *environment* factors such as peer interactions, SES, medical illnesses, living arrangements, social interactions, exposure to violence, and parenting styles.

Assessment Task 3: Clarify the Problem(s)

Marty shared these observations with London's parents and stated that another problem may need to be evaluated prior to addressing the behavioral issues. While London's aggressive behaviors need attention, Marty wants to rule out any medical issues. Marty wants London's parents to make an appointment for London to see a pediatrician and have a hearing test conducted to rule out any hearing issues before addressing the behavioral issues.

Assessment Task 4: Explore Coping Skills, Resources, and Supports

Marty speaks with the parents about several referrals of pediatricians since London has not had a well visit recently. London's parents indicate that they are living with maternal grandparents and rely on friends for transportation and on medical assistance. Together, they investigate pediatricians who are providers with their plan. Marty explores how the guardians currently react to London's hostile behaviors and offers possible new responses to integrate, such as looking at London to see facial expressions.

Intervention Task 1: Normalize and Educate

At appropriate times, Marty normalizes how the parents must be feeling during their discussions with Marty (e.g., frustration, not feeling like good parents, fearing a possible medical condition, worry about finances). Marty also provides some education into what happens at a wellness visit to the pediatrician and insight into age-appropriate developmental tasks of 4-year-olds.

Intervention Task 2: Explore Options, and Intervention; Task 3: Develop a Plan and Obtain a Commitment

Marty informs the parents that the well visit and hearing test need to be completed to rule out any medical issues before proceeding with behavioral intervention. Together, the guardians and Marty develop a time frame to contact a pediatrician, evaluate health care options, and schedule a follow-up appointment after the medical appointment is completed.

Intervention Task 4: Prepare Documentation, Follow Up, and Provide Referrals

Marty documents the assessment and plan as agreed upon, including the hearing and pediatric referral, and schedules a follow-up visit with London and her guardians.

In the end, London was medically assessed as having a 90% hearing loss in both ears. The behavioral symptoms were the reason for the mental health referral but, after the holistic assessment and intervention, it was determined that the priority for crisis assessment was medical intervention. Marty could not have addressed the behavioral issues effectively if the medical issue (i.e., hearing loss) was not addressed first.

Discussion Questions

1. What other environment variables could have been explored to develop a holistic plan of action for London?
2. What educational information could also be provided to the parents?
3. What could Marty do if the parents refused to have London's hearing evaluated?
4. Aside from this case study, what other medical conditions could have symptoms that could be misdiagnosed for mental health issues?
5. Are there other referrals that could be made to assist the family?

VOICES FROM THE FIELD 1.3

Treating the Whole Person: The Need for Medical and Mental Health Collaborations

Lisa R. Jackson-Cherry

During the early part of my clinical career, I fought the medical model. I worked in agencies where the medical model was the foundation and often little attention was given to psychosocial, let alone spiritual and religious, issues. I soon became frustrated that all my training and focus on mental health concerns were secondary, at best. As coordinator of a mobile crisis team in Baltimore, I worked with psychiatrists, case workers, psychiatric nurses, and mental health counselors. I became aware that many medical issues mimic mental health symptoms, but I continued to be frustrated with the lack of understanding for addressing mental health issues in crisis situations.

I have learned over the years not to fight the medical model because, while professing to treat the whole person, I might miss an important aspect of the treatment for the client. I learned the importance of collaborating to treat the whole person. Over the years, I observed medical issues often resembling mental health symptoms and came to realize that if medical issues are not ruled out, the mental health treatment plan could be totally inaccurate. I grew more aware of this in my private practice, where I was referred individuals in crisis, many of whom continued on my caseload for long-term counseling.

My practice over the last decade, although independent, has been with a family medical doctor in a rural area. One case stands out as a good example of the need for collaboration with other professionals to provide treatment for the whole person. I was referred a young man about 10 years ago after a recent discharge from the hospital for attempting to kill a family member. At the time, he was 12 years old. He had been diagnosed with an unspecified psychotic disorder; had been hospitalized multiple times per year for suicidal or homicidal behaviors; had been with multiple mental health counselors; and had a list of medical issues, including type 1 diabetes and unassessed "cognitive abilities." These issues did not allow him to monitor his diabetes without assistance from others. In addition, he lived many years being severely abused and neglected by his biological parents and siblings, and his charts indicated that his biological mother was a heavy substance user during her pregnancy. He was eventually adopted into a loving family but only after many years of maltreatment in foster homes. Early on, I spoke to his family and asked if he ever had been evaluated for fetal alcohol syndrome (FAS). My observation of his interactions, facial features, and processing, as well as the history of his early upbringing, made me wonder whether FAS had anything to do with his cognitive abilities and presentation. At the same time, I could tell he was a very insightful child—more insightful than others his age and even some adults. I referred him to Children's Hospital where he received a diagnosis of FAS. The need to integrate the development and environmental factors in his treatment plan were crucial.

Ten years later, he lives in a residential community. I see him not only for stabilization of his psychosis but also for PTSD, relationship issues, and to assist his team in the goal for him to become more independent. It is imperative for his team and me to work together on all these issues. Often, he presents with a sluggish,

(*continued*)

withdrawn affect and behaviors. His cognitive abilities during these times seem more disorganized and incoherent than his normal baseline. My first thought, if I did not know his medical issues, would be that he is depressed and possibly having current racing thoughts—a mental health focus. However, my first question is not about mental health. In knowing his medical issues and his inability to monitor his diabetes (developmental assessment), my first question is, "When was the last time you checked your blood sugar?" followed by, "What was the reading?" Then, we usually take a few minutes to check his blood sugar again and, in most instances, his observed "depressed mood" is the result of a blood sugar reading in the range of 40–50. The next step is not to continue with mental health discussions but to contact his residential counselor, who waits for him in the waiting room, and follow the treatment plan outlined by the facility to raise his blood sugar to an acceptable level before any mental health counseling can occur. Medical concerns must be addressed prior to any mental health counseling. At the end of each session, I complete a form for the agency that includes any medical issues addressed during the session, my consultation with team members, and actions I have taken.

I have learned the importance of collaboration and the need to address medical issues. Of course, some professionals believe they are more knowledgeable about the human condition than others. That is okay. My goal is to help the client. However, over the years, in collaborating with other medical professionals, often on my initiating contact, I have witnessed them reach out more and include mental health updates in their treatment plans. Perhaps we are all seeing that treating the whole person involves many professionals.

OTHER MODELS OF CRISIS ASSESSMENT AND INTERVENTION

Learning Objective 1.7 Apply other models of crisis assessment and intervention to crisis situations.

Let's now examine other approaches or models of crisis assessment and intervention, including the ABC-X and double ABC-X models of crisis, elements of stress and crisis, and ecological and contextual considerations.

The ABC-X and Double ABC-X Models of Crisis

The *ABC-X* and *Double ABC-X models of crisis* were developed through research with families, but the concepts outlined in the models may be generalized to individuals or to larger groups or communities. From his studies of families experiencing separation and reunion as a result of World War II, Reuben Hill (1949, 1958) postulated the ABC-X model of family crisis (see Figure 1.1). According to this theory, there is an interaction among a provoking stressor event (A), the family's resources (B), and the meaning that the family attaches to the stressor event (C). The crisis (X), a state of acute disequilibrium and immobilization of the family system, is an outcome of this interaction.

Hill's original ABC-X model continues to provide a framework for much research in stress and crisis; however, a few scholars (Boss et al., 2017; McCubbin & Patterson, 1982) expanded the model. Among the better-known variations of Hill's work is the double ABC-X model of crisis proposed by McCubbin and Patterson (1982). Writing from a systems orientation, which assumes that systems naturally evolve and become more complex over time, McCubbin and Patterson considered recovery and growth following crisis. The concept of adaptation was introduced to describe lasting functional changes that occur to meet the demands of a crisis or stressful event. According to these scholars, Hill's original model

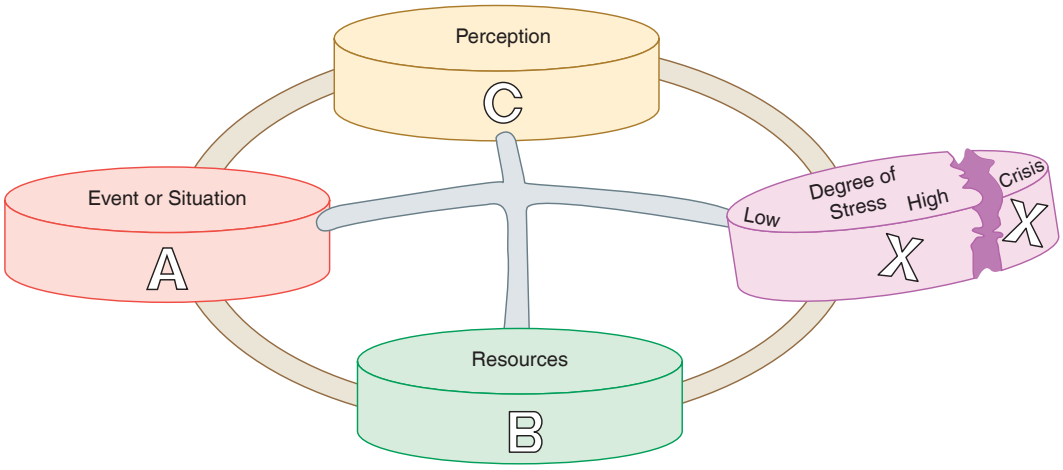

FIGURE 1.1 The ABC-X model of family crisis.

was somewhat incomplete in that it outlined only those factors that contributed to a crisis or breakdown in functioning. Following a state of disequilibrium or incapacitation, additional stressors may accrue and additional resources may be identified and acquired. Subsequently, new perceptions that consider the original event and related hardships or stressors are formulated, along with the application of resources and coping strategies to meet the needs of those stressors.

According to McCubbin and Patterson (1982), there is a "double A" factor that includes Hill's original concept of a provoking stressor plus the buildup of further stressors that must be addressed. These stressors may include unresolved issues related to the crisis-provoking event, changes that occur unrelated to the event, and any consequences of attempts to cope (Bush & Price, 2020). The "double B" factor refers to resources available at the time of the provoking stressor along with tangible and intangible resources that have been acquired or strengthened. Fortified coping resources would be included in this concept. The "double C" factor refers not only to perceptions and meanings assigned to the original provoking stressor but also to accumulated stressors, resources, coping, and the entire situation. Perceptions are influenced by religious beliefs, family values, cultural values, and how the situation may have been reframed. In the model proposed by McCubbin and Patterson, the original crisis (Hill's X factor) constitutes a beginning point, while adaptation ("double X") occurs later in time. Adaptation is an outcome variable involving changes in functioning and perception. More than the simple reduction of stress, adaptation is the degree to which long-term change has occurred in response to the demands of stressor and crisis events.

McCubbin and Patterson (1982) referred to positive long-term changes that occur in response to crises as *bonadaptation*. It was their contention that following crises, some individuals and families do not merely return to their previous states of equilibrium; instead, they are better able to function and handle stress and crisis events. While McCubbin and Patterson referred to positive adaptation as bonadaptation, others have labeled this concept *stress-related growth* (Park et al., 1996) or posttraumatic growth (Tedeschi & Calhoun, 2004).

Elements of Stress and Crisis Theory

Let's now examine elements of *stress and crisis theory*, including provoking stressor events, resources, meaning or perception, and coping.

PROVOKING STRESSOR EVENTS Boss et al. (2017) categorized stressor events and situations by source, type, and severity. Sources are either internal (i.e., originating within the family) or external (i.e., attributable to someone or something outside the family). Examples of internal stressors include partner violence and chemical dependency. These stressors begin within the family and are accompanied by changes in the way the family functions. Partners and family members often adopt various roles, behaviors, and communication styles in their attempt to cope with and survive the actions of the abusive or addicted person. As familiar coping strategies become increasingly inadequate, families cease to function effectively and healthily. "Crises" in these families are rarely a one-time occurrence.

Another example of an internal stressor is infertility. Within the context of the family life cycle, most couples expect to conceive and carry a pregnancy to term. Infertility is considered an internal stressor because the inability to achieve a successful pregnancy originates with one (sometimes both) of the partners as opposed to something outside of the family. It can become a crisis situation because the partners face psychological and relationship changes that affect the way they function. In response to this crisis, many couples can redefine their relationships and reframe their meanings of pregnancy and parenting; unfortunately, however, many couples do not respond well and ultimately separate.

External stressors originate outside the family but affect functioning, nonetheless. Examples of external stressors include natural disasters, terrorism, financial decline due to the stock market, and the rising costs of goods and services. Some external stressors are attributable to forces of nature (e.g., hurricanes, tornadoes, earthquakes), some to humans (e.g., violent crimes, job termination), and some to a combination of the two. Individuals and families typically have less control over external stressors than they do over internal stressors. For example, families have little control over the stock market. However, the ramifications of a declining stock market could have a substantial influence on a family's future.

As with internal stressors, however, the degree to which crises caused by external stressors are experienced is contingent on available resources and the meaning attached to the stressor event. In the United States, hurricanes and tornados (external stressors) annually affect normal functioning and trigger crises for thousands of families that experience deaths, injuries, or various losses. Families who seem to be the most resilient and to have the least difficulty recovering from this crisis are those with available resources such as social support, housing, and financial assets.

Beyond classifying stressors by their source and type, Boss (2006) described stressor situations according to their duration and severity. Some stressors are onetime events, happen suddenly, and resolve quickly; these are considered acute stressors. An automobile accident, for example, might disrupt a family's equilibrium as broken

bones heal and alternative transportation is used but, ultimately, the family's balance is restored. Other stressors, such as lifelong illnesses, infertility, and poverty, persist over long periods of time and are considered chronic. Families caring for aging family members may experience chronic stress and may face disequilibrium and crisis each time the family member's health takes a turn for the worse. The crisis is triggered by changes in health but is the result of additional demands for financial, social, and other resources as well as the perception and meaning the family attaches to the older adult's decline. Finally, it is important to consider whether a stressor situation is an isolated event or part of an accumulation of stressor events. It is often the case that any one stressor event may not be enough to trigger a crisis, but the cumulative effect of the pileup of stressors taxes resources, disrupts equilibrium and functioning, and results in crisis.

RESOURCES Resources may be defined as traits, characteristics, or abilities that can be used to meet the demands of a stressor event and that can be available at the individual, family, or community level. Resources may be tangible (e.g., food, clothing, shelter) or intangible (e.g., social support, self-esteem). When resources are adequate to meet the demands created by a stressor situation, the crisis is less likely to be perceived as problematic—and less likely to lead to trauma. Two types of resources are important: those that are available and used to mediate the initial stressor and those that are acquired, developed, or strengthened after a crisis (McCubbin & Patterson, 1982). Individual resources may include finances, education, health, and psychological qualities. Family resources include the internal, systemic attributes of cohesion and adaptability, along with resources such as financial management, communication skills, compatibility, and shared interests. Community resources include external supports, such as social networks, on which the individual or family can draw.

MEANING OR PERCEPTION Whether a stressor event results in crisis depends not only on available resources but also on the meaning attached to the event. The meaning and perception of the event is the main determination of whether an event becomes a crisis. The meaning attributed to a stressor event is subjective and comes from the way it is appraised through both cognitive and affective processes. Factors contributing to this qualitative variable include the ambiguity associated with the stressor situation, denial, and the belief and value orientation of the individual or family. Ambiguity occurs when facts cannot be obtained. It is often the case that specific information about the onset, development, duration, and conclusion of an unpredictable stressor event is unavailable. When information is unavailable, individuals may be uncertain in their perception of who is included in their families or social support systems. With limited understanding about who is in and who is out, it becomes difficult to ascertain how various roles, rules, and functions will be carried out. Whereas sometimes stressors themselves are ambiguous because data are not available, at other times facts are available but are ignored or distorted. The resulting denial may be a useful coping strategy in the short term but may be damaging if it prevents further action during a crisis (Bush & Price, 2020).

COPING In his original ABC-X model, Hill (1949, 1958) considered coping behaviors as part of the family's resources (represented by "B") to be used in response to demands of a stressor event ("A"). While many researchers agree that coping behaviors are a subset of available resources, coping itself is a separate construct, often interacting with both resources ("B") and perception ("C"). Any effort taken to deal with stress may be considered coping. Thus, coping is a process that requires cognitive as well as behavioral activities (Bush & Price, 2020). Cognitively, people experiencing stressor events must appraise what is happening and assess any potential for harm. They must also evaluate the consequences of possible response actions. Following appraisal, three types of coping responses may be used: direct actions, intrapsychic mechanisms, and efforts to manage emotions.

Direct actions are those behaviors that typically are thought of as "fight, flight, or freeze" responses. Examples include acquiring resources, asking for help, and learning new skills (Bush & Price, 2020). These actions are used in relation to the environment to master stressors and are thought of as problem-focused coping strategies. Emotion-focused coping strategies, conversely, involve mechanisms used to change feelings or perceptions when there is little that can be done to change a stressor. Intrapsychic responses are those that are often thought of as defense mechanisms (e.g., denial, detachment) and allow people to alter their interpretations of the stress-provoking situation (Boss, 2006; Bush & Price, 2020). Additional emotion-focused strategies are used to manage emotions generated by the stressor. Examples include the use of resources such as social support or of alcohol and drugs. Obviously, specific coping responses are neither adaptive nor maladaptive; they are simply efforts to manage.

Ecological and Contextual Considerations

Boss (2006) and Collins and Collins (2005) suggested that stress and crisis are affected by contextual factors. Collins and Collins advocated a *developmental–ecological perspective* to conceptualizing crises. Some crises are triggered by stressor events that are developmental in nature; that is, these developmental crises are expected events in the lifespan of the individual or family. However, regardless of whether the stressor event is developmental or situational, lifespan variables must be considered to determine their meaning and impact on the stress or crisis situation.

In addition to considering developmental factors, Collins and Collins (2005) maintained that the context provided by environmental factors such as interpersonal relationships, community resources, and society at large must be recognized. This approach suggests that each crisis is unique since the ecological determinants for each person will be unique. Therefore, crisis counselors need to be aware that even though many of the clinical considerations and approaches may be similar for a specific crisis and for many individuals experiencing that crisis (e.g., sexual assault), the person in crisis will experience the crisis differently than someone else due to the unique and personal ecological determinants. In essence, do not simply generalize and implement a generic approach to a particular crisis without first assessing the client's unique ecological factors. Complete Think About It 1.2.

THINK ABOUT IT 1.2

Think about the things in your life that have created stress. Were they developmental or ecological? How did these stressors affect you?

Boss (2006) proposed a similar contextual approach to the study of stress and crisis, stating that "factors in addition to the stressful event influence family vulnerability or breakdown" (p. 28). According to Boss, stress is mediated by contextual dimensions, which may be either internal or external. The internal context comprises three dimensions that may be controlled or changed: the structure of the family, psychological elements such as perception and assessment, and philosophical elements such as values and beliefs. The external context comprises environmental or ecosystemic dimensions over which there is no control. External influences on stress and crisis include historical, economic, developmental, hereditary, and cultural contexts. Now analyze the stressor events experienced by the Nguyen family as Case Study 1.1 continues and concludes.

CASE STUDY 1.1

The Nguyens: A Natural Disaster Affects a Family System

Stressor events are those occurrences—positive or negative, predictable or unforeseen—that provoke change in the functioning of a system. They may be categorized as normative or non-normative. The Nguyen family has experienced several provoking stressors, some of which they chose, others that they did not.

Stressors may be categorized by source, type, and severity, and they should be considered within the external context. The external context includes variables such as culture, history, education, and heredity over which the Nguyen family has no control. Crisis counselors who work with the Nguyens need to explore the family's Vietnamese culture as well as possible additional cultural issues related to being part of an immigrant community along the Gulf Coast of the United States. Imbedded in these considerations is the fact that the Nguyens chose to leave Vietnam for the United States some 30 years after the Vietnam War. What kinds of social prejudices and biases does this couple face simply by being Vietnamese? Are there additional biases that they endure by virtue of being immigrants in a post-9/11 U.S. society?

The Nguyens' situation is complicated by their SES, another component of their external context. Prior to the hurricane, they were making a living in the shrimp industry. They had few bills and no debt; however, they also had no medical insurance. On those rare occasions when they required medical attention, they were able to use community public health resources. In the aftermath of the hurricane and the tough economic times that followed, numerous public nonprofit health agencies were forced to close their doors, making it difficult for the Nguyens to access prenatal care. When their daughter was born with a birth defect, they found it necessary to travel to a larger city to receive care for her needs. Thus, they incurred transportation and lodging expenses, further affecting their delicate SES.

Having lost their livelihood as shrimpers to the hurricane, the Nguyens were forced to look for work elsewhere. They were fortunate to have a rather large social support network as well as acquaintances who helped them find employment once retail outlets began to reopen in the months after the hurricane. Unfortunately, the retail jobs they found paid little more than minimum wage and did not include medical insurance. The Nguyens' educational background and minimal fluency in English made it difficult for them to pursue higher-paying jobs.

The Nguyens were also influenced by internal contextual factors—those factors that originate within the family and are accompanied by changes in the way the family functions. When they chose to leave their family and friends in Vietnam to move to the United States, the structure and definition of their family became less clear, particularly given the limited opportunities they had to return for visits. They also were forced to wrestle with issues related to caring for their aging parents. Once the Nguyens were living in the United States, their family structure changed further when they became parents themselves.

For the Nguyens, a relatively young couple, becoming parents could be considered a "normative" stressor—something normal, predictable, and developmental in nature. The stressor of becoming parents could also be considered volitional, as a degree of choice was involved. Conversely, non-normative stressors are those that are unexpected. A catastrophic event such as the hurricane and all its ramifications should certainly be classified as non-normative. For the Nguyens, having a child with spina bifida is also a non-normative and nonvolitional stressor, and its lasting implications make its presence chronic.

To ameliorate their situation, the Nguyens have several resources, derived from both internal and external contexts, on which they may rely. Although they left friends and family behind to move to the United States, they are members of a large immigrant community. From this community, they receive a tremendous amount of social support. In addition, the Nguyens were able to use public community resources for health care and for other basic survival needs in the weeks and months after the hurricane. The Nguyens possess strengths such as initiative, resourcefulness, and a strong work ethic that have helped them to be resilient in the face of their stressors.

Discussion Questions

1. Apply the task model of crisis assessment and intervention to the case of the Nguyens and develop a treatment plan for intervention.
2. What sources of resilience and support can the Nguyens harness to cope with their circumstances?
3. What additional developmental or situational crises are likely to occur in this family in the years ahead? How can you help prepare or inoculate them to better adjust to those circumstances?

Table 1.2 provides some helpful resources for dealing with various crisis circumstances the counselor may encounter. Then read Voices from the Field 1.4, which discusses responses in the wake of a school shooting.

TABLE 1.2 Crisis Resources

General Crisis Intervention Hotlines for Youth (dealing with conflicts, family stressors, suicide, runaway youth, drugs and alcohol, homelessness, and so on)

- National Mental Health Hotline: dial 988
- Boys Town Suicide and Crisis Line: 1-800-448-3000 (voice)/1-800-448-1833 (TDD)
- Covenant House Hotline: 1-800-999-9999
- National Hopeline Network, Suicide & Crisis Hotline: 1-800-442-HOPE

Child Abuse Hotlines

- ChildHelp USA National Child Abuse Hotline: 1-800-4-A-CHILD (voice)/1-800-2-A-CHILD (TDD)
- National Child Abuse Hotline: 1-800-25-ABUSE

Domestic Violence Hotlines

- National Domestic Violence/Child Abuse/Sexual Abuse Hotline: 1-800-799-SAFE (voice)/ 1-800-787-3224 (TDD)/1-800-942-6908 (Spanish speaking)

Substance Abuse/Alcoholism Hotlines

- Alateen: 1-800-352-9996
- SAMHSA's National Drug Information Treatment and Referral Hotline: 1-800-662-HELP

Poison Control Centers

- Poison Control: 1-800-222-1222

Sexual Assault Hotline

- National Sexual Assault Hotline: 1-800-656-4673

Suicide Prevention Hotlines

- National Suicide Prevention Lifeline: 1-800-273-TALK/1-888-628-9454 (Spanish speaking)

Centers for Disease Control and Prevention

- Provides resources for stress and coping, as well as grief and loss specifically during the COVID-19 pandemic (https://www.cdc.gov/mentalhealth/stress-coping/index.html)

Mayo Clinic

- Resources for building resiliency (https://www.mayoclinic.org/tests-procedures/resilience-training/in-depth/resilience/art-20046311)

VOICES FROM THE FIELD 1.4

Chardon High School

Jason M. McGlothlin

The day after the shooting at Chardon High School in February 2012, I volunteered to work with the students who witnessed the shooting as well as the students' families. I met with two students for about an hour and a half each and a family for about 2 hours. I listened to their graphic and detailed reports of what happened and saw the tears they shed. I heard the worries they had about returning to school and the condolences for their classmates who were injured and dead. I heard the countless statements of reaching out to others and of "I hope ____ is okay" and "I need to call ____'s family." I share this story not to discuss what I did but the effect it had on me. Listening and seeing such tragedy, such raw

(*continued*)

emotion and devastation, had a great impact on me not only as a professional but also as a person. I remember coming home that day and giving my 6-year-old an extra-long hug and listening to him explain the shooting because his teachers talked about it at his school.

However, seeing these students reach out to others and care for their community instilled in me a sense of hope. The notion of self-care, burnout prevention, and taking care of self is critical. However, crisis intervention is a roller coaster of emotions for counselors. Counselors quickly become immersed in the emotional state of the crisis, and then leave. We all need to find a unique way to take care of ourselves.

SUMMARY

A crisis is a precipitating stressor event or situation. If the crisis is perceived as leading to distress, and diminished functioning when the distress is not relieved by familiar coping resources, a person may experience trauma. Many individuals and families are resilient and benefit from having met the challenges of a previous or current crisis. How well an individual or family adapts following a crisis is often determined not only by the nature of the crisis event itself but also by the presence or absence of other stressors, the availability of tangible and intangible resources, and how the entire situation is perceived. Crises may occur during normal, developmental transitions during the lives of individuals and families, by situational events that often are sudden and unexpected, or by existential issues that are related to purpose and meaning. It is not uncommon for a crisis event to be perceived as fitting into more than one of these categories.

Crisis intervention begins with an assessment of the provoking event, responses to the provoking event, and the contextual variables that influence the situation. While there are times when a crisis counselor's response to a crisis appears indistinguishable from traditional counseling, crisis intervention is generally thought to be different from traditional counseling because of the occurrence of a crisis event. Paraprofessionals and other mental health professionals provide crisis intervention services, often working in teams where each professional may focus on a particular area of expertise.

Complications of working in crisis situations and with clients in crisis can take a physical, emotional, spiritual, and professional toll on crisis counselors. Each crisis has its own complications. Whether it is miscommunication with resources and agencies, waiting for follow-up from other parties, dealing with the realities of a lack of resources, or seeing clients "take a step back" in dealing with the crisis, counselors need to remain professional and patient. Counselors need to rely on their training, skill, and knowledge to focus on the basic skills in building a therapeutic relationship with people.

A task model of crisis assessment and intervention comprises four essential crisis assessment tasks: (1) Address Safety, Stabilization, and Risk; (2) Follow a Holistic Biopsychosocial-Spiritual Assessment Approach; (3) Clarify the Problem(s); and (4) Explore Coping Skills, Resources, and Supports. These tasks are followed by four essential crisis intervention tasks: (1) Normalize and Educate; (2) Explore Options; (3) Develop a Plan and Obtain a Commitment; and (4) Prepare Documentation, Follow Up, and Provide Referrals.

It is important when reading the remainder of this text to consider the various options for self-care while helping others manage a crisis—proactive ways to avoid burnout and vicarious trauma; manage personal emotions during highly emotional crisis situations (e.g., when working with suicidal clients); and ensure optimal physical, cognitive, and psychological presence when working with clients in crisis. Working with others in crisis is not easy, but it is immeasurably rewarding. Counselors need to stay abreast of their own well-being to help others effectively.

CHAPTER

2 Safety Concerns in Crisis Situations

James Jackson, Lisa R. Jackson-Cherry, Latofia Parker, and Bradley T. Erford*

PREVIEW

When responding to a crisis, counselors must act promptly and safely; thus, preparedness is essential to best practice prior to a crisis occurring as well as during emergency situations. A vital part of crisis preparedness is attention to counselor safety and security. This chapter presents an overview of safety and security procedures and related issues of concern during crisis intervention.

LEARNING OBJECTIVES

Upon completing this chapter, readers will be able to:

2.1 Explain why it is important for crisis counselors to take a proactive approach to safety.
2.2 Describe the role of collaboration in safely managing a crisis.
2.3 Describe workplace safety issues related to crisis counseling, including high-risk clients, physical counselor safety, and working with individual clients in crisis.
2.4 Describe professional counselor responsibilities and skills related to workplace safety.

THE IMPORTANCE OF PROACTIVE APPROACHES TO SAFETY

Learning Objective 2.1 Explain why it is important for crisis counselors to take a proactive approach to safety.

In addition to an emotional and psychological toll, a crisis event can produce safety and security concerns. Basic safety precautions by the crisis counselor when working with high-risk or dangerous clients can reduce the chances of harm to the counselor, client, crisis workers, and others. The ultimate success of crisis intervention depends on planning, training, and the preparedness of team members. A proactive and safety-conscious plan should be in place prior to the actual crisis. Even though most plans will need to be flexible because every crisis is unique, a preliminary plan is better than no plan.

* We thank Charlotte Daughhetee for contributions to the first three editions of this chapter.

Although some professional counselors are naturally inclined toward preparedness and plan for every reasonable contingency, many counselors have good intentions about developing an emergency plan but most never seem to follow through with preparations to completion. Indeed, most crisis plans are developed in response to a crisis. However, it is essential for agencies, schools, universities, institutions, and communities to have a crisis plan in place prior to a crisis occurring. When a crisis event occurs, particularly a large-scale disaster that affects many people, crisis teams must be ready, with well-defined roles, resources, and training. Unfortunately, while crises have huge impacts on systems and communities, they are usually low-priority items with respect to planning and resources.

The Federal Emergency Management Agency (FEMA; 2021) stresses that crisis planning is an ongoing cycle of planning, organizing, training, evaluating, and improving. These components can be used as a framework when setting up safety plans for personal individuals and team safety. Chen et al. (2013) found that communities that facilitated collaborative partnerships between private and public organizations were the most resilient when faced with a crisis or disaster event. It is better to have a plan in place to be able to revisit and revise than to not have any plan at all.

Hughes and Gilmour (2010) researched the concept of work safety and climate. Community mental health workers, particularly those who work with clients at high risk for violent behavior, would benefit from increased safety training and general attention to safety issues in the workplace to create an atmosphere where safety is attended to and valued. Hughes and Gilmour reported that most workers who attended voluntary training on managing violent clients felt better prepared and safer because of such training. Flannery et al. (2011) stressed that safety training for new professionals is an important priority. Ongoing training should be provided to address the use of self-defense, appropriate restraint practices, and restraint-free procedures. Additionally, they recommend communication skills training, supervision, and mentoring.

Crisis teams are also essential in educational settings and comprise personnel within the school or university who have appropriate expertise (see Chapter 13; see also Brock, 2013). School and university crises can involve events such as the death of a student or faculty member, violent occurrences within the community, or large-scale disasters as well as a crisis experienced by a single student. Schools and universities are often at the center of a community or neighborhood, so a crisis that affects a school or university has a ripple effect within the larger community and vice versa. As recent events show, schools and universities must be prepared for a wide range of crisis events, including threats and actions of violence. Threat assessment teams on campuses are essential and should incorporate input from all pertinent stakeholders (see Chapter 13).

COLLABORATION AND CRISIS MANAGEMENT

Learning Objective 2.2 Describe the role of collaboration in safely managing a crisis.

As part of a crisis team, counselors contribute to the success of crisis intervention through collaboration with other crisis workers. Ultimately, this collaborative spirit enhances public safety.

A crisis team comprises workers from across the community or organization who come together in response to a crisis event. Mental health providers are an essential component of any crisis team, and all members of a crisis response team should work together toward the same goal, which is to advance the recovery process following a disaster. Survivors of crises need access to medical care, food and water, shelter, mental health services, basic safety, and sometimes religious/spiritual connections; the realization of recovery is possible only when there is interagency cooperation.

It is important to remember that each crisis worker has a unique role. In the event of large-scale emergency situations (e.g., natural disasters, terrorism, civil unrest), the safety and security of the area will be maintained by law enforcement, in some cases with the assistance of National Guard troops or federal agencies. Law enforcement is responsible for the physical safety of the community. Firefighters and medical personnel provide rescue and first-aid assistance. These first responders will collaborate with crisis counselors by referring clients in need of immediate mental health evaluation services.

In addition to collaboration as team members during a large-scale disaster or major crisis, counselors may also collaborate with law enforcement officials regarding a crisis with an individual client. For example, law enforcement may be involved in welfare and well-being checks in situations where a client has become an immediate threat to self and to others. Wilcox (2015) states that it is beneficial for those in law enforcement to receive training in the assessment and management of individuals diagnosed with mental disorders; however, this is not their first priority. Counselors and the community should know that the role of the law enforcement officer is to maintain safety—personal safety for themselves and their partners as well as for the community. They are not counselors, and their policies, practices, and trainings are focused on safety. Law enforcement intervention is sometimes the first step in a process that results in treatment for individuals in acute mental health crisis.

SAFETY ISSUES

Learning Objective 2.3 Describe workplace safety issues related to crisis counseling, including high-risk clients, physical counselor safety, and working with individual clients in crisis.

Even as they are working collaboratively to achieve public safety, crisis counselors must also be aware of their own safety and take proactive steps to ensure a secure and effective counseling environment.

High-Risk Clients

When clients are a danger to themselves or others, crisis counselors may work alone or in partnership with local authorities to ensure everyone's safety. Law enforcement is usually involved when a client presents a threat to public safety, as with high-threat homicidal and violent clients. Clients who pose a danger to themselves and others may need to be hospitalized. (See Chapter 6 for information on voluntary and involuntary hospitalization.) Counselors should not transport such clients to the hospital and should instead enlist the client's family or friends if the client is low or moderate risk and is voluntarily entering an inpatient unit. The only exception is when a crisis counselor is part of a specialized crisis team for which transportation is the expectation.

Even as part of a crisis response team, at no time should a counselor accompany a high-risk client without another team member. In these situations, several commonsense procedures should be followed:

- The counselor should make sure the car does not have any objects that could be used as weapons, and car keys should always remain in the possession of the counselor.
- The client's belongings should be packed in plain view of the crisis team member, and the client's bag should be placed in the trunk of the car.
- The client should be seated where the counselor can see them, and the client should wear a seat belt that also serves as an extra barrier and restraint.
- If a client decides to leave the car, the counselor should not physically prevent the client from leaving. However, if the client leaves and poses a risk to self or others, it is crucial for the counselor to contact law enforcement or the community service board (depending on the mechanism in the state, Commonwealth, or territory) who can provide another layer of assessment and involuntary hospitalization assessments and/or execute an emergency petition.

A high-risk client who needs to be involuntarily hospitalized will likely need to be transported to the hospital by a family member, a crisis response team, an ambulance, or law enforcement. If a high-risk client suddenly leaves a counselor's office, the counselor should not attempt to physically stop the client and should notify law enforcement for assistance. The counselor can assist by offering information to authorities as to what the client was wearing, the direction the client was heading, and the brand, model, color, and license plate of the vehicle the client used (if appropriate). Having updated information on client address and emergency contact person is essential to give to law enforcement.

Because counselors work closely with hospitals and medical personnel during intervention and treatment of suicidal and homicidal clients, a strong working relationship between mental health providers and local authorities is essential for effective crisis intervention. It is the policy of most police departments, during the transport of a person who is high risk, to provide a restraint (e.g., handcuffs) for the safety of the person, the public, and the law enforcement officer. As noted earlier, law enforcement officers are not counselors; they are sworn to protect the public. They are also armed with their service weapons, so a person who is suicidal or homicidal being able to gain control over a law enforcement officer's weapon is potentially harmful to not only the client but also to the law enforcement officer and the public. Chapter 6 provides important information regarding assessment of suicidal and homicidal ideation.

Physical Safety

Given the likelihood that crisis counseling often takes place in the field, counselors are likely to find themselves in hazardous surroundings and therefore must be aware of safety procedures for their own security and the well-being of their clients. According to FEMA (2021), first responders (e.g., firefighters, police, paramedics) are on the front lines of disaster and crisis and face danger from numerous threats, including damaged

infrastructure; secondary collapse; unfamiliar surroundings; unstable structures; falling or tripping hazards; falling material or flying objects; exposure to hazardous materials; decontamination; and exposure to smoke, dust, fire, noise, electrical hazards, contaminated air and water, and dangerous equipment.

Crisis counselors with disaster response teams (e.g., Red Cross, critical incident stress management) may not be on the front lines of disaster initially but may still be in dangerous areas and should be aware of potential hazards. After the primary response, counselors may be out in the field and should be careful around damaged structures and other hazards. It is imperative that mental health field workers obey all field safety guidelines established by the emergency management team. Crisis counselors should follow the recommendations for medical evaluations and screenings established by federal or local response agencies after exposure to hazardous materials.

In addition to personal physical safety, crisis counselors who arrive at a disaster management scene after initial first responders will be working with clients who are in a state of disbelief and intense grief and who may be unpredictable in their behaviors due to uncontrollable circumstances. The mental health of the community members in a disaster situation will likely be unknown to the crisis counselors, making situations unpredictable and unknown. It is important to gather as much information as possible in the precrisis briefings regarding environment, unique circumstances of the disaster, persons affected, resources available, and community support members who may be able to intervene.

Crisis counselors may be part of community or state crisis response teams and they may be one of the initial responders to a mental health emergency. As with other medical emergencies, crisis counselors may have limited presenting information about the clients. Mental health emergencies can result in unpredictable client responses. In many situations, someone other than the client may have contacted the crisis response team (e.g., family member, community, school, neighbor), often causing client resistance to a helping response. Clients may also experience intense fear, anger, and powerlessness during the initial crisis and may project those feelings onto counselors.

Clients experiencing crises are sometimes reluctant to leave their homes and belongings, which may put their own lives and the lives of others at risk. In these situations, counselors may only be able to offer information about disaster relief services and basic safety. Counselors must be aware of their surroundings and the context of interactions with survivors. It is easy for counselors to become complacent with misguided ideas such as "I'm a helping professional—who would hurt a person trying to help someone else?" The reality is that individuals who perceive they or someone they care for is being threatened (e.g., physically, mentally, spiritually) may go into fight, flight, or freeze mode, thereby creating a precarious situation for counselors and other crisis workers.

Safety at school locations requires a great deal of forethought by the school's crisis intervention team. Staff must ensure all students are accounted for and safely returned to their guardians. This means taking attendance during evacuations and introducing a checkout system for student departures, such as when parents and guardians are required to pick up all children being sent home because of a crisis situation. Child safety is a critical issue during crises. Chapter 13 provides a great deal more information on school safety. Now read Voices from the Field 2.1 for insights on dealing with physical safety.

VOICES FROM THE FIELD 2.1

Dealing with an Aggressive, Hostile Woman in a Prison Setting

Matthew J. Paylo

As the mental health director at a women's maximum-security prison, I was familiar with managing aggressive behaviors. It seemed like a daily occurrence that my staff was asked by prison personnel to aid in the deescalation of an individual who was managing a specific stressor or rebelling against the restrictiveness of the prison setting in general. As a director, I often used professional development training to help new mental health staff deal with various crisis situations specific to the prison setting. Later, I would realize I still had a few things to learn about managing crises myself.

One Saturday in November, I found myself trying to get ahead on evaluations and paperwork, and I was the only mental health staff member in the prison. A correctional officer contacted me regarding Delia, a woman who initiated a dispute with another woman on her wing. The correctional officer had managed to lead her off the wing and into a holding cell. He reported that she was screaming and clearly agitated by the situation. Delia had been involved in our anger management program, and I recalled that she had recently head-butted a sergeant. Thus, the staff had limited tolerance for Delia's actions and behavior.

When I got to the scene, I evaluated the environment and deemed it was safe to enter and engage the woman. Another correctional staff member asked if I wanted to enter the cell; having worked with the prisoner for over 6 months, I did not hesitate. Assessing on the continuum of escalation, I had surmised that Delia was not just anxious or defensive but was acting out and using intimidation and posturing to relay her level of frustration. I spoke her name softly and respectfully inquired about the precipitating event. Using my already existent relationship with Delia, I transitioned her to focus on communicating verbally instead of acting out physically. Trying not to overreact to her language and threats, I attempted to show her empathy on how difficult this situation must have been for her. Avoiding a power struggle with Delia was another aim of mine and she seemed to be deescalating slowly (i.e., she was no longer pacing in the cell and her fists were no longer clenched).

Yet on reflection, I remembered that I was standing not off to the side but directly in front of Delia. This face-to-face posture was an obvious point that I had missed at that moment. Her eyes gazed at the floor and when she raised them toward me, she reacted to my stance in a less than positive way. She immediately stated, "Who the f*** do you think you are trying to control me? You can't stop me from leaving. I'll take you out." At that moment I realized my posture was intimidating, and I moved two quick steps to my left. Delia immediately threw a right-handed hook as I moved just out of her reach. The correctional staff flooded the cell.

The point of this story—the same point I have relayed to my staff and students—is that even if you are saying all the right things in the right way, nonverbal communication (posture, positioning, affect, facial reactions, etc.) is often more important and can set the tone for the entire interaction.

Individual Clients in Crisis

Individual clients in crisis can present an increased risk for the counselor, particularly a sole practitioner in private practice. One of the most potentially compromising settings for a counselor is an isolated private practice. Reeves (2016) noted that a vulnerability of sole private practitioners is the location of their practices, which might be secluded from others; this becomes particularly challenging without an adequate prescreening or assessment process to identify the potential types of risks clients may pose. Reeves identified important purposes and benefits for practitioners assessing risk at the outset of the therapy process, including:

- Knowing what is being assessed and what questions are most important to ask
- Providing information that is clear and can inform the evaluation process

- Documenting the evidential factors upon which our evaluation is based
- To delineate the boundaries of the therapeutic contract
- To ensure compliance with specific risk management protocols
- To guide case conceptualization discussions in supervision
- To promote clients' understanding and engagement in treatment
- To provide an opportunity for risk assessment that has the potential to promote insight and potential for change (p. 22)

A counselor practicing alone at home faces additional risks due to the isolation from others and allowing clients to gain the counselor's personal information. Even counselors who have solo practices in multipurpose buildings with other people nearby are at risk, as the other occupants in the building may not be aware of calls for help. Often, such multipurpose office buildings provide a false sense of security. The counselor may feel safe because of people and activity in adjacent rooms; however, the workers in adjoining suites are focused on their tasks and are not attuned to the needs and potential risks of the private practitioner next door. Having other people in the building does little to help if a counselor decides to see clients in the evening after the other offices have closed and the workers have gone home. For example, counselors who provide services for school-age children often see clients in the evening hours after school.

Counselors should always have a clear escape route that cannot be blocked by chairs or coffee tables, remove objects from the office that could be used in an attack, install panic alarms, and consider carrying pepper spray. If a client becomes threatening, the counselor should remain calm and attempt to redirect the client, thereby defusing the threatening situation. In any situation where the escalation is becoming unmanageable, safeguarding the counselor's personal safety and that of peers is the most prudent decision. Often this means allowing clients to leave or leaving the situation and immediately contacting authorities or others trained to intervene.

Although private practitioners rely upon referrals for much of their livelihood, it is prudent to consider who is referring the client and to obtain as much information as possible from the referral source. New clients should be screened carefully with as much initial information that can be provided: presenting problem, any current diagnosis, current symptoms or behaviors leading to concern, current and history of level of risk to self or others, referral source, and emergency contact information.

Ask why the client is seeking counseling now to determine whether there is a recent crisis event in the client's life. Be alert for information regarding past incidents of self-harm or violence. Pay attention to the client's tone of voice and any verbal cues that the potential client might express, such as "You must get lonely by yourself in that office." Safety procedures for new clients could include having another person present in the office when seeing a new client. Some factors can indicate a greater risk for dangerous behavior from a client seeking private-practice counseling services. Signs for concern in a client include a history of violent behavior, a psychiatric or forensic history, intoxication, erratic speech and behavior, and sexual posturing and insinuation. Additional warning signs include a history of self-harm and abuse, vengeful or paranoid statements, and recent changes in the client's situation or support system (Reeves, 2016).

Whenever possible, counselors should get a thorough history, preferably from the referral source, before seeing a client. Of course, this is not often the norm for crisis centers. Once again, the dangers of self-referred clients and those referred by nonprofessionals

are evident because it often is not possible to get such a history. Prescreening is often a luxury. With many crisis situations, the amount of information gathered prior to making the first appointment varies and may be rather limited. Thus, having a safety plan in place prior to seeing clients is strongly advised.

Counselors in private practice must be prepared to manage additional types of risk beyond those associated with the referral and intake processes. Reeves (2016) notes that clients experiencing chaos who present for services benefit from a counseling process that balances client autonomy with sufficient containment and boundaries to provide clients with a sense of stability and predictability. To best address risk in the counseling process, Reeves proposed moving beyond a binary perspective of risk toward a multidimensional perspective that invites counselors to consider factors such as boundary management, personal biases, fear and anxiety, and protocols that might be incongruent with philosophy of a therapeutic approach. Rather than considering risk as something to be "managed and minimized," Reeves suggested that risk might be better conceptualized "as an important aspect of therapy that, with care and attention, can provide a fertile space for clients to explore the issues that have brought them to therapy and consider key points of change and insight" (p. 8). Now read Case Study 2.1, which considers safety precautions for counselors, then complete Think About It 2.1.

CASE STUDY 2.1

Safety Precautions for Counselors

Ellery is a part-time sole practitioner, a professional counselor who sees clients in the evenings from 5:00 to 9:00 p.m. at an office building. On most nights, Ellery is the only person in the building. Ellery is trying to build a practice and has been networking and promoting the practice as practicable. Ellery receives a voice mail from someone named Mr. Smith, who explains that he is looking for a counselor. Ellery returns his call, and the following conversation takes place.

Ellery:	Hello, is this Mr. Smith?
Mr. Smith:	Yes, that's me.
Ellery:	This is Ellery Jones. I'm returning your call.
Mr. Smith:	Oh yeah, the counselor.
Ellery:	You left a message saying you want to set up a counseling appointment? I'm calling to see if you are still looking for a counselor and what you are seeking a counselor for to determine if I am able to assist you.
Mr. Smith:	Great, yes, I found your name and picture on that site about counselors and I thought you looked nice. So I decided to call you.
Ellery:	Can you give me an idea of why you are interested in counseling?

MR. SMITH: Well, my girlfriend just broke up with me and I'm pretty upset about that. I can't believe she would do this to me after all we've been through. I guess I shouldn't be surprised, women always let me down. I've been in counseling before and I know how it works. I've got plenty of money, so you don't have to worry about me being able to pay your fees.

ELLERY: So, you want to seek counseling to discuss the recent loss of this relationship.

MR. SMITH: Yeah, I know I've got to get a handle on this before I spiral out of control. My last girlfriend called the cops on me when I confronted her new boyfriend, and I don't want that to happen again. I really want to handle things differently this time. I know I have to change how I react to things like this.

Discussion Questions

1. Should Ellery take on Mr. Smith as a client? Explain.
2. If yes, what safety precautions should Ellery take?
3. How might Ellery's desire to increase client numbers get in the way of screening?

THINK ABOUT IT 2.1

Think about why a counselor might be reluctant to terminate a counseling relationship with a dangerous client. What ethical considerations should be implemented if a counselor chooses to terminate such a relationship? What safety precautions should a counselor put in a personal safety plan?

SPECIAL ISSUES RELATED TO SAFETY AND THE COUNSELOR

Learning Objective 2.4 Describe professional counselor responsibilities and skills related to workplace safety.

According to the Occupational Safety and Health Administration (OSHA; 2015), 20% of workplace injuries take place in health care and social service settings; 50% of these injuries are due to assaults and violent incidents from patients and clients. The actual rate may be much higher, since research shows that assaults of this type are often unreported.

According to the Bureau of Labor Statistics (2021) Census of Fatal Occupational Injuries, of the 5,333 fatal workplace injuries that occurred in the United States in 2019, 761 were intentional injuries by another person. This census report suggested workers with the highest risk of violence against them are delivery drivers, health care professionals, public service workers, customer service agents, and law enforcement personnel. The reason health care and social service workers are at a higher risk for workplace violence is due to multiple factors, including working with a client population that has a history of violence and/or substance abuse, facility design that impedes visual contact with other workers, poor lighting, and a lack of means to communicate for help during an emergency.

Factors that can increase a client's risk for violence to self and others include long waits in emergency departments, the feeling that one has limited control over their current situation, a lack of support from family and friends, a sense of hopelessness, and a pattern of impulsivity. Additionally, budget considerations may leave facilities understaffed and the staff overworked. Combined with a high turnover rate among workers, these factors create a dangerous work environment and a high risk for assault. According to the U.S. Department of Labor (2019), the most effective protections offered by employers is a zero-tolerance policy on workplace violence. This policy includes all workers, patients, clients, visitors, and contractors, with no exceptions.

What effect does workplace violence have on mental health workers? According to OSHA (2015), workers who have experienced a violent incident may have:

- Short- and long-term psychological trauma
- Fear of returning to work
- Changes in relationships with coworkers and family
- Feelings of incompetence, guilt, and powerlessness
- Fear of criticism by supervisors or managers (p. 22)

Mental health workers who have been assaulted should have access to debriefing and counseling to address trauma and stress concerns. It is very important that workers who have experienced a violent incident find support and care as soon as possible to mitigate the shock and distress that may result from such an episode. When an incident occurs that includes injury, death, or near misses, the U.S. Department of Labor (2019) suggests the following:

1. Determine who needs to be notified, both within and outside of the organization.
2. Include all workers who were involved in the incident in the investigation.
3. Identify the underlying issues leading to the incident, such as the initial incident and response.
4. Collect and review other information, such as training records and past incident reports, that may be relevant to review.
5. Investigate near misses for potential future issues or why the near misses did not lead to the same outcome.

Mental health providers working in forensic inpatient units (e.g., health care institutions to which incarcerated patients have been diverted) have an increased risk of harm from violent clients. Such clients may require physical restraints, and staff at inpatient units must be trained in proper restraint methods. Staff training should focus on using therapeutic methods to deescalate a violent client and on empowering the client to take ownership of treatment so that the need for restraint can be reduced (see Chapter 4; see also Simon, 2011). Inpatient mental health workers should be cognizant of the fact that federal policy requires that alternatives be attempted before restraints are used and that restraints be used only when necessary. In addition, a client can remain in restraints for only a defined amount of time, and a restrained client must be assessed in a timely manner. Freeman and Sugai (2013) point out that recent advocacy efforts have encouraged many states to reexamine state laws regarding restraints and to use restraint and seclusion as a last resort. All of these instances also resulted in a call for the integration of trauma-informed practices in the workplace, which is also recommended.

Trauma-Informed Treatment

Trauma-informed treatment has emerged in recent decades due to an increasing awareness of the long-term effects of traumatic events on individuals. According to the Substance Abuse and Mental Health Services Administration (SAMHSA; 2014), trauma from past events can arise when a client does not feel safe or supported in a therapeutic environment. A trauma-informed therapeutic environment engages all participants in a collective approach to treatment. Resilience and hope are fundamental elements of this approach. Substance use disorder, eating disorders, depression, and anxiety can be current symptoms that are connected to previous trauma; however, most clients who seek counseling for such issues may not realize that these presenting problems are connected to earlier trauma in their lives. Trauma-informed treatment calls for mental health practitioners to work in a collaborative manner to empower clients and families and to promote resilience. SAMHSA puts forth six key principles for working from a trauma-informed perspective:

- Safety
- Trustworthiness and transparency
- Peer support
- Collaboration and mutuality
- Empowerment, voice, and choice
- Cultural, historical, and gender issues

With an understanding of the possible long-term effects that can emerge from a previous trauma, counselors can work with clients from a more perceptive, informed, and strengths-based perspective. The SAMHSA website has many resources on creating trauma-informed therapeutic environments and provides useful information on trauma-informed treatment (www.samhsa.gov/nctic/trauma-interventions). Complete Think About It 2.2.

THINK ABOUT IT 2.2

Think about the ways a counselor's previous trauma might impede work with clients. How might this previous trauma manifest itself in the session? What should a counselor do if experiencing emerging trauma while working with a client? What should you do if you become aware that a counseling colleague is becoming impaired due to a previous trauma?

Counselors Most at Risk

Flannery (2015) indicated that inpatient settings and residential house counselors are most at risk for assault from clients. Counselors-in-training who are young and with less experience are also at increased risk. Female counselors are just as likely as male counselors to be assaulted. Violent incidents increase because inpatient and residential clients are likely to be diagnosed with severe psychopathology and because staff members spend more time with clients. Counselor education programs should directly address the concept of safety and security, especially prior to internship placement.

Violent clients pose a real threat to counselors, and some form of aggressive client behavior is likely to occur during every mental health provider's career. Kivisto et al. (2015) found that three out of four psychologists surveyed had experienced harassment from clients, and one in five had been threatened.

According to the American Mental Health Foundation, data on worker assaults in recent years indicate that clients most at risk for assaulting mental health workers include those with a history of violence, a history of substance abuse, and a diagnosis of schizophrenia (Flannery, 2015). Counselors should also take note of any loss of reality testing, such as delusional thinking or hallucinations. Impulse control issues (e.g., mania, intoxication) are also risk factors for violence. Every counselor, regardless of setting, should institute safety rules and abide by them. To assume that violent client behavior "couldn't happen here" is to put oneself in peril.

Stalking

Another danger faced by mental health providers is the threat of being stalked by a client. Purcell et al. (2005) surveyed 1,750 psychologists and found that 19.5% reported being stalked by a client. More recently, Kivisto et al. (2015) found that one in seven psychologists surveyed had been stalked. Most stalking incidents involved male outpatient clients stalking a female psychologist. Most of the stalkers had a diagnosis of a personality disorder or psychotic disorder; however, some of the stalkers were diagnosed with substance use, major depressive disorders, or anxiety disorders or had no previous clinical diagnosis.

Purcell et al. (2005) found that clients who stalk were motivated by a need for intimacy, infatuation, resentment or revenge, or a misinterpretation of the therapeutic relationship as a friendship or romance. They stress that mental health providers must be very clear about the nature of the therapeutic relationship and set firm boundaries, preferably in writing. Setting boundaries should include not allowing contact between sessions, not extending session length, keeping personal information private, not friending clients on social media, and not placing personal objects (e.g., family photographs) in the office, which may give the client too much personal information or may create an atmosphere of familiarity.

Cyber stalking has become a major threat as well. Personal information about practitioners is readily available on various websites, and clients can harass counselors using social media and online forums to stalk and verbally attack practitioners.

When clients cannot respect the nature of the therapeutic relationship and continually violate boundaries, the counseling relationship should be terminated and the client referred for a psychiatric evaluation. In extreme cases, the counselor may have to obtain a restraining order for self-protection. As always, counselors should seek supervision and consultation and should clearly document all events, actions, and decisions.

In-Home Counseling

Some agencies offer in-home counseling, especially to clients who have low incomes, homebound, or older and those who may have difficulty finding transportation to

counseling centers. In-home counseling is a highly effective service delivery method, but it brings with it increased safety concerns. In-home services should never be conducted alone; counselors should be accompanied by co-counselors or behavioral aides. In-home counseling agencies should make it a practice for counselors to check in by phone prior to each session and after leaving the client's home. When conducting in-home counseling, counselors should familiarize themselves with the community and the environment prior to the actual session. Where are the building exits? Is there adequate lighting? At what time of day is the session scheduled? Who will be home when a session is scheduled? If the counselor feels uneasy about the environment, the counselor should leave the premises. See Case Study 2.2, which addresses in-home counseling safety issues, then read Voices from the Field 2.2.

CASE STUDY 2.2

Ensuring the Safety of Young Clients

For the past 3 months, you have been seeing an 11-year-old client referred for behavioral concerns by the special education teacher at an elementary school located in a small, rural community. Your client has demonstrated several temper outbursts with his teachers and has also been in fights with boys in his class who he claims were "making fun" of him. The client is large for his age and has cognitive and other developmental delays, and the principal and the special education teacher want him to learn strategies for emotional regulation and anger management. He lives on a small farm with his father and grandfather, both of whom agree that he would benefit from interventions focused on his behavior; consequently, these counseling goals are stated in his individualized education plan (IEP). Because his father and grandfather have expressed concerns about his behavior at home, you have been seeing the client at home as well as at school. These sessions have gone well; he has readily engaged in the therapy process, rapport is clearly in evidence, and you, his father and grandfather, and the school are pleased with his progress.

Sitting outside on the back porch during today's home session, however, a different concern arises. The client tells you that his father and grandfather sometimes beat him with a belt when he is "bad." He lifts his shirt to show you "where it hurts," and you see his back and stomach covered with dark bruises, many of which clearly show the outline of a belt buckle. You realize your client needs you to take action to protect his safety; at the same time, you are aware that the father and grandfather, both large men, are inside the house.

Discussion Questions

1. How will you act to ensure the safety of your client, while at the same time maintaining your own personal safety?
2. What is your responsibility as a mandated reporter?
3. What will you do next?

VOICES FROM THE FIELD 2.2

Reflections on Personal Safety During In-Home Counseling

James Jackson

"Mom, it's an insurance salesman" is the typical reaction from children answering the door when I make an initial home visit with a client family. This reaction is not surprising since my outward appearance in terms of physical features (i.e., a middle-aged White male), my business casual attire, and my briefcase mark me as an outsider who appears different from most who call upon this home. Over the years of providing in-home counseling services to children and families as part of my contract work with the Department of Human Resources, I have grown accustomed to this reaction. Scheduling changes for client sessions are an expected aspect of this work as clients' lives are impacted by systemic forces outside their control; minimum-wage employment does not often offer much flexibility in terms of work schedule, and unreliable—or absent—transportation is a common challenge faced by many.

Many of my clients are amazingly effective at coping with limited resources and thrive while focusing on taking life day by day, adapting as needed. So, when a regularly scheduled weekly session is changed due to a work schedule conflict, I appreciate that the client still wants to have the session, although at a time much later in the day after the client gets home from work. Safety never enters my mind. I have been to this client family's apartment several times in the past. It is located in an older subsidized housing complex made up of regimented brick buildings that appear almost identical in the daylight; strange how different it looks, and feels, by night. "Why did I have to park so far away?" I wonder to myself as I walk alone on the dark, cracked sidewalk past the silent windows of the buildings, streetlights broken or dimly buzzing. Suddenly, I am aware. I clutch my briefcase a little tighter, realizing my reaction offers no real security. I am aware that I do not feel entirely safe, and even as the realization strikes me, I begin thinking about my client's family members who live and sleep here each night. How safe do they feel? What steps are they taking to minimize risk? I resolve to invite them to discuss safety planning as part of our work together—and to engage in intentional planning for my own safety as well.

Research and Recommendations on Client Violence

Jussab and Murphy (2015) state that client violence is not uncommon and may be underreported; therefore, the significance of the violent incidents may not be appropriately acknowledged and tracked. Counselors who have suffered a violent incident with a client must often continue to work with the client, or at least work in the same facility. The long-term effects of close proximity to a source of trauma and the inability to distance oneself from an attacker can contribute to increased burnout and stress. Mueller and Tschan (2011) report that fears and anxiety around the attack often linger and that mental health workers who have suffered an assault are prone to lower levels of job satisfaction and may begin to question their professional competency.

Jussab and Murphy (2015) examined the aftereffects of client violence on counselors. They identified three main themes with six subthemes that may emerge after an experience of client violence. The first main theme is "processing the moment to moment experience of client violence" (p. 290). Embedded within this main theme are the subthemes of experiencing fear and having an inner dialogue about what to do to contain the incident. One of the counselors who participated in the research commented:

> Relying on therapeutic skills to deal with the violent incident brought a sense of foolishness and the inner dialogue highlighted frustrated failure as she registers her disappointment with the usefulness of her clinical engagement skills in the midst of the incident. (p. 291)

The second main theme is "professional vulnerabilities and needs as a result of client violence" (Jussab & Murphy, 2015, p. 290). The questioning of one's competency as a practitioner and the need to get appropriate support from supervisors and coworkers are the subthemes for this theme.

Finally, the third main theme is "the ruptured therapeutic relationship" (Jussab & Murphy, 2015, p. 290). Within this theme, the subthemes include rejection of the client or accepting the client through formulation. Client acceptance could be regained through empathy and a process of examining the reasons for the client outburst. The resultant emotional and psychological harm experienced by a counselor in this situation would often go unnoticed by supervisors. Thus, it is imperative for supervisors and organizations to be mindful of the inner turmoil experienced by counselors who have suffered a violent attack from a client.

Jussab and Murphy (2015) make the following recommendations for managing the aftereffects of a violent incident committed by a client:

1. Practitioners should be aware that client violence happens infrequently, sporadically, and is usually perpetrated by a lone client.
2. Practitioners should engage in intentional self-care.
3. Practitioners should examine and accept emotional and psychological distress.
4. Practitioners should be alert to coworker distress and provide support.
5. Workplaces should examine safety policies and procedures.

Jussab and Murphy (2015) specifically note that even low levels of violence in a facility should not be tolerated. Their research indicates that practitioners need to process the experience of a violent client incident within a context of safety and support. Jussab and Murphy also assert that each incident be reported so that a clear picture and accurate statistics can emerge regarding this workplace safety threat. Failure to do so leads to a state of passive acceptance of violence and an increased risk of burnout and workplace violence within the profession.

This concept of passive acceptance of violence was also noted by OSHA (2015) as a factor in the high risk for violence to health care and social service workers. If clients perceive that acting out and being aggressive are tolerated and will not be reported, they are more likely to engage in such behavior. Passive acceptance of low-level violence fosters an undercurrent of threat and undermines safety for workers as well as the therapeutic process of clients.

See Case Study 2.3, which provides an example of workplace safety considerations for counselors, then read Voices from the Field 2.3.

CASE STUDY 2.3

Counselor Safety

Jules works as a college counselor at a small university counseling center that has one other counselor on staff. The counseling center is in the basement of a campus office building that is vacant each day by 6:00 p.m. Jules and a colleague have an unwritten safety rule that they will not schedule appointments later than 4:30 p.m. Jules occasionally breaks this rule when working longer term with a trusted client. One day a married student named Nova calls to inquire about marriage counseling; the partners, Nova and

Patton, are disagreeing about many issues and they want couples counseling to improve communication. Jules suggests several possible appointment times, but Nova works and cannot possibly come in until 6:30 or 7:00 p.m. Since the case seems straightforward, Jules agrees to meet the couple at 7:00 p.m. for an initial session.

When Jules unlocks the counseling center door and admits the couple, Jules immediately feels anxious. Patton's appearance seems unkempt, and Patton does not make eye contact with Jules. Jules and the couple proceed into the office, fill out intake forms, and go over informed consent. Although Patton is quiet, Patton appears to be amenable to treatment, signing all documentation. Jules feels increasingly uneasy and for the first time realizes that the office is arranged with the clients blocking the only door. Patton stares at the floor while Nova explains to Jules that they have been disagreeing because Patton has a diagnosis of schizophrenia and refuses to take medication. Nova hopes that through couples counseling Patton will become more compliant with medication and they can learn to communicate and interact more positively. Jules maintains an appearance of calm and listens to Nova while Patton remains silent.

When Jules finds out from Nova that Patton is under the care of a psychiatrist, Jules explains to the couple that before any relationship work with Jules can begin, it is necessary that all medical issues be addressed. Jules further explains that it would be inappropriate for Jules to work with the couple unless Jules was working collaboratively with the psychiatrist who is caring for Patton. The couple agrees to make an appointment with the psychiatrist the next day, and Jules has them both sign releases in case the psychiatrist wishes to communicate. Jules escorts the couple from the counseling center and breathes a sigh of relief as the door locks behind them.

Discussion Questions

1. What was Jules's initial mistake?
2. How could Jules have handled the situation differently?
3. What factors might have caused Jules's complacency about safety?
4. What safety plan changes should Jules and the colleague institute?

VOICES FROM THE FIELD 2.3

Knowing Your Client's History and Avoiding Retraumatization

James Jackson

After hearing various accounts of an 8-year-old client's explosive behavior that was occurring in multiple settings, I was determined to assess firsthand the troubling pattern of outbursts. My consultation with client treatment team members and caregivers confirmed that a behavioral assessment was indicated, so I planned to conduct the assessment in the group room of the care facility where most of the outbursts had been reported. Upon my arrival, I received further information from the direct care facility staff, who indicated the client was required almost daily to be placed in the "time-out room," and that after her daily visit to the room, the client was able to maintain appropriate behavior the remainder of the day. This was my first time visiting this particular care facility, and I was pleased that video surveillance cameras located in the group rooms made it possible for me to conduct the assessment from a monitoring room without interfering with the client's normal routine. I used a structured behavioral observation template for recording client behaviors and noted the time each behavior occurred.

I began my observation as the client entered the group room with her aide. The client appeared appropriately engaged with the other group members and was completing her assigned task, which included a coloring activity, with minimal redirection. After an hour of observing, I was beginning to wonder if this might be one of those rare days when the client was able to consistently maintain appropriate behavior. However, just as the group was transitioning to a new activity, the client threw her crayon to the floor and said, "No, I wanna keep doing this." Her aide picked up the crayon and tried to hand it back to the client, but she refused and proceeded to crumple the paper she had been working on. The aide informed the client that she would now have to go to the "time-out room," took the client by the hand, and calmly led her from the group room into the next room with the designated time-out area; I watched with interest, as the care staff had indicated that this intervention was used almost daily.

I saw the aide lead the client (who seemed rather complacent) to a corner of the room to what appeared to be a lit storage area with a windowed door. Although the client now began to resist, the aide was able to force the client into the storage area and quickly shut the door, holding the doorknob tight so the client could not open the door. The client became visibly upset and began beating the plywood and cinderblock walls with her bare fists, yelling epithets at the aide while demanding to be released. "Not until you calm down," replied the aide, keeping both hands on the doorknob. Watching in shocked disbelief as this scene began to unfold on the video monitor, I recalled the client's history of repeated traumatic experiences, as well as seizures of unknown origin. Immediate safety concerns for my client flooded my mind, followed closely by liability concerns.

While the focus of this chapter has been the safety of the counselor, clients who initiate violent incidents are also at risk. They are, of course, at risk for harm to themselves while in the act of lashing out violently. Moreover, once a client has engaged in violence, the client will then be labeled violent, which may set off a self-fulfilling prophecy effect that will influence the client's behavior and treatment in the years to come. Now complete Activity 2.1 to practice thinking about safety precautions in various counseling settings.

ACTIVITY 2.1

In this chapter, we have reviewed a variety of safety concerns for counselors and have pointed out that, while some settings may have a higher potential for danger, counselors in all settings should be aware of safety issues and have safety policies and plans. In small groups, identify potentially dangerous situations in the following settings. Also, for each setting, identify some security policies and procedures that would improve safety.

- Pre-K–12 school counselor's office
- University counseling center
- Inpatient psychiatric hospital
- Sole practitioner private practice
- For-profit counseling agency
- In-home counseling program

SUMMARY

Working in crisis situations, counselors are at an increased risk for their personal safety and well-being. Proactive, preventive crisis planning and awareness of crisis safety protocols are vital. Collaboration, planning, and training with first responders and other crisis workers increase safety and improve crisis intervention services.

Counselors in solo practices are at a high risk for potential physical assault by violent clients. Private practitioners are cautioned

to screen new clients carefully, particularly if these potential clients have been referred by nonprofessional sources or if the clients have self-referred. In addition to screening, counselors should design their offices for safety: they should install panic alarms, remove objects from the office that could be used in an attack, and have an exit strategy in the event of a violent incident.

Counselors who work in inpatient and residential treatment facilities are at the greatest risk for assault from a client, and new professionals and young trainees in such facilities are more likely to be assaulted. The effect of these attacks can result in trauma, doubts about one's effectiveness as a practitioner, complacency, and burnout.

Research indicates that therapeutic support for counselors who have been assaulted is essential for the psychological well-being of the counselor and organization. Administrators should review safety procedures, should not tolerate even low levels of violent behavior from clients, should provide debriefings when safety issues occur, and should revise safety plans as needed with staff input.

CHAPTER

3 Ethical and Legal Considerations in Crisis Counseling

Paul F. Hard, Laura L. Talbott-Forbes, Lisa R. Jackson-Cherry, and Bradley T. Erford*

PREVIEW

Crisis planning is never perfect, but it does provide a framework for appropriate responses and decision making in crisis situations needing immediate responses. Prepared counselors well versed in crisis procedures and processes can provide ethical and effective interventions in all types of crisis conditions. This chapter examines the ethical and legal considerations related to preventive measures, federal legislation, sentinel court findings, and relevant applications of best practice regarding privacy matters in crisis counseling. Crisis counselors need to be particularly aware of state and federal laws and professional ethics that govern and advise clinical practice in crisis situations. The discussion focuses on prevention considerations; the *Health Insurance Portability and Accountability Act* (HIPAA); the *Family Educational Rights and Privacy Act* (FERPA); application of the *Tarasoff* decision; negligence and malpractice; documentation and record keeping; informed consent; confidentiality; termination in crisis intervention; and technological, spiritual, and multicultural considerations.

LEARNING OBJECTIVES

Upon completion of this chapter, readers will be able to:

3.1 Describe steps counselors can take to prevent ethical and legal problems.
3.2 Describe HIPAA, a federal law that protects sensitive mental health information.
3.3 Explain why counselors working in the higher education settings must comply with a different set of federal guidelines (FERPA).
3.4 Describe the effects of the *Tarasoff v. Regents* court case on the duties of counselors.
3.5 Describe special ethical, legal, and other considerations related to crises.

*We thank Mary L. Bartlett for her outstanding contributions to the first three editions of this chapter.

PREVENTION CONSIDERATIONS

Learning Objective 3.1 Describe steps counselors can take to prevent ethical and legal problems.

It is far more effective and economical to prevent a problem rather than to correct it. Primary prevention of ethical and legal issues in counseling is a far more effective strategy than the costly and distressing efforts to address an ethical or legal lapse after it has occurred. Counselors can focus on five areas to foster core prevention: (1) wellness, (2) continuing education, (3) peer supervision and consultation, (4) ethical decision-making models, and (5) an understanding of federal laws (discussed later in this chapter) and state laws applicable to crisis intervention. We explore each of these areas next. While there is no panacea to prevent ethical or legal violations, when these five areas are synthesized into the counselor's professional worldview, best practices follow and act as a shield for both client and counselor.

Wellness

The American Counseling Association (ACA) Task Force on Impaired Counselors (Lawson & Venart, 2005) observed that counselors working in disaster or crisis circumstances are particularly vulnerable to secondary trauma, vicarious trauma, and compassion fatigue. Because such a compromised condition may leave the counselor vulnerable to ethical or legal lapses, attention to the well-being of the counselor is vital as a preventive measure (Malinowski, 2014).

A seminal work by Pearlman and MacIan (1995) noted 10 helpful strategies for crisis counselors to use to promote and maintain wellness: (1) discussing cases with colleagues; (2) attending workshops; (3) spending time with family or friends; (4) travel, leisure time, hobbies, and movies; (5) talking with colleagues between sessions; (6) socializing; (7) exercising; (8) limiting caseloads; (9) developing one's spiritual life; and (10) receiving supervision. Prevention strategies also helped alleviate stress-related problems among health care professionals during the COVID-19 pandemic (Heath et al., 2020), demonstrating that wellness and self-care are vital to counselors.

The care counselors provide to others is only as effective as the care they give themselves. While self-care and wellness practices are imperative for crisis counselors, these practices are not yet mapped and incorporated into the Council for Accreditation of Counseling and Related Educational Programs (2024) standards. In addition, challenges exist related to the underlying assumption that counselor educators are themselves models of self-care and wellness for counselors-in-training (Moate et al., 2016). For example, counselor educators navigating experiences of COVID-19–related stress struggled to balance their roles as educators, professional counselors, and supervisors, often neglecting their own self-care (Harrichand et al., 2021). In addition, clinical and nonclinical staff were at greater risk of mental distress because they worked longer hours, and the potential of becoming exposed to COVID-19 had a negative impact on morale. These conditions led to stress, anxiety, burnout, and depressive symptoms and an increased need for sick or stress leave (Xiaoyun & Fenglan, 2020). This affected the care of patients, the health of counselors, and the delivery of mental health services during the crisis.

Continuing Education

Continuing education is required by certifying and licensing bodies to help counselors' practice remain both competent and current. Many such entities also require that a portion of the continuing education be related to ethics to ensure that counselors continue to be versed in current ethical issues and decision-making models. Counselors should participate in continuing education to effectively compare their professional behavior with the standard of care and routinely review professional codes of ethics as part of their ongoing personal education. Counselors practicing in isolation without peer contact are at higher risk for impairment, and in-person continuing education is an important component in counselor wellness (Hard, 2020). This need for peer contact also includes supervision and consultation.

Peer Supervision and Consultation

Peer supervision and consultation are essential to ethical instruction and practice (Corey et al., 2019; Remley & Herlihy, 2020), and counselors should prepare for crisis situations by preestablishing sources of in-person and online supervision and peer consultation. Engaging in peer consultation when confronted with ethical concerns exposes those concerns to colleague input and enhances protection for both the counselor and client. Avoiding conversations related to ethical dilemmas out of shame or fear often results in negative outcomes. By contrast, pursuing open and honest dialogue about client challenges reduces the likelihood of negative outcomes that may occur when one works in isolation. When counselor educators and supervisors neglect their own self-care and wellness practices, their ability to model ethical practices and work effectively is compromised.

Remote supervision requires mindfulness of power differentials and technological communication nuances. While the supervision relationship is complex under normal circumstances, dynamic core elements include the relationship between supervisor and clinician, rapport building, and boundary negotiation. Experienced clinicians understand the limits of their roles, responsibilities, and accountability and the purpose of the counseling relationship. More challenges occur during individual and group online communication of the complex nature of assessing and making decisions about another person's condition.

One benefit of the pandemic transition to digital platforms was reduced travel time; this was especially beneficial for people located in rural areas. Cameron et al. (2020) found that remote supervision via videoconferencing provided increased confidence, new learning strategies, and reduced feelings of isolation for clinicians in rural areas. Miller (2020) found that telephone and e-mail discussions enhanced the supervision experience.

Online platforms provide advantages for some counselors, especially when distance and timing make supervision unfeasible. The online process may allow for greater selection among supervisors. Online platforms also allow real-time reviewing of information, enhancing opportunities for reflection, dialogue, and professional preparation.

The main negative of online supervision is difficulty managing and being fully attentive to counselors who perform poorly. Poor performance may raise ethical and safety issues because it is difficult to observe weak spots and inappropriate procedural

issues in a virtual context. Importantly, online counseling potentially increases the possibility of missed diagnoses. Counselors have the ethical duty to transparently communicate about the differences in in-person and online delivery by attending to issues like Internet buffering, frozen screens, loss of connections, and interpretation of multidimensional data that assist with the flow of communication and interactions. For example, the inability to see body language, potential for disrupted eye contact, use of silence as a technique, challenges to being fully present, altered voice characteristics, and physical appearance can contribute to the lack of fluency in online interactions.

At least one face-to-face meeting assists in establishing rapport with clients and supervisors alike, allowing for the negotiation of boundaries. Supervisors may sense that counselors they are assisting may be missing some unmet needs or have formed blind spots in the therapeutic relationship that should be addressed. As we discuss further in this chapter, one's confidence with using an online platform, technology issues, consent, privacy, and confidentiality have placed the counseling field in uncharted territory.

Ethical Decision-Making Models

The *American Counseling Association (ACA) Code of Ethics* (2014) provides guidelines on key areas of counselor decision making. However, the code provides little direction on the process counselors should follow when making ethical decisions. One way that counselors practice soundly is by rooting their ethical decisions in a theoretical model. In fact, section I.1.b of the code recommends that counselors "use and document as appropriate an ethical decision-making model" (p. 19). The code also outlines some components of an effective decision-making model.

There are a variety of ethical decision-making models a counselor can use. The counselor should determine which model to use consistently based on personality, values, expectations, the treatment setting, client base, and available time (Corey et al., 2019; Remley & Herlihy, 2020). Regardless of the model chosen, counselors should apply the core principles of autonomy, nonmaleficence, beneficence, justice, fidelity, and veracity when making ethical decisions.

Sentinel work by Forester-Miller and Davis (2016), titled *A Practitioner's Guide to Ethical Decision Making*, is the basis for the ethical decision-making standards of the *ACA Code of Ethics* (2014, section I.1.b). The practitioner's guide outlines a seven-step decision-making model: (1) identify the problem, (2) apply the *ACA Code of Ethics*, (3) establish the parameters of the dilemma, (4) generate all possible courses of action, (5) consider the consequences of all options and select a course of action, (6) evaluate the course of action selected, and (7) implement the course of action.

A sampling of models provided by Corey et al. (2019), Remley and Herlihy (2020), and Sheperis and Sheperis (2015) includes the following:

- ***Beneficence model:*** Uses an A-B-C-D-E worksheet that includes the steps of assessment, benefit, consequences and consultation, duty, and education.
- ***Feminist model:*** Creates a gender-equal ethic that is based on nonsexist moral principles such as holistic, intuitive, emotional, compassionate, personal, and contextual processing styles. The model calls for involvement of the client at every stage of the decision-making process.

- ***Social constructionist model:*** Uses an interactive process that places the decision in the social context and involves negotiating, conceptualizing, and possibly arbitrating.
- ***Transcultural integrative model:*** Includes cultural factors counselors can use in a variety of settings with client populations. It emphasizes principles that link all cultures, such as altruism, responsibility, justice, and caring.

Any of these ethical decision-making models can support counselors in making effective decisions.

While no two models are the same, all include some form of the following constructs: (1) identify the problem, (2) check your feelings, (3) identify potential issues (e.g., cultural, spiritual, familial), (4) review relevant professional codes, (5) know laws and regulations, (6) involve the client in the process, (7) obtain peer or supervisor consultation, (8) consider courses of action, (9) consider the implications of decisions, (10) choose a course of action, and (11) follow up with a supervisor, legal counsel, and the client.

It is important to understand that a counselor's decisions must also always abide by the parameters set forth by state and federal legislation that govern mental health practice. Now respond to the ethical dilemma provided in Activity 3.1.

ACTIVITY 3.1

Using the seven steps in the *Practitioner's Guide to Ethical Decision Making* (Forester-Miller & Davis, 2016) noted previously, address the following ethical issue:

> *Ryder has just entered treatment with you for substance use disorder. Ryder reports that a previous therapist used drugs with Ryder; the case manager who brought Ryder to the session confirms this. The client is reluctant to "hurt" the old therapist, and the case manager doesn't want to "rat" on a coworker.*

How would you handle this based on the Forester-Miller and Davis model? Search the web for an alternate model and address the same case with it. Describe how your outcomes or processes are different using the alternate model.

HEALTH INSURANCE PORTABILITY AND ACCOUNTABILITY ACT

Learning Objective 3.2 Describe HIPAA, a federal law that protects sensitive mental health information.

The *Health Insurance Portability and Accountability Act (HIPAA)* of 1996, also known as the *Kennedy-Kassebaum Bill*, is federal legislation that was implemented to address numerous perceived shortcomings in the management of information in the health care industry. It was the first federal legislation to protect automated client records, and it mandated that all electronic transactions include only HIPAA-compliant codes, provide security provisions, and require national provider identification numbers. The current version of the legislation covers a wide range of areas that impact both the health care industry and patients (HIPAA, 2011; Hebda et al., 2019).

What Is HIPAA?

In the 1970s and 1980s, the American public perceived that health care information was not controlled and protected by the health care industry to the degree that it should be, in view of the increasing ability during those years for data to be compiled, manipulated, and used to negatively affect individuals and families. As a result of several high-profile security breaches and misuse cases and the resulting pressure on Congress, the legislative branch created health care information handling and protection standards of their own and communicated to the health care industry that these rather restrictive standards would be implemented if the health care industry did not solve the problem on its own.

The health care industry subsequently promulgated standards for information handling, management, and control that met congressional approval. Congress enacted these standards and HIPAA (Public Law 104-191) went into effect in 1996, including four areas of legislation: (1) privacy requirements (the specific rules for which would be subsequently constructed), (2) electronic transactions, (3) security requirements, and (4) national identifier requirements (Corey et al., 2019; Hebda et al., 2019; Pozgar, 2018; Remley & Herlihy, 2020; U.S. Department of Health and Human Services [DHHS], 2002).

However, the version of HIPAA that became law in 1996 did not include privacy rules. Congress instructed DHHS to dictate privacy rules for the health care industry within 3 years. The 3-year period was provided to give the health care industry a chance to develop privacy rules for their own profession, submit the privacy rules to Congress, and obtain congressional approval. This did not occur, and in the absence of such legislation, DHHS followed the requirement of the 1996 act and implemented standards for privacy of individually identifiable health information, referred to as the *Privacy Rule*, which became effective in 2003. A few of the privacy provisions are as follows:

1. Patients must be able to access their records.
2. Patients have the right to request and/or make corrections to their records.
3. Patients must be informed of how their personal information will be used.
4. Patient information may not be used for marketing purposes without prior consent.
5. Health insurers or providers must designate a privacy officer and train employees (Pozgar, 2018).

The Privacy Rule is still in effect and can be located at the HIPAA website of the DHHS Office for Civil Rights (Corey et al., 2019; Hebda et al., 2019; Remley & Herlihy, 2020).

The Privacy Rule seeks to balance the concept of individual privacy against the need for medical professionals to have access to information to best do their jobs. It establishes that the right to privacy about medical information is not absolute and therefore puts the onus on designated health care providers to protect the information they collect, use, and transmit. The Privacy Rule and other aspects of HIPAA govern information that contains personal identification categorized as protected health information. The protected health information must be treated according to the standards and procedures established by HIPAA whenever it is created, transmitted, or received in any form whatsoever: electronic, paper based, or by means of oral transmission. In short, if information contains individually identifiable material, it is considered protected health information and is fully controlled by HIPAA (Pozgar, 2018; Remley & Herlihy, 2020).

Those who collect, use, or transmit protected health information and who are covered under the requirements of HIPAA are referred to as *covered entities*. HIPAA defines *covered entities* as health plans, health clearinghouses, and health care providers. *Health care provider* is sometimes used to refer to covered entities. *Health care provider* refers to a wide variety of health professionals (including counselors) in private and public practice who provide medical care and public health services—such as primary care, disease diagnosis, and public education—and are paid by a third party (Wilensky & Teitelbaum, 2023). HIPAA defines many terms that relate to the health care arena and establishes standards through the Privacy Rule that define how individuals and organizations in health care are to collect, use, transmit, and handle protected health information.

Counselors also need to be familiar with the *HIPAA Security Rule*, which took effect in April 2005. The Security Rule requires that covered entities maintain three safeguards—administrative, physical, and technical—so that electronic protected health information remains confidential, secure, and integral. An example of an administrative safeguard is the clear identification by an organization of which employees or classes of employees can have access to protected health information. An example of a physical safeguard is the placement of workstations of those who have access to protected information away from high-traffic areas, with computer monitors shielded from direct view of the public. This safeguard has affected the way in which counseling settings are architecturally designed and how workstations are set up. An example of a technical safeguard is ensuring that information systems housing protected health information are secured against intrusion (Pozgar, 2018). See Case Study 3.1, which considers a situation where protected information was accessed and misused. Also read Voices from the Field 3.1.

CASE STUDY 3.1

The Misuse of Protected Information

River had to pick up her 14-year-old, Rowan, early from school due to a plumbing problem that closed the school early. Since River, the custodian of client records for an outpatient mental health center, was scheduled to work for several more hours, River allowed her daughter to complete her homework at the work desk near her. Since River had to run charts to clinicians at various times, Rowan was left unsupervised. Bored, she began browsing through patient charts that had been left on the desk and through client records on River's computer (Rowan gained access with the password, posted inside River's desk drawer). Rowan even went as far as to prank-call clients who had been tested for HIV, pretending to be with the clinic and reporting false results. One client died by suicide after receiving Rowan's call that she was HIV positive.

Discussion Questions

1. List the HIPAA violations committed in this case study (based on an actual 2003 incident).
2. Who is liable in this case? Why?

VOICES FROM THE FIELD 3.1

HIPAA and Social Media

Gina Palmer

HIPAA and social media do not mix. With the popularity and pervasiveness of social media in our everyday lives, it is easy to see how patient privacy can be breached without even realizing it. My rule is to never post messages or comment about work in any way; never post pictures from work and never "friend" any current or past patients via social networking sites. For any other related concerns, the normal client boundaries and ethics of disclosure still apply.

How HIPAA Affects Crisis Counseling

Crisis counseling differs from the kinds of services that are typically provided by health care workers. Crises such as hurricanes, tornadoes, and homicides typically occur quickly and often in unanticipated ways, so counselors must respond immediately to stabilize the situation and provide a way for people to move forward. There may be limited time to obtain HIPAA-required releases when critical care is needed, or it may be impossible to obtain such releases because of severe injury, unavailability of family members, or other unforeseen consequences of the disaster. According to DHHS (2022b, 2022c), HIPAA is not intended to interfere with the provision of emergency medical care associated with declared emergencies such as hurricanes, and covered health care providers may exercise their professional judgment and act if such actions are in the best interests of the patient.

Actions permitted by health care providers may include disclosure of individually identifiable medical information to government officials at the local, state, or federal level; police; first responders; public health officials; or anyone whom the health care providers deem necessary to best serve the patient. In any case, the federal government has the authority to waive sanctions and penalties associated with violations of the Privacy Rule, even in cases where a public health emergency is not declared (DHHS, 2022b). At the same time, it is important for crisis counselors to work under the direction of the point agency (e.g., Red Cross, Green Cross, Federal Emergency Management Agency) during a crisis. Well-intentioned counselors who make decisions that counter the point agency may do more harm in some circumstances.

For work with clients who are a serious threat to safety, the HIPAA Privacy Rule allows disclosure of protected health information under the "Serious Threat to Health or Safety" provision. Release of information to anyone whom the health care provider reasonably believes may lessen the threat to health or safety (including the target of the threat) is allowed; however, in the case of a threat to health and safety, release of protected health information to law enforcement is allowed only when the information is needed to identify or apprehend an escapee or a violent criminal (DHHS, 2022a).

In cases where a crime has occurred or where law enforcement officials are investigating a suspected crime, covered entities are allowed to disclose protected health information under the "Law Enforcement Purposes" provision of HIPAA. This provision sets out conditions in which disclosure of protected health information may—and, in some circumstances, must—be made (DHHS, 2022a).

Disclosure of protected health information to law enforcement is allowed in the following circumstances:

- In response to a court-ordered warrant, subpoena, or summons
- In response to an administrative or investigative demand by law enforcement
- When a request for protected health information is limited to information required to locate a suspect, fugitive, material witness, or missing person
- To apprehend a perpetrator of a crime against the member of a covered entity's workforce
- To apprehend a violent criminal
- To report child abuse or neglect (no consent required)
- To report adult abuse, neglect, or domestic violence (if the individual agrees or it is required by law)
- To report the death of an individual when death is suspected to be a result of criminal conduct
- To report a medical emergency
- To comply with federal security activities associated with national security
- When a request is made by a correctional institution or official having lawful custody of an individual

Professional counselors should distinguish *permissive* aspects of the Privacy Rule (i.e., "may" be released) when other statutes such as state privilege laws may be more restrictive. That is, while the Privacy Rule may indicate conditions in which protected health information can be disclosed, other statutes, such as state privilege laws and organizational policies, do not allow disclosure. In such cases, the client's private health information should be protected. See Case Study 3.2, which considers how the HIPAA Privacy Rule applies to the sharing of protected health information. Also see Case Study 3.3, which examines a protection of privacy issue.

CASE STUDY 3.2

HIPAA and Protected Health Information

Kalani is a licensed professional counselor at a community mental health center (CMHC). A physician at a local hospital requested a substance use evaluation on a current patient. No cooperative agreement exists between the hospital and the CMHC. When Kalani arrives at the hospital, the charge nurse provides the patient's chart and invites Kalani to sit in the records area to review the patient information prior to seeing the patient. Kalani sees the patient, performs the evaluation, obtains all the necessary releases, and leaves documentation with the physician. While there, the physician asks Kalani to see another client, and the charge nurse provides the other client's chart.

Discussion Questions

1. What, if any, violations of the HIPAA Privacy Rule have occurred here?
2. If violations were committed, who is liable?

CASE STUDY 3.3

Protection of Privacy

Rose is a person transitioning from male to female. She is living in a shelter after her home was destroyed by coastal flooding. Rose disclosed her transgender status to the shelter counselor and requested special accommodations for privacy. The counselor left Rose's documentation visible on the desk while tending to other clients. A shelter worker inadvertently saw the note and, attempting to be of help to the counselor (and to Rose), mentioned the special needs request to other shelter workers. Rose's circumstances were then made known to other shelter residents, resulting in objections from the community and embarrassment to Rose.

Discussion Questions

1. How are privacy regulations for client privacy to be observed in disaster shelter circumstances?
2. What minimum expectations for privacy should a client have in a disaster shelter?
3. In circumstances of a breach of information, what steps should be taken to sanction these actions?
4. Who should be sanctioned and how?
5. What can be done to minimize the damage of a breach of privacy?

FAMILY EDUCATION RIGHTS AND PRIVACY ACT

Learning Objective 3.3 Explain why counselors working in higher education settings must comply with a different set of federal guidelines (FERPA).

In 1974, Congress passed the *Family Educational Rights and Privacy Act (FERPA)*, also known as the *Buckley Amendment*, prompted by concerns about privacy violations and the inclusion of immaterial comments and personal opinions in educational records (Rainsberger, 2019; Sutton, 2015). Confusion regarding HIPAA and FERPA continues to be of concern as institutions of higher education and governmental partners struggle to strike a balance between the two federal laws. This is of particular concern for counselors who work in an educational environment and are providing direct mental health services. Counselors who work in an educational environment should remain current in their understanding of federal privacy regulations, the content to be disclosed in the educational record, and the need to document student mental health needs (Goodwin, 2016; Hope, 2022; U.S. Department of Education, 2022; DHHS, 2021).

What Is FERPA?

Essentially, FERPA (20 U.S.C. § 1232g; 34 CFR Part 99) gives parents certain rights to their child's educational records until the child is 18 years of age (an "eligible student"). After this point, the right to privacy regarding the contents of the educational records is provided to the eligible student, thus denying parental access without consent (U.S. Department of Education, 2022). All schools, public or private, that receive federal funding must follow FERPA or face the loss of federal funds (Edwards, 2018). According to

the DHHS and the U.S. Department of Education (2022), a parent or an eligible student has the right to inspect and review the student's educational records. Additionally, the parent or eligible student who believes an educational record to be inaccurate may ask the school to amend the record. If a school refuses to amend an educational record, the parent or eligible student can request a formal hearing and may place in the record a statement about the information being contested.

Schools must obtain written permission to release information; however, FERPA allows the release of information in certain situations—for example, when a student transfers to another school and there is a legitimate educational interest or when financial aid is being requested, a judicial order has been issued, or there is an emergency. Parents are to be notified when records are transferred to other schools and may receive copies of those records upon request (Edwards, 2018). Under FERPA, colleges may release educational records, such as grades and financial records, to parents of college students claimed as dependents on parental tax forms. The release of student records to parents occurs either when the parent shows tax record proof that the student is a dependent or when the student signs a waiver allowing the release of records to parents (Elliott et al., 2014). See Case Study 3.4, which focuses on the statutes that apply to a student's counseling records.

CASE STUDY 3.4

Student Counseling Records

Wylie, a student in a clinical mental health counseling program, is completing an internship in a mental health center. The center has an outreach program at the local high school, and its counselors have been given space to work in the school's counseling office. Wylie has been working with Skye, a 10th-grade student who was referred for trouble concentrating and declining grades. Skye's teacher and parents are very interested in Skye's progress in counseling and check in with Wylie frequently. After three sessions, Skye reports depression since an older sibling died by suicide 4 months ago while at college. Skye's teacher is heading Wylie's way for an update.

Discussion Questions

1. Are Skye's counseling records covered by HIPAA? By FERPA? Or both?
2. Do any factors complicate this case regarding access to counseling records by Skye's parents, Skye, or school faculty?

Prior to FERPA, educational institutions could be quite lax about how they communicated student information. In fact, Senator James L. Buckley of New York was motivated, in part, to introduce this legislation due to such abuses of confidential material (von Feigenblat et al., 2015). Schools were known to reveal contents of student records to parties who had no educational interest or need to know. FERPA has served to advance professionalism in educational institutions, while at the same time securing students' rights to privacy and confidentiality.

FERPA contains a provision that exempts certain personal records. Records that are exclusively in the possession of the counselor and are not revealed to others are excluded from FERPA (Elliott et al., 2014). These records include those made by school personnel, such as counselors, who keep such records in their "sole possession." Due to this exemption, counseling case notes are not considered part of the educational record and do not have to be revealed (Rainsberger, 2019). Now read Voices from the Field 3.2 and 3.3.

VOICES FROM THE FIELD 3.2

Protecting Confidentiality During a Crisis

Cindy Wiley

Your first priority as a counselor is to protect your client—in this case a school-age student, Shiloh, has come to you in crisis, trusting in you to alleviate pain while guarding and maintaining confidence. When a student presents in such a state, it is hard to remind that student, and yourself, that while what you talk about is confidential, there are exceptions to that confidentiality. I think most counselors are afraid that the counselor–student relationship, which is paramount, will be ruined even if confidence is appropriately breached for the safety of the student. If worded in a manner that adequately portrays your concern and need to involve other resources, most students will understand. What students will ultimately remember is that you genuinely care for them and their well-being. In a state of crisis, most students are truly asking for help. Your job is to make sure that you "circle the wagons" to support them.

Counselors in any setting have the responsibility to convey to clients the limits of confidentiality in the beginning of any counseling relationship rather than when an emergency is experienced where the limits may be broken. Providing this information at the beginning of the relationship allows counselors to establish boundaries and shared responsibility for information relayed in sessions. The *ACA Code of Ethics* (2014, A.2.a, Informed Consent) states:

> Clients have the freedom to choose whether to enter into or remain in a counseling relationship and need adequate information about the counseling process and the counselor. Counselors have an obligation to review in writing and verbally with clients the rights and responsibilities of both counselors and clients. Informed consent is an ongoing part of the counseling process, and counselors appropriately document discussions of informed consent throughout the counseling relationship.

Therefore, even in circumstances where a client is unable to give consent (e.g., due to detox, resistance), the information should be addressed. In the event of a refusal or inability to sign, a counselor can indicate "client refused to sign" or "client was unable to sign." This shows you still met your professional responsibility.

VOICES FROM THE FIELD 3.3

The Importance of Crisis Training for University and College Staff

Yulanda Tyre

Accidental death is one of the leading causes of mortality among college students 18 to 24 years of age. These occurrences can have a devastating ripple effect for a campus community. During my first year of work as a college counseling center director, the student body experienced the loss of a popular athlete in a tragic motor vehicle accident. The incident happened on a summer night, allowing students to pour out onto the university quad to grieve and express condolences together. It was immediately evident that many students experienced the event as a crisis. The request for counseling services among students increased, along with department requests for counseling staff to speak

to small groups of students and faculty on topics of death, grief, coping skills, and resilience in the days following the event.

Mortality rates among traditional-age college students support the need for effective crisis intervention planning and training among counseling center staff and university professionals who support the emotional well-being of students. Crisis planning and implementation provides opportunities for collaboration with trained places of worship, doctors, independent professional health care providers, and organizations like the Red Cross, National Alliance on Mental Illness, public health departments, and other community organizations. Training is crucial to addressing the negative short- and long-term emotional and biological effects of a crisis for a college-age student during what can be a pivotal stage of development.

How FERPA Affects Crisis Counseling

According to the U.S. Department of Education (2020a), an emergency situation creates an exception, and schools may release information from general, medical, or mental health records to protect the health and safety of others. It is important to note that this exception is only for the time of the emergency and is not blanket consent to release student information. The information may be released to appropriate individuals who are tasked with protecting health and safety, such as law enforcement officials, medical personnel, and public officials. Law enforcement officials hired by schools are considered school officials and therefore have an "educational interest" and are permitted to view records. Threatening remarks made by a student and overheard by school personnel can be reported to appropriate officials because such remarks are not considered part of the educational record.

These policies apply to higher education settings as well, with the following additions. Regarding disciplinary actions and rule violations, the results of a disciplinary proceeding may be released to an alleged victim even if the perpetrator has not been found guilty of violating rules or policies (U.S. Department of Education, 2020c). If a perpetrator has been found guilty of violating rules or policies, the institution may disclose the results of a disciplinary hearing to anyone. Most colleges and universities have their own campus police; investigative records of campus law enforcement are not subject to FERPA and may be released to outside law enforcement without student consent. Now read Case Study 3.5, which considers whether HIPAA or FERPA were violated when student information was released by a university.

CASE STUDY 3.5

FERPA Violation or Not?

A female student attending a public university in Oregon accused three players from the university basketball team of raping her in March 2015. One of her attackers had been suspended from a previous college team over similar allegations. The female student received counseling at the university's counseling clinic. Nothing was done to the three athletes until May 2015 (well after basketball season), when university administration removed them from the basketball team and expelled them from the university. When the

female student learned that the coach had recruited one of the athletes despite his history of sexual assault, she filed a suit against the university for negligence and emotional distress. The university accessed the female student's counseling records and sent them to its attorney. University officials contended that since the female student went to the health clinic, the records belonged to the university and further held that the claim of emotional distress (a medical condition) entitled the school to her medical records under FERPA.

Discussion Questions

1. Did the university violate FERPA or HIPAA?
2. What facets of this real-life case are operative in the decision of whether a violation has occurred?

In situations concerning health and safety, higher education institutions may disclose educational records to a parent if the student is involved in a health or safety issue or is under 21 years of age and has violated the school's drug and alcohol policies. Also, according to the U.S. Department of Education (2021), "a school official may generally share information with parents that is based on that official's personal knowledge or observation of the student." Therefore, in situations where a student's behavior involves the health and safety of self and others, school officials can communicate with parents. In fact, a school official may report concerns about student behavior to anyone, although it is wise to make such reports only to campus personnel, such as administrators, counselors, or campus police, who can intervene appropriately with the student (Chan, 2018).

For mental health practitioners working with college-bound students and families prior to enrollment in an institution of higher education, transition work should help the student and parents in the early college selection process become familiar with mental health and supportive services at each college being considered. If, for example, your client is in addictions recovery, the client should know whether the institution has a Collegiate Recovery Community and whether the student can connect with others in recovery to maintain sobriety (Brown et al., 2018). Even universities within the same geographic region can have vast differences in the types of services provided. The proactive counselor will participate with the student and family in reviewing the college for services that best match the mental health needs of the young adult in transition to a university setting (Chan, 2018).

All institutions of higher education should address changes in a student's mental health status and provide a clear path for advocacy that identifies and supports students at risk. Student support programs, mental health interventions, and resources should be available to address these concerns, especially during a crisis (Burke, 2018). Students should be proactive about their mental health, particularly during a pandemic or other crisis, and should seek help when feeling mentally distressed. Violence, substance use, suicide, and other at-risk behaviors bring national attention to mental health problems on campus, and institutions must offer a diverse repertoire of services to help address student needs. For example, services can be offered by counseling centers, health centers, or integrated programs. The real challenge is keeping up with demand for services on campus (Chan et al., 2015). Now read Voices from the Field 3.4.

VOICES FROM THE FIELD 3.4

The Administrative Struggle with College Student Mental Health

Mary Ann Bodine Al-Sharif

Even before COVID-19 was a reality, many university administrators were struggling with the ability to find resources to provide mental health services for students. The COVID-19 pandemic only heightened a tragic awareness of what I already knew about the state of college student mental health: It is failing. For the most part student mental health concerns do not develop just because a student is in college. Many students were dealing with these concerns long before they entered the university doors. Poor mental health is a societal issue that presents in higher education; however, higher education is ill prepared for the demand. Already strained budgets have been further impacted by difficulty with recruitment and retention due to the pandemic causing budget cuts to higher education. It is without question that students will continue to present in mental health crises on college and university campuses, but higher education is quickly running out of resources to put a bandage on this growing social issue. Administrators can only work with the resources they are provided, so as funding runs low and community resources become limited, students suffer.

The Application of HIPAA and FERPA to College and University Student Records

In higher education institutions, the interaction between HIPAA and FERPA can be very confusing and frustrating to parents, staff, and faculty members. Wise et al. (2011) explained that HIPAA contains an exemption that excludes FERPA-covered records from HIPAA regulations. However, treatment records such as university student counseling case files are exempt from FERPA but may be covered by HIPAA. The interplay between HIPAA and FERPA has yet to be fully understood, and higher education institutions currently interpret HIPAA and FERPA in different ways. Figure 3.1 summarizes information from Wise et al. on when HIPAA and FERPA are invoked, overlap, and do not apply in university training clinics, specifically. FERPA was never intended to block the information flow between higher education institutions and others concerned about

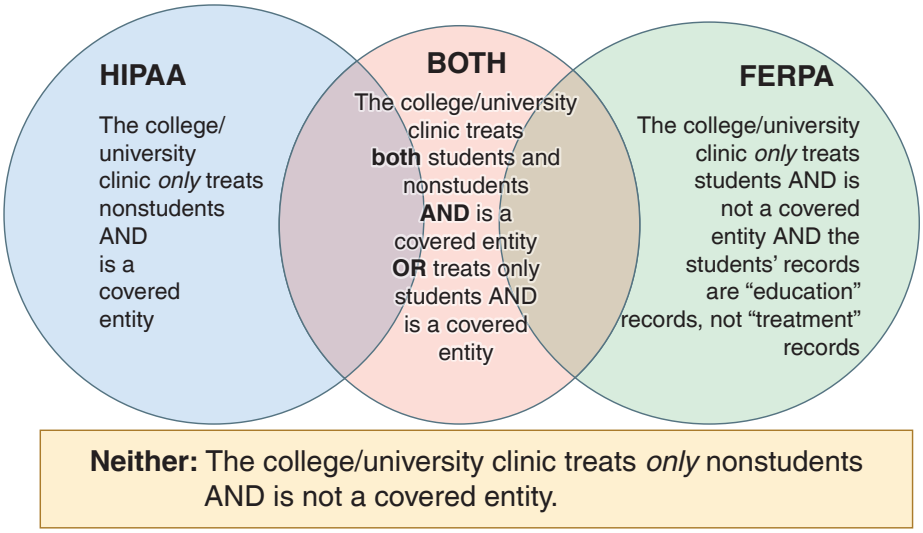

FIGURE 3.1 When do HIPAA, FERPA, both, or neither apply to college/university training clinics?

student health and safety. Therefore, faculty, staff, and administrators who are troubled about the welfare of a student may contact the student's family or other entities and express general observations about health and psychological concerns, while protecting the privacy of the student's medical or mental health records (Chan et al., 2015).

College and university settings focus on providing prevention services to students and, as a result, students are a captive audience for counseling services as colleges and universities strive to provide easily accessible, quality mental health services (Chan, 2018). Many college and university students needing mental health services do not seek assistance for a variety of reasons (Eisenberg et al., 2015). This finding is problematic, as suicide is the second-leading cause of death among college-age students (American Association of Suicidology, 2021; Eisenberg et al., 2015).

The convergence of HIPAA and FERPA makes it difficult to determine when these statutes apply to student clients, how they affect student-client rights, and how they affect counselors and their supervisors. It is important for university clinics to follow all HIPAA requirements. Doing so is particularly confusing at university counseling training facilities and requires even more diligence because clients are also often students at the university (Chan et al., 2015). Counselors who inadvertently transfer a client's mental health record (e.g., clinical treatment notes) to an educational record or file without the client's consent have failed to maintain the confidentiality of the mental health record and increased who can access the record. Therefore, this unfortunate transfer could mean that the counselor and supervisor violated federal law and caused significant harm to the client. Confidentiality and record keeping are covered in more detail later in this chapter. Complete Activity 3.2, which asks you to apply FERPA as a school counselor. Then complete Think About It 3.1.

ACTIVITY 3.2

It is not uncommon for school counselors to contact a parent once it is determined that the child is a potential threat to self. Consider the following example:

COUNSELOR: Hello, Mrs. Smith. This is Ms. Collins. I'm your daughter Jill's school counselor. Today your daughter's teacher, Mrs. Ramirez, found a note Jill had written that suggests she is feeling suicidal and may have a plan to kill herself. I met with Jill and believe she is having passing thoughts of suicide but do not believe she has a current plan. It is school policy for me to contact a parent to pick up the child and make a referral for further assessment.

PARENT: I can't come right now. I'm in the middle of a meeting. Jill has been talking about suicide for several weeks since her father left us. I think she is just seeking attention.

COUNSELOR: Yes, Jill mentioned she is feeling very sad about her father leaving, and this is why she is considering suicide as an option. We take all reports of potential suicide seriously to ensure student safety and require that the student be seen for follow-up before returning to school. I realize this is inconvenient, but I truly believe your daughter is in emotional distress.

PARENT: Thanks for your concern, but I really don't think it's that serious and I simply can't leave work right now, especially since my husband left. I can't risk losing my job.

Discuss how the conversation in this scenario will continue from this point. Consider who you might also need to speak with if the parent continues to resist. Reflect on the limitations and latitude FERPA allows and how to proceed.

THINK ABOUT IT 3.1

Think about the *ACA Code of Ethics*, HIPAA, and FERPA. What potential conflicts immediately come to mind when you consider working in a crisis or disaster situation? How might these conflicts change as the crisis becomes less acute? How might you address these conflicts?

THE *TARASOFF V. REGENTS* DECISION

Learning Objective 3.4 Describe the effects of the *Tarasoff v. Regents* court case on the duties of counselors.

In the *ACA Code of Ethics* (2014), counselors are charged with recognizing trust as an important component of the counseling relationship, and counselors are obligated to facilitate a trusting relationship through the maintenance of client confidentiality. However, disclosure is required "to protect clients or identified others from serious and foreseeable harm or when legal requirements demand confidential information must be revealed" (section B.2.a). This standard was adopted as part of counseling practice after the landmark California Supreme Court case of *Tarasoff v. Regents of the University of California* (1976).

As context for the *Tarasoff* case, in 1969 Prosenjit Poddar was seen by a psychologist on a voluntary outpatient basis at the University of California, Berkeley. Poddar confided his intent to kill a woman he claimed to be his girlfriend, a woman the psychologist surmised to be Tatiana Tarasoff, when she returned from Brazil. Alarmed by the disclosure, the psychologist contacted the campus police and then began proceedings to have Poddar committed for a psychiatric evaluation. In the interim, police picked Poddar up and questioned him. The police found Poddar to be rational, and he did not demonstrate risk in their presence. He promised not to have contact with Tarasoff, so the police released him. The psychologist followed up on his concerns with a letter to the campus police chief in which he again expressed his concern; however, the supervisor of the psychologist requested that the letter be returned, ordered all case notes destroyed, and ordered that no further action be taken.

Poddar did not return to see the psychologist, nor was he committed or evaluated by any additional mental health professionals. Furthermore, no one contacted Tarasoff or her family to alert them that a threat had been made. Two months later, Tarasoff was stabbed to death by Poddar; her parents sued the University of California Board of Regents (Corey et al., 2019; Remley & Herlihy, 2020). Initially, a trial court ruled that the university was not liable because Tarasoff was not the patient in this case, and therefore it had no duty of care toward her. Upon appeal, however, the California Supreme Court ruled there are some circumstances in which a therapist should break confidentiality, specifically when such a disclosure is necessary to avert danger to the client or others. This act of breaking confidentiality when disclosure is necessary to avert danger is known as the *duty to warn*.

What Was the Effect of *Tarasoff v. Regents*?

Part of what makes this case and the duty to warn/protect complicated is that there have been many interpretations of this case ruling. An examination of the various state laws demonstrates that how and when a counselor should break confidentiality differs depending on the circumstances and, since many states depend on court rulings rather

than statutes to guide the actions of a counselor, this issue is not clear across all jurisdictions (National Conference of State Legislatures, 2022). Additionally, many legal experts are unable to give specific guidance as to when counselors should alert the police or an identified victim regarding a threat to a person's life. While some courts have limited the application of the duty to warn to situations in which victims are identifiable, subsequent decisions have indicated that the duty to warn extends to unknown victims, which further complicates the matter. Another confusing part of this ruling involves the terms *duty to warn* and *duty to protect*, which are often used interchangeably by professionals.

Legal definitions of the terms *duty to warn* and *duty to protect* are established by state legislation and state court rulings, so these definitions vary from state to state. Counselors must be aware of the laws applicable to the states in which they are licensed and in which they intend to practice. Generally speaking, the duty to warn and the duty to protect represent degrees of what is essentially the same duty. First, the duty to inform someone of a danger or hazard with the idea that informed persons will take action to protect themselves (this is the duty to warn) and, second, the duty to protect someone by taking an action that reduces the danger or hazard to that person directly (this is the duty to protect).

For example, when a counselor makes a telephone call to a family member whom a client has threatened, the counselor does so under the concept of duty to warn. If, on the other hand, the counselor arranges to have a client hospitalized who has threatened to harm themself, that action is taken under the concept of duty to protect, since it consists of a direct action (hospitalization) that makes it less likely that the client will execute the threat.

In the *Tarasoff* case, the victim was not protected, and the appellate court ruled that the health care provider should have taken steps to protect the victim. In addition, in *Gross v. Allen*, a 1994 California appellate court case, the court ruled that the *Tarasoff* concept applied not only to cases in which clients threaten homicide but also to cases in which clients threaten suicide, so reasonable measures are expected to be taken in cases of threatened suicide (Erford, 2023).

The *Tarasoff* ruling, in fact, establishes a duty to protect and not just a duty to warn. The ruling and interpretation of the *Tarasoff* principle for counseling professionals involves the awareness of the protective measure as a legal obligation to warn an intended victim of potential harm. For example, case outcomes have indicated that the duty to warn is extended to those who are foreseeably endangered by a client's conduct, including people who are unintentionally injured by the client, whole classes of students that have been threatened by the client, bystanders who may be injured by the client's act, and individuals whose property is threatened by the client (Corey et al., 2019; Remley & Herlihy, 2020).

Furthermore, in *Ewing v. Goldstein* (2004), the court ruled that in California, a therapist could be held liable for failure to warn when the information regarding the client's potential to harm another person is obtained from family members who may be participating in counseling with the client. The precedent established from this case requires a counselor to weigh the credibility of the person offering the information. Additionally, Remley and Herlihy (2020) point out that the *Tarasoff* principle is not applied in every jurisdiction of the United States; specifically, Texas has rejected it.

When a decision is made that a client is a danger to another person or persons, the counselor must then determine what the necessary steps are to prevent the harm from occurring. After notifying the potential victim or contacting appropriate authorities as

needed to ensure the intended victim's safety, the counselor must take other steps to prevent harm, which can include continuing to work with the client on an outpatient basis. This option is considered less restrictive, whereas the option to pursue an involuntary commitment would be considered very restrictive.

These decisions are challenging for most counselors. Therefore, it is highly recommended that counselors seek peer supervision and consultation on a regular basis, but particularly whenever the issue of breaking confidentiality becomes a factor in practice. This principle is known as the *duty to consult principle* and can assist crisis counselors in decreasing liability issues in similar cases. Counselors should not practice in isolation, should practice according to reasonable standards, and should maintain accurate documentation to be able to verify when and why a decision to break confidentiality was made, particularly as it relates to the duty to warn (Knapp et al., 2017).

How *Tarasoff v. Regents* Affects Crisis Counseling

Collins and Collins's (2005) seminal work on the *Developmental-Ecological Model of Crisis Intervention* identifies three goals for a crisis worker dealing with a potentially violent individual:

1. Ensure that the client remains safe, reduce lethality, and stabilize the environment.
2. Help the client to regain short-term control.
3. Connect the client with appropriate resources.

The course of accomplishing these three goals, however, is balanced against the counselor's decision that a duty to warn exists and confidentiality must be broken in part to accomplish those very goals. Counselors working with people in crisis often encounter unique circumstances when a duty to warn or protect prevails.

The *Tarasoff* case and deliberations surrounding a counselor's duty to warn have become a routine aspect of counselor education and are an expected consideration in the standard of care. Counselors, however, should be cautious when broadly applying the duty to warn since various states may apply this principle differently and others have enacted no law like the *Tarasoff* decision. Johnson et al. (2014) reported that, while 34 states have an actual duty to warn in statute or common law, 11 states give permission to warn (which may not shield the counselor from liability) and six states have no such statute or common law guidance.

Furthermore, in those states that have some form of *Tarasoff* law, the health care providers to whom it may apply (counselor, physician, etc.) may vary widely, and these providers are often poorly informed of their state's law on the matter. For example, Pabian et al. (2009) queried psychologists representing four states regarding their knowledge of legal and ethical obligations to clients in imminent danger, which resulted in alarming findings: (1) the majority were incorrectly informed about their state's law and believed that they had a legal duty to warn when they did not or (2) they assumed that warning was the only available legal option when other options were available that did not impinge on client privacy.

While the principles of duty to warn and duty to protect are useful, counselors should not assume that *Tarasoff* applies without explicitly researching their state law. Counselors should be aware of the changing nature of legislation and of the pertinent regulations for their own state.

Interestingly, Houser and Thoma (2012) noted that, even though counselors in Texas may not break confidentiality to warn or protect a person whose life has been threatened by another, counselors are mandated to report suspected child abuse and have the option of reporting positive HIV results to various entities.

Likewise, the *ACA Code of Ethics* (2014) indicates that counselors are "justified in disclosing information to identifiable third parties, if the parties are known to be at serious and foreseeable risk of contracting the disease" (section B.2.c). The *ACA Code of Ethics* further stipulates, however, that counselors must confirm the diagnosis and assess the client's intent to inform the third party about the disease or to participate in any behavior that may be harmful to an identifiable third party. The word *justified*, as compared with *required*, warrants consideration; in addition, counselors are encouraged to evaluate their own thoughts about whether they will agree to work with, and how they will effectively work with, clients who are HIV positive. It is also important to keep in mind that not every exposure to HIV results in harm, thus making the process of determining how to proceed a gray area that a counselor must navigate carefully.

When dealing with this and other precarious issues, counselors are wise to stay informed about specific statutes in their jurisdiction that have been passed regarding the duty to warn and third-party conversations and, of course, to review the guidelines of confidentiality periodically with clients and seek supervision and consultation as needed (Corey et al., 2019; Remley & Herlihy, 2020). Case Study 3.6 takes a closer look at the duty to warn.

CASE STUDY 3.6

Assessing When to Implement a Duty to Warn

Ridley, a retail employee, escaped harm from a building during an active shooter incident. A few months later, Ridley began experiencing symptoms of posttraumatic stress disorder and was sent to counseling through an employee assistance program when Ridley's supervisors noticed an increase in conflicts with coworkers. In the most recent session, Ridley became agitated and shouted to the counselor, "I hate my boss and coworkers—I'm going to kill them all," after which Ridley abruptly left the session.

Discussion Questions

1. Does this incident indicate a duty to warn?
2. What *Tarasoff* criteria are met or not met?
3. How should the counselor proceed?

SPECIAL CONSIDERATIONS RELATED TO CRISES

Learning Objective 3.5 Describe special ethical, legal, and other considerations related to crises.

In this section, we examine special ethical and legal considerations related to crises, including negligence and malpractice, documentation and record keeping, informed consent, confidentiality, and termination in crisis intervention. We also discuss spiritual and multicultural considerations and crisis intervention in a technological world.

Negligence and Malpractice

Once a counselor and a client have entered a professional relationship, the counselor has assumed a duty of care toward the client. Providing the best care is an issue of competence, and counselors are responsible to ensure their decision-making process is sound and driven by evidence-based practices. Generally, when poor treatment interventions are selected, a counselor's competence may be called into question (Remley & Herlihy, 2020).

A breach of duty owed to another is considered *negligence*; negligence that occurs in a professional setting is *malpractice*. Professional negligence results when a counselor departs from the usual standard of practice that is expected of professionals in a similar situation (Corey et al., 2019). References to legal and ethical issues that can arise during practice are discussed throughout the *ACA Code of Ethics* (2014), particularly in section I. In section I, matters related to standards and the law, ethical decision making, and conflicts between ethics and laws are addressed.

Malpractice is established if the court finds that a counselor has breached a professional duty, failed to take appropriate action (omission), or did something that should not have been done (commission) relative to the standard of care (Remley & Herlihy, 2020; Scott & Resnick, 2012). A standard of care is clinical treatment that does not depart from what a reasonable and prudent counselor with similar training and in a similar situation would carry out (Corey et al., 2019; Zhang & Parsons, 2016). The challenge of a standard of care is that the "standard" may vary by court rulings. To prove malpractice, four factors (the four *D*s) must be present: "a dereliction of duty that directly results in damages" (Scott & Resnick, 2012, p. 540). In the event of a lawsuit, a counselor's only defense is documentation in the case file that proves that the counselor conformed to the standard of care. A means of protection is to never practice without having professional liability insurance.

Common reasons for counseling-related malpractice suits include issues around documentation, informed consent, client abandonment, premature termination, sexual misconduct, and failure to control a dangerous client (Corey et al., 2019). While the primary cause of litigation for counselors continues to be inappropriate relationships with clients, the inappropriate handling of suicide crisis is a close second (Bartlett & Forbes, 2015; Remley & Herlihy, 2020). According to the Healthcare Providers Service Organization (HPSO; 2021, 2022), proper documentation is crucial in legal action and is often the only source of evidence in a case; in fact, good documentation can keep a counselor out of court. Specifically, in crisis situations risk management concerns will focus on documentation, record keeping, informed consent, confidentiality, and termination. It is prudent for counselors to be familiar with risk management strategies, regardless of the client's situation (Zhang & Parsons, 2016). Basic elements of risk management include being client centered, being clinically appropriate, being supportive of treatment and the therapeutic alliance, having a working knowledge of legal regulations and issues, and remembering to do no harm (Simon, 2012). For example, Bartlett and Forbes (2015) state that "suicide litigation has resulted in many of the largest settlements awarded to plaintiffs; the act of suicide is deemed preventable by law, and as a result lawsuits brought against practitioners are pervasive" (p. 27). Read Voices from the Field 3.5 and complete Think About It 3.2.

VOICES FROM THE FIELD 3.5

Avoiding Litigation When Providing Crisis Counseling

Emmett Poundstone

When providing crisis counseling, two major elements are necessary to avoid litigation. First, the counselor must have the necessary education/training, experience, and credentials to provide crisis counseling. If there are any doubts, the counselor should consult with an individual who has more credentials or the state regulatory board. Second, the counselor must provide adequate documentation regarding the counseling services. If a counselor has any doubts about activities, it is advisable to consult with an attorney who has mental health experience.

THINK ABOUT IT 3.2

Think about the following article on boundary violations. Read the ACA legal case *Failure to Properly Monitor and Supervise Services Treating Counselor Who Continually Failed to Establish Professional Boundaries and Who Developed a Personal Relationship with the Client* found at https://www.hpso.com/Resources/Legal-and-Ethical-Issues/Counselor-Case-Study-Failure-to-properly-monitor. After a review of this case, discuss the following questions: What errors did the counseling supervisor make? How should the counseling supervisor have initially intervened? What are your thoughts on having supervision of supervision? What errors did the treating counselor make? What are your thoughts on the resolution of this case? Should there be a risk management recommendation for university-employed counselors?

Documentation and Record Keeping

There is a useful old saying in the helping professions, "If it's not written down, it didn't happen." A counselor may have done everything right with a client, but failure to document means that no evidence exists of the counselor's actions and decisions. Documentation is an essential part of the standard of care of the counseling relationship, as discussed in the *ACA Code of Ethics* (2014, section A). Appropriate professional documentation and record keeping display diligence and promote client welfare (Bond & Mitchels, 2014; Luepker, 2012). While client welfare is a counselor's main purpose, there is no denying that documentation can either help or hurt a counselor in legal cases (HPSO, 2022; Welfel, 2016). For improvements in charting, see *Documentation Do's and Dont's* (HPSO, 2021), which lists many effective documentation practices. Good documentation is particularly crucial in crisis situations such as suicide, homicide, mandated reporting (e.g., children, older adults, and vulnerable persons), and disaster relief where the risk of litigation increases. Complete Activity 3.3, which asks you to rewrite case note entries.

ACTIVITY 3.3

Examine the following case note entries. Rewrite these entries to improve them.

1. Client says doing better today.
2. Client's appearance is good.
3. Client went to the doctor as requested.
4. Client began new medication last week.

Zhang and Parsons (2016) suggest that client records should include identifying data, background/historical data, diagnosis and prognosis, treatment plans, informed consent, progress notes, and a termination summary. Diagnoses are seldom provided in a true crisis because it would be inappropriate to diagnose prematurely. Incorrect diagnoses lead to inappropriate treatment plans. Outside of a true crisis, counselors document a client's diagnosis and whether it was reported by the client or received with the referral (e.g., if the client was discharged from a hospital). It is important to indicate by whom and when the diagnosis was provided so that no confusion exists that the diagnosis was given by a party other than the counselor.

It is crucial that a risk assessment be conducted on every client, regardless of the stated presenting problem. If the assessment produces any degree of lethality (either stated or observed) it is equally important to justify the actions taken and to follow up in subsequent case notes on the status of the risk.

In addition, inappropriate information should not be included in client records, meaning anything that is not treatment related and that could be problematic if viewed by others. Such information includes personal opinions, discussions of a third party, sensitive information, and any past criminal behavior. However, in cases of threats to self or others, documentation of follow-up assessments should be included in the client's records (Corey et al., 2019; Remley & Herlihy, 2020; Zhang & Parsons, 2016).

The requirement to maintain and secure client records is not limited to ACA; it is also endorsed by organizations such as the American Mental Health Counselors Association, American School Counseling Association, American Association for Marriage and Family and Therapy, American Psychological Association, and National Association for Social Workers.

Counselors must keep documentation of their interventions with clients. Admittedly, crisis conditions (e.g., a disaster site) are not always conducive to writing case notes, but counselors working with clients in crisis need to clearly document exact dates, pertinent details, assessments, and treatment decisions. It is important to remember that although case notes must be succinct, they should never be vague. Case notes must be specific enough to demonstrate that the treatment reflected prevailing standards of care. The documentation must clearly illustrate the session with details that capture the essence of what occurred and validate treatment decisions. The need for client files to be in a locked and secured location applies during crisis events and providing for confidential file storage in crisis situations must be addressed in crisis planning (American Red Cross, 2022; Kolski & Jongsma, 2014).

Another consideration is that crisis responders are transitioning to the use of on-site and cost-effective electronic health records (EHRs) to meet the real-time data capture needs of vulnerable clients. Portable EHR systems employed in a crisis must be adaptable to situational changes and available to the provider in multiple languages (Glasgow et al., 2012) because disasters do not discriminate by geography. The ability of the EHR to document mental health data is an efficient and patient-centered approach in the field. The use of EHR is not limited to community-based disaster relief settings but has the potential for daily use in hospital emergency departments, where mental health emergencies are likely to present (Furukawa, 2011). This evolution in medical record keeping produces new legal and ethical considerations for counselors responding to crisis situations. Complete Think About It 3.3, which pertains to documenting in a mass disaster, and read Case Study 3.7, which looks at a client's privacy rights in relation to documentation.

THINK ABOUT IT 3.3

Think about being in a mass disaster situation. How would you keep records when in the field after a flood, tornado, or house fire? You are given the choice of documenting client information with an EHR system or on paper-based forms. Which option would you choose, and why? What would the advantages and disadvantages be of each documentation approach for you as the crisis counselor? What legal and ethical issues might you need to consider when comparing both approaches?

CASE STUDY 3.7

Privacy Rights Records

When Jeff was 16 years old, he saw a counselor for conduct issues. During one of those sessions, Jeff volunteered that he had tried on his sister's undergarments. Ten years later, Jeff is experiencing a life crisis. His estranged partner is suing for custody of their children, and Jeff is concerned that his childhood counseling record may be discoverable by the court.

Discussion Questions

1. If you were Jeff's adolescent counselor, how might you have best documented the issue described?
2. What information should and should not be entered into the clinical record?
3. Can you offer any reassurances regarding the retention and availability of the older record?
4. If you were the current counselor, how would you record this disclosure about previous counseling?
5. What information should Jeff's attorney be aware of regarding his rights of confidentiality?

Informed Consent

Clients enter counseling without much information about what will take place. They may in fact often have many misconceptions about the process of counseling and the nature of the counseling relationship. Clients must enter counseling freely and be fully informed of what to expect in the counseling relationship. Informed consent occurs when clients understand all the possible risks, benefits, and potential outcomes of counseling (Remley & Herlihy, 2020). The *ACA Code of Ethics* (2014) states that "counselors have an obligation to review in writing and verbally with clients the rights and responsibilities of both counselors and clients" (section A.2.a). The *ACA Code of Ethics* further specifies that the counselor must explain to the client "the purposes, goals, techniques, procedures, limitations, potential risks, and benefits of services; the counselor's qualifications, credentials, relevant experience, and approach to counseling; continuation of services upon the incapacitation or death of the counselor; the role of technology . . . " (section A.2.b). Informed-consent forms should also include an explanation of office policies, fees, billing

arrangements, record keeping, the right to refuse treatment, and the potential effect of refusal. Both client and counselor must sign the informed-consent form for it to become part of the counseling record.

In a crisis or large-scale disaster, obtaining informed consent will rarely involve papers and a clipboard (American Red Cross, 2022). Documented consent in such circumstances may be impossible, impractical, or delayed at a minimum. For example, a counselor may be called on to provide support to someone wrapped in a thermal blanket at the scene of a disaster standing in front of the ruins of their home. Counselors should introduce themselves, explain their role, describe the general limitations of confidentiality, and ask permission to speak with the client. The counselor may ask a few practical questions:

- "What are you not comfortable with me disclosing when I contact those you have asked me to?"
- "How do you want me to identify myself and my association with you when I call?"
- "If I contact a person who does not answer the phone, may I leave a message and identify myself?"

If a crisis victim refuses intervention, the counselor should withdraw, offering to be available if future service is wanted. If the client agrees to speak with the counselor, consent needs to be reassessed throughout the exchange and may be withdrawn at any point should the survivor elect to discontinue (American Red Cross, 2022). The privacy of the client should be protected as much as the circumstances of the disaster allow. Protecting privacy may involve moving the survivor from more trafficked areas or moderating one's voice to minimize the likelihood of being overheard. Often the counselor may be asked to contact others on the survivor's behalf, such as community resources or loved ones. The counselor should clarify with the survivor that they give consent for the contact, who they wish to have contacted, and what specific information to share, being mindful to share only the minimal amount necessary. If a client is unable to sign, the crisis counselor should note why there was no written consent.

The COVID-19 pandemic and online services created an awareness of even more informed consent challenges when one considers the harmful effects of pandemic-related stress, including changes in the quality and patterns of sleep, difficulty concentrating, increased use of substances, mental health disorders, and suicide (Centers for Disease Control and Prevention, 2020b; Thakur & Jain, 2020). Clients became even more vulnerable due to the negative effects of social isolation, pandemic-related anxiety, lifestyle changes, economic problems, fear of poverty and homelessness, and instances of COVID-19–related discrimination compound mental health distress (Fiorillo & Gorwood, 2020; Litam, 2020; Litam & Hipolitio-Delgato, 2021; Litam & Oh, 2020), making informed consent even more challenging to navigate.

Confidentiality

Gladding (2017) defined *confidentiality* as "the professional, ethical, and legal obligation of counselors that they not disclose client information revealed during counseling without the client's written consent" (p. 34). The *ACA Code of Ethics* (2014) states that "counselors disclose information only with appropriate consent or with sound legal

or ethical justification" (section B.1.c). As noted earlier, in situations where a client is a danger to self or others or where the professional counselor is mandated by law to report abuse or is required by a court of law to disclose information, confidentiality can be legally breached (HPSO, 2021). In situations other than these, a client must give consent for the release of information. Client records are confidential, and they must be kept in a secure and locked location for a specified time in accordance with professional ethical standards and the legal requirements of the state and HIPAA and FERPA regulations (Zhang & Parsons, 2016).

In crisis and disaster circumstances, provisions for confidentiality are certainly more challenging; however, the ethical standards and requirements of the profession still apply. Counselors should be aware of the latitude that may be allowed for confidentiality in difficult circumstances yet strive to ensure the protection of client information. For example, it may be difficult for a university to conceal the names of students who were victims of a sexual assault as the media may acquire the names and report details of the incident before a university spokesperson can make an official statement. While university law enforcement officers generally work in conjunction with other local and state authorities, it is not always possible for the details to remain sealed. Furthermore, it is also impossible to prevent media sources from listening to police and hospital scanners and reporting information that is potentially false ahead of the justified facts.

Finally, there are numerous special considerations when providing services to school-age children. Many of these considerations are addressed later in the book (especially in Chapter 13). But for now, it is essential to understand that crisis counseling with children can become quite complicated. Surely, counselors need to convey to children in developmentally appropriate language what confidentiality means as well as its limitations. Parents and guardians are often hypervigilant and overprotective during crisis interventions, so counselors must be clear about boundaries and limitations while still establishing rapport, a therapeutic alliance, and intervening to benefit the child.

Another important ethical consideration for school counselors is ensuring all students are accounted for and safely returned to their guardians. This means taking attendance during evacuations and introducing a checkout system for student departures, such as when parents and guardians are required to pick up all children being sent home because of a crisis situation. As specified in Chapter 13, school crisis intervention teams need to think about how these elements of crisis management will be implemented before a crisis actually occurs. Now complete Activity 3.4, which focuses on client confidentiality. Then read Voices from the Field 3.6 and 3.7 and complete Think About It 3.4.

ACTIVITY 3.4

You have just determined that your client is a potential suicide threat and must break confidentiality and notify someone to discuss a plan of safety. Decide whom you will contact and exactly what you will discuss. Write a narrative of the conversation you will have with both the client and the person(s) to whom you will be disclosing. Consider how much you may disclose, what guidelines you are given by the *ACA Code of Ethics* to handle this scenario, and what other sources you might consult to determine how to guide your conversations.

VOICES FROM THE FIELD 3.6

Confidentiality and the Suicidal Client

Dr. Mary Bartlett
It is no small matter to break confidentiality, so it is important not to do so without first consulting another counseling professional regardless of how long you have been practicing. Two important things to remember are to avoid practicing in isolation, especially when working with a client who is having suicidal thoughts, and to document the conversation you have with another professional as input received in consultation about the case. This helps to ensure you are following best-practice guidelines and have considered all angles before breaking an important trust with your client even if ethically obligated to do so.

THINK ABOUT IT 3.4

Think about potential breaches of confidentiality. Read the ACA legal case *Alleged Breach of Confidentiality* found at www.hpso.com/risk-education/individuals/legal-case-study/Alleged-Breach-of-Confidentiality. After reviewing this case, discuss the following questions: What would you have done differently? How would supervision have helped? How would peer consultation have assisted this counselor? When would a conversation with an attorney have been appropriate?

VOICES FROM THE FIELD 3.7

Confidentiality During Chaotic Circumstances in the Field

Erin Shifflett
When providing counseling services in a disaster mental health capacity, you may find yourself facing a variety of ethical challenges. One issue to consider is the feasibility of ensuring confidentiality in potentially chaotic situations. Bear in mind that many first responders with whom you will be working may request access to information to maintain the welfare of those being helped collaboratively. Approach these requests from a treatment team perspective to ensure clients feel informed and empowered at a potentially disempowering and confusing time in their lives.

Termination in Crisis Intervention

Because crisis intervention is generally brief and focused on immediate needs, crisis counselors must use appropriate termination skills with clients. Appropriate termination should not only bring closure for the crisis counseling relationship but also bolster the goals and plans laid out in the crisis counseling process. A distinction should be made between termination and abandonment. Appropriate termination may decrease the allegations of perceived abandonment by a client.

Termination occurs when a counselor or a client decides to end the counseling relationship. According to section A.11.c of the *ACA Code of Ethics* (2014), professional counselors terminate cases when the treatment goals have been reached and the client no longer needs counseling, when the client is not being helped by further counseling or might be harmed by further counseling, when the client or someone associated with the client poses harm to the counselor, or when the client does not pay counseling fees.

In those circumstances where clients need further counseling services after termination, the counselor provides the client with referral information for other appropriate mental health providers. Unfortunately, lack of attention to termination documentation can lead to lawsuits based on abandonment (Corey et al., 2019). A final termination summary should be added to the client file. Bond and Mitchels (2014) recommend that this summary include the reason for termination, a summary of progress, the final diagnostic impression, a follow-up plan, and other pertinent information. Now, complete Think About It 3.5.

THINK ABOUT IT 3.5

Think about termination documentation. How would the termination summary for a client in a disaster response setting differ from the termination summary for a student graduating from a university? How would the termination summary for a client in a disaster response setting differ from the termination note for a client discharged from a clinical mental health center? When a client has a suicide crisis and does not return to the clinical mental health center following hospital discharge, how would the final client contact be documented?

When a client is neglected, deserted, or negligently terminated, the counselor has committed abandonment of the client. The *ACA Code of Ethics* (2014) prohibits abandonment; section A.12 states, "Counselors do not abandon or neglect clients in counseling. Counselors assist in making appropriate arrangements for the continuation of treatment, when necessary, during interruptions such as vacations, illness, and following termination." Special considerations for termination at university counseling clinics would include times when a student drops out of school, leaves school for medical purposes, is placed on academic probation or conduct suspension, or graduates from the institution.

A counselor must think about myriad factors when considering or faced with terminating a therapeutic relationship. Counselors should discuss the process of termination with the client during the first appointment and have clinical documentation of this discussion. While a discussion about termination at the outset of the therapeutic relationship does not prevent unforeseen circumstances for termination, it better prepares the client and goes far in the preservation of the counseling relationship. The following list highlights a few of the many circumstances that increase a counselor's risk of malpractice due to abandonment:

- The counselor is no longer receiving compensation from the client for professional services.
- The counselor recognizes that a referral might be in order.
- Therapeutic goals are no longer being achieved.
- The counselor is abruptly unavailable.
- The counselor fails to follow up after a client's hospital discharge.
- The counselor does not respond to a client's request for crisis treatment.
- The counselor allows managed care to dictate termination.
- The counselor lacks clinical coverage during planned absences (i.e., vacation time).
- Records do not verify outreach efforts to a client who breaks or misses appointments.

- A high-risk client drops out of treatment.
- A client is "fired" or refused treatment.
- A client is not notified in writing that a case is being closed.
- The client's record indicates a failure to review/consult/refer.
- Another treatment team member or staff member's notes do not verify that a treatment plan is being followed.

For further information on client abandonment, see ACA (2014), Corey et al. (2019), and Remley and Herlihy (2020).

Corey et al. (2019) recommend that clients be given written information about the closure plan, including contact information for other counselors or resources. If a client misses appointments and stops coming to counseling, the counselor should send a follow-up letter expressing interest in the client's welfare and asking the client to contact the counselor by a specified date if the client wants to resume counseling. If a termination letter is to be sent to the home of the client, written consent to send any information to that address should have been obtained during the informed-consent process. Similarly, written consent to leave any information on a client's phone should have been obtained during intake. The written consent should state the specific address or phone number for which the client has granted permission. The counselor should document all attempts to contact a client. Case Study 3.8 focuses on the factors a counselor must take into consideration when terminating a therapeutic relationship.

CASE STUDY 3.8

Termination Considerations

Alex lost a home and two family members in a series of tornadoes that recently swept across the state. Alex began to experience symptoms of acute trauma and approached you for care as a disaster relief counselor. You began seeing Alex regularly since arriving at the disaster site. Alex has engaged well therapeutically and appears to be making progress. You were notified by the Red Cross that your term of service will be up in a week and need to terminate with each of your assigned cases at the location. When you mention discontinuing services, Alex becomes distressed.

Discussion Questions

1. What obligations do you have to Alex?
2. How will you ensure Alex's continued recovery?
3. To what resources can you refer Alex?
4. Which ACA ethical standard(s) is(are) applicable in this case?

As mentioned previously, termination in a mass crisis or disaster circumstance should be understood from a special perspective. Formal assessment, admission, and ongoing counseling normally are not possible under such circumstances. According to the American Red Cross (2022), ongoing care and provision of counseling services

should be referred to local counseling services, case managers, and area providers to avoid therapeutic abandonment. An example of how such a conversation would occur follows:

Counselor: I'm glad we had the chance to work together these past few weeks since your home was destroyed. I understand this has been a very surreal and overwhelming time for you.

Client: I don't know what I would have done without you.

Counselor: Well, actually you did a lot of the work to move forward under difficult circumstances and have a lot to feel proud of. You have devised a plan of recovery for yourself, are in the process of securing more permanent housing, and have been able to return to work with transportation assistance. All of this means our work together has reached the right conclusion. At this time, I'd like to suggest community resources you may access for longer-term recovery support.

The severity of responses to mass crisis may have long-lasting effects for some. Psychological distress may remain high long after crisis counseling support services have been provided to those affected. This begs the question: What ethical responsibility do counselors have to ensure a healthy transition from crisis counseling to appropriate community-based treatment services once the additional support has been removed? There is some uncertainty about this limit of responsibility; therefore, many counseling training programs include crisis intervention coursework. For those who are already in practice, it is recommended they seek some training in crisis response counseling to explore whether they want to work with crisis populations or how they will work with a person who has survived a mass disaster and is referred to them for support (Norris & Rosen, 2009).

Spiritual and Multicultural Considerations

To dismiss discussions surrounding a client's religious or spiritual views related to a crisis is to devalue the client's worldview and not treat the entire person. Often, clients may entertain ideas that tragedy has come about because of God's wrath or disfavor. Counselors should be prepared to safely explore these concerns. This requires that counselors examine their own religious or spiritual biases and understand that many survivors have symptoms of secondary trauma of a spiritual nature (Zalaquett et al., 2009). Disaster workers might hear clients question whether their sin had brought the disaster upon them. Furthermore, disaster counselors may have to contend with remarks from volunteers working the crisis who may express such concepts of judgment to clients (Hard, 2020).

If the disaster occurs in a school system where the common separation of church and state is stressed, counselors must take that into consideration. Keep in mind that according to the U.S. Supreme Court, the First Amendment as applied to public schools was intended for public schools to not sanction or support a specific religion or faith over others. Counselors must find a balance between honoring the guidelines of the state and recognizing (and honoring) the needs of clients, regardless of where the tragedy took place (Remley & Herlihy, 2019).

In the case of a counselor responding to a person's life crisis, perhaps related to suicide, an examination of personal religious beliefs is essential to ensure maintaining a value-neutral assessment. Failure to take a nonjudgmental stance may result in further risk to the client; by assuming that the client has the same moral or religious beliefs, counselors may foster shame and anger in the client who then may withhold pertinent information needed to assess the level of lethality (Granello & Granello, 2007). Even secular counselors should exercise sufficient latitude to explore these questions with clients; doing less results in a failure to meet the needs of the client.

Research indicates that marginalized populations may find that predisaster norms set the tone for the postdisaster reality. Therefore, contrary to popular belief, disaster is not a social leveler, and underrepresented groups may find that their circumstances have declined (Hard, 2020). Persons in the sexual, affectional, intersex, and gender-expansive communities (lesbian, gay, bisexual, transgender, questioning [LGBTQ+]) may become further marginalized following a disaster. As Hard (2020) observed, while national service organizations (e.g., American Red Cross) may have LGBTQ+–affirming policies, individual volunteers may be from less accepting backgrounds, thus requiring counselors to act as diversity advocates on behalf of LGBTQ+ victims. Therefore, counselors should be versed in multicultural sensitivity and make efforts to indicate their sensitivity to multicultural issues.

The counselor's respect for cultural diversity should not vary according to setting, whether in a school, private practice, or university setting, which is often a multicultural microcosm. For example, Asian culture has a focus on time orientation that is either past or present (e.g., valuing ancestral wisdom and present experience; Hays & Erford, 2023). In contrast, Native American culture tends to be present oriented (i.e., focusing on the present experience; thinking about the present) when exposed to traumatic circumstances. These cultural variances require that counselors be aware of how time orientation and similar factors influence the crisis response of clients in order to accurately assess current mental status and needs (Remley & Herlihy, 2020). At the same time, it is important to note that the above observations are trends that do not necessarily apply to all persons within these cultures. Therefore, we should not place all persons from a perceived culture into the same template but assess individuals in accordance with their unique cultural perception. Understanding differences is important but making decisions based on assumptions may not be helpful.

In addition, some cultures are more socially focused rather than individually focused. Responses from people of these cultures will be directed toward what is most beneficial to the community's best interest. People from different cultures will vary in terms of eye contact, body language, affect, willingness to discuss personal matters, and in their response to authority figures. Ethical practice requires the development of multicultural competence among counselors; therefore, counselors are responsible for being well versed in multicultural competencies when serving others (ACA, 2014; Remley & Herlihy, 2020).

Crisis Intervention in a Technological World

Technology influences crisis intervention. In an age where many adolescents and adults seem always to have smartphones in hand, crises are reported immediately to authorities and help is often obtained quickly and in some instances in real time (e.g., via Skype or Zoom). Crisis intervention information and resources can be accessed quickly on one's

smartphone. Social media such as Facebook and Twitter provide opportunities to share experiences of a crisis in real time. It is also obvious that social media holds the possibility for quickly spreading misinformation. So, counselors must be aware of accurate information in real time to help clients navigate anxieties caused by false information. Additionally, a client in crisis can have easier access to a counselor or other mental health providers via text messages, e-mail, and other modes of electronic messaging. Importantly, evidence supports the effectiveness of tele-mental health for addressing depression, anxiety (Rees & Maclaine, 2015), and posttraumatic stress disorder (Turgoose et al., 2018). Videoconferencing, online forums (Kaur et al., 2016), smartphone applications (Kerst et al., 2020), text messages, and e-mails (Torniainen-Holm et al., 2016) are useful communication methods for mental health services.

Furber et al. (2014) found that both text messaging and phone-based psychotherapy were beneficial resources for clients in crisis. According to Almond and Budden (2012), text messaging is seen as a positive tool during hostage negotiation, suicide prevention, and general crisis intervention. Sude (2013) found that text messaging used in mental health practice increases the responsiveness of counselors to clients, allows clients to be more candid in their communication, is helpful to clients who prefer this method of communication, and may create a more connected feeling between client and counselor. The limitations of texting in practice include ethical concerns such as boundary issues, maintaining confidentiality, and potential for a diminished therapeutic relationship because the client and counselor are not face to face.

Technology also has influenced online crisis counseling services. Gary (2010) reported that online crisis counseling services are a valuable resource for clients in crisis who are geographically isolated, self-sequestered, or have limited access to face-to-face counseling (e.g., transportation problems, work release time). In a meta-analysis of the literature regarding online counseling, Richards and Vigano (2013) found positive client perceptions of online counseling with benefits in the areas of client anonymity, convenience, and client disinhibition. Conversely, although the digital domain may enhance speed and access to services, danger of the swift spread of disaster-related rumors and disinformation and exposure to those who prey on the vulnerable remain problematic. Counselors must stay attentive to new technological tools that can help them support clients in times of crisis and promote counselor and client safety.

Miller (2020) expands on the caveats of digital consultations versus face-to-face sessions. The virtual sessions may be more beneficial in certain circumstances when there is a danger that those who already have reduced access to health care (e.g., materially disadvantaged, less able to navigate the changes to health care delivery, and less digitally fluent). However, the digital divide also may increase polarization in mental health care.

During the COVID-19 pandemic, tele-mental health services were practical, time- and cost-effective, and appropriate for the support of clients, family members, and counselors (Smith et al., 2020), when increased mental health symptoms were observed among diverse populations. In particular, tele-mental health allowed isolated and remote clients access to services that were otherwise unavailable (Xiaoyun & Fenglan, 2020). Furthermore, expanding telehealth to include mental health reduced interpersonal contact and the spread of COVID-19 by reducing risk exposure of noninfected individuals in waiting room areas.

However, the use of technology during the pandemic was not all positive. Providers assumed that clients had Internet and computer access, could relate to the new method of care delivery given the technology-mediated relationship, and that the convenience and cost-effectiveness of providers working remotely would be beneficial, without determining the holistic cost to the patient. While the social isolation brought on by the pandemic may have reduced the viral spread, it also led to reduced access to family, friends, and other support systems and created feelings of loneliness, anxiety, and depression. Unmanaged mental health symptoms may have lengthened the health effects on patients, adding to the potential cost burden of managing the disease. Therefore, protocols for those with COVID-19 should address both the physiological and mental health needs of clients.

Now read Voices from the Field 3.8 for further insights into the use of telehealth counseling during the pandemic. Finally, Table 3.1 provides additional resources for further study and training.

VOICES FROM THE FIELD 3.8

Telehealth Counseling and the COVID-19 Pandemic

Emily Tucker, LPC, NCC

How did the pandemic change the way you practice and what were your initial concerns? Initially the greatest concern was how to navigate scheduled appointments while also honoring the safety concerns and mandated business closures. Fortunately, our administrative staff was responsive and assisted in communicating ongoing changes to clients and to each therapist. During the first month or two of the pandemic, appointments were handled over telephone or telehealth platforms such as HIPPA video, TheraPlatform, Zoom, or SimplePractice. These are just a few examples of the hundreds of telehealth platforms available today.

What was the most challenging administrative issue you encountered when moving from face-to-face versus telehealth? Initially, finding and evaluating telehealth platforms quickly while also coordinating schedule changes, new client intake, documentation, and other daily responsibilities felt overwhelming. The complete shutdown occurred within a few days, so there was a very limited time frame in which these important decisions had to happen. My counseling practice had to adapt and had to do so practically overnight. Peer resources, networking groups, and national and state agencies provided some clarity and guidance regarding these changes and the necessary order of importance. Personal health and safety from possible exposure to a novel virus was paramount, followed by maintaining continuity of care and connection with clients as a nearly equal second. This prioritization allowed some grace and time for providers to explore and transition to the best telehealth platform or method of providing telehealth services. Some providers, due to necessity in the client population or due to service type (e.g., working with children and families or those in crisis), were able to maintain an in-person practice with very specific prevention protocols in place. As time progressed, guidance increased for those meeting in person. Consent forms for in-person COVID-19 risk waivers were provided as well as similar informed consent documents for telehealth. In summary, action came first with a priority on continuity of care and maintaining client contact followed by various options to notate, document, and address liabilities and concerns of clients' insurance companies and providers.

What are the greatest liabilities and ethical concerns you faced when crisis counseling? That is an in-depth question because as a former crisis phone counselor I feel there are many layers to this answer.

(*continued*)

In that setting (i.e., crisis telephone hotline) I focused on responding to clients in crisis where they were in that moment. Connecting, communicating support, reflecting, exploring, and assessing for suicidal ideation were critical. This included discussing how and when the caller intended to act. This suicide assessment process required a delicate balance of gentle directive questions, reflective listening, and unconditional positive regard. Ideally, this balance honored the feelings of the client while helping me internally navigate my own reactions and fears for an actively suicidal client.

Concerns for liability were coupled with our roles as listener and holder of a safe place for the client to process their thoughts and feelings. This discussion typically included complicated and often polarizing feelings related to an inner experience of despair, helplessness, and isolation coupled with an urgency for relief from the intense emotional pain they are experiencing. As the listener and support person for someone in this much pain, a therapist or call center volunteer must maintain some inner sense of calm, curiosity, and connection with the client. Thus, self-care and self-awareness are critical to best serve our clients.

Liabilities also include documentation of location (including address), confirmation of privacy of the client, and review of consent and exceptions to confidentiality. As therapists providing telehealth services, we are required to first clarify to our clients' reasons we as counselors would have to break confidentiality while also seeking to navigate creating a sense of trust and safety. Building rapport with each client is essential. This is no different from face-to-face settings. The additional piece when working with someone via a telehealth platform is that we must confirm the location, address, and contact information of our client at the onset of every contact in case information is disclosed that requires immediate intervention (i.e., an actively suicidal client discloses immediate intent, means available, and intention to harm self or others during a call). Without ascertaining the address and location as well as an emergency contact, we are far less able to ensure an outcome that protects the client and honors the therapist's ethical and legal obligations. Essentially, it is difficult to send help to someone when we do not have their location.

Additional concerns surrounding liability included clients traveling or returning home, especially at the onset of the pandemic. In early 2020, each state required a licensure process unique to that state. A few states had mutually reciprocal agreements so that counselors could practice in either state, but this was not the case in my home state. Fortunately, the emergency guidelines put in place at the federal level and in many (but not all) states were generous about relaxing interstate practices and distance-counseling restrictions. Organizations like Person Centered Tech and IFS Telehealth Collective helped organize the overwhelming amount of legal, administrative, and ethics data as a resource for therapists struggling to continue serving their out-of-state clients while also complying with all laws and ethical considerations. All in all, the transition to telehealth had moments of discomfort and anxiety, as well as opportunities to develop and grow as a therapist and a human being. I have embraced the transition to telehealth and am grateful for this opportunity to continue connecting throughout a very challenging time.

TABLE 3.1 Resources for Additional Study and Training

American Counseling Association (ACA): The ACA offers written and interactive resources through their website. ACA's *Knowledge Center: Mental Health Resources: Trauma and Disaster* webpage (https://www.counseling.org/knowledge-center/mental-health-resources/trauma-disaster) provides links to a variety of disaster mental health and trauma resources. The ACA Store (https://imis.counseling.org/store) also provides resources. A single chapter may be downloaded from the Stebnicki text and studied for continuing education credit, and the text by Webber and Mascari is also available here and is a valuable resource:

- Stebnicki, M. A. (2021). *Counseling practice during phases of a pandemic virus*. American Counseling Association.
- Webber, J. M., & Mascari, J. B. (2018). *Disaster mental health counseling: A guide to preparing and responding* (4th ed.). American Counseling Association.

American Red Cross: The American Red Cross offers disaster mental health training and certification for mental health professionals as well as graduate students who wish to be called for service in disaster and crisis circumstances.

- American Red Cross. (2017). *Disaster health and mental health volunteers.* https://www.redcross.org/volunteer/volunteer-opportunities/disaster-health-mental-health-volunteer.html

Substance Abuse and Mental Health Services Administration (SAMHSA): SAMHSA offers online training modules in disaster and crisis counseling through their Distance Technical Assistance Center (DTAC).

- SAMHSA. (2020). *Online DTAC training courses*. https://www.samhsa.gov/dtac/education-training

SUMMARY

Legal and ethical matters can be confounding for counselors in the best of circumstances. In times of crisis, it is even more imperative that counselors be well acquainted with legal and ethical obligations. Crisis counselors may often find their normal coping mechanisms overwhelmed during disaster or crisis interventions. When the counselor's normal coping is impaired, lapses in professional judgment may occur. The prevention of ethical and legal entanglements is far preferable to remediation of problems following a lapse in behavior or judgment. This chapter discusses preventive considerations for crisis counselors, including wellness recommendations, continuing education, supervision and peer consultation, and ethical decision-making models.

HIPAA was enacted to address many perceived failings in the management of information in the health care industry. FERPA was enacted to address concerns about privacy violations and the inclusion of immaterial comments and personal opinions in student educational records. HIPAA and FERPA have not only added layers of protection to client records but also increased the complexity of counselor responsibility in protecting both the client's and their own interests. HIPAA and FERPA have provisions for crisis situations, but confidentiality is still central to ethical practice, even during times of crisis. Crisis counselors must be familiar with the particulars of HIPAA and FERPA, including when they apply and overlap, as well as other legal requirements in their state. Furthermore,